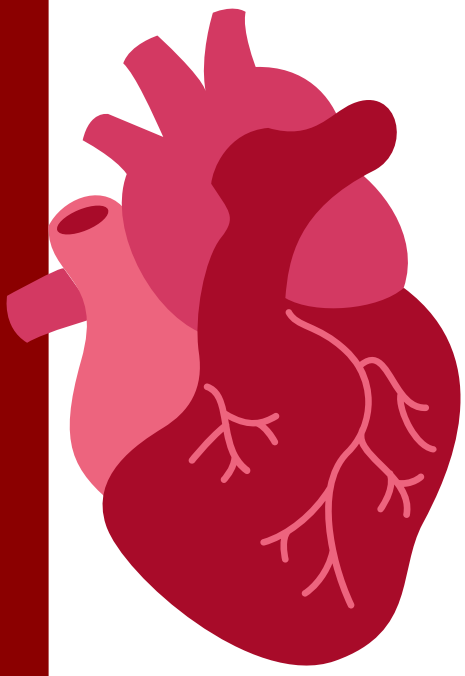
سُبْحان الله وبحمده، سُبْحان الله العظيم



MED25

Circulation & Breathing Module Testbank

RS Previous Years

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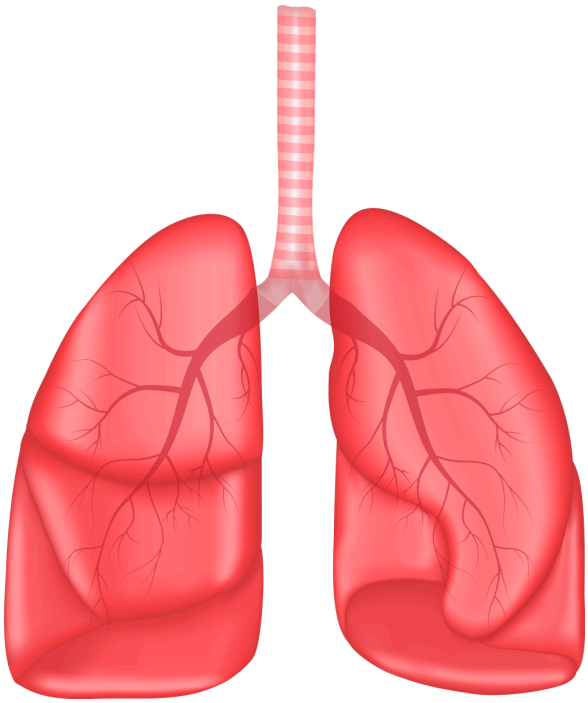
وزدني علمًا.



Fisrt Mid

Lessons Included:

1. **Drugs used in treatment of pulmonary Embolism**

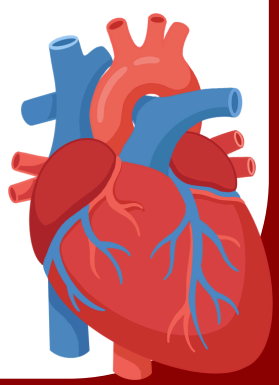
RS Final Locker21

18) Which of the following is one of advantages of LMWH?
a-easily montir using INT
b-cheap
c-safe in pregnancy
d- fully response to sulfate

23) What is the antidote for warfarin overdose?
a- Vitamin K
b-Vitamin B12
c-Vitamin D
d-vitamin A

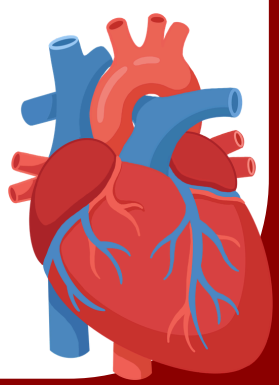
27) 32-year old female has a valvular heart disease and takes warfarin . She decided to follow a new diet with lots of green vegetables. How would that affect her?
a-Vitamin k is increased which makes warfarin less efficient
b- Vitamin c is decreased which makes warfarin more efficient
c- Vitamin k is decreased which makes warfarin more efficient
d- Vitamin c is increased which makes warfarin less efficient

18	23	27
C	A	A

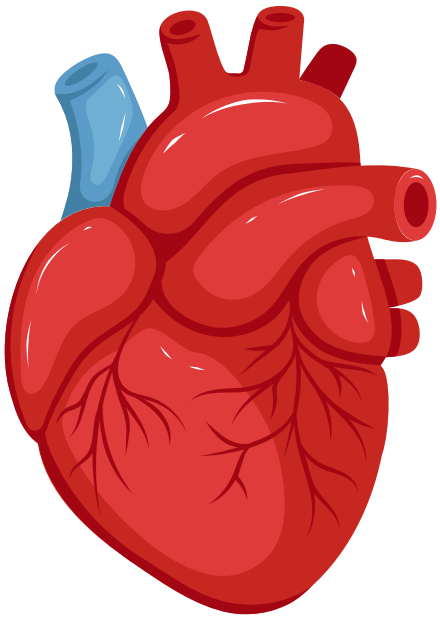


Final REPSA

58-patient with massive pulmonary embolism with hemodynamic instability what is the treatment ?
A-Alteplase
B-Heparin
C-Warfrin



58
A



Second Mid

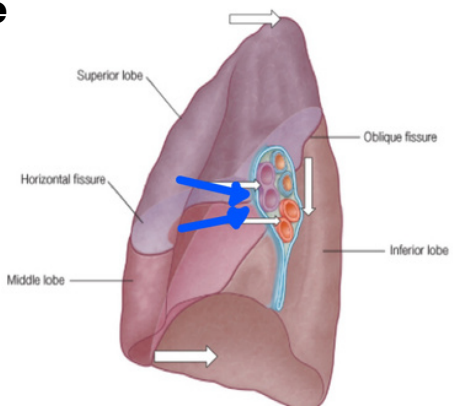
Lessons Included:

1. Anatomy of Thoracic Wall
2. Anatomical Relations of the lungs
3. Histology of the respiratory system
4. Development of respiratory system
5. Anatomy of nose and para nasal sinuses
6. Anatomy of the larynx
7. Acid-base Homeostasis & the role of respiratory system
8. Metabolic functions of the lung and Pulmonary Surfactant
9. Clinical approach to pleural diseases
10. Lung immunology: Innate and acquired immune responses in respiratory infections
11. Lower respiratory tract infections
12. Verminous pneumonia (Parasitic larvae invading lung)
13. Cystic parasitic diseases of the lung
14. Obstructive airway disease
15. Restrictive airway disease
16. Pathology of bronchial asthma
17. Atelectasis and acute lung injury
18. Pulmonary infection
19. Pulmonary vessel disease
20. Upper respiratory tract neoplasm
21. Lung and pleural tumors
22. Drugs used for treatment of asthma
23. Drug therapy for cough and COPD 1
24. Drug therapy of respiratory infections
25. Treatment of TB
26. Mechanics of breathing
27. Pressure-volume relationship
28. Gas diffusion and blood flow
29. Transport of oxygen in blood
30. Transport of carbon dioxide in blood
31. Neural control of breathing

Final REPSA

1. Relation of Vagus nerve to marked structure

A)inferior
B)anterior
C)posterior
D)superior



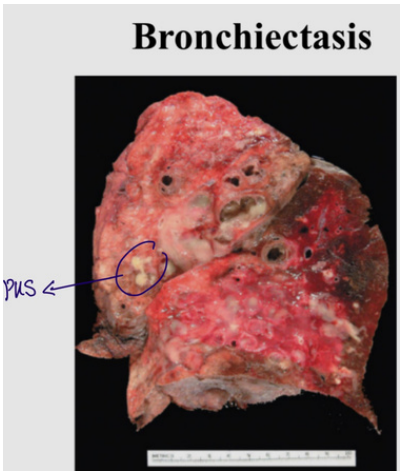
3. Which of the following lymph nodes groups is inside the lung?

A) pulmonary
B) Bronchopulmonary
C) Tracheobronchial
D) Pretrachial

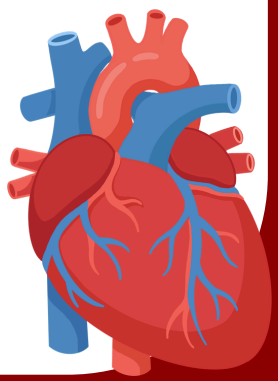
5. case about filamentus branching bacteria with sulfur, what is the risk factor?

A)Facial truma

6. Cystic fibrosis

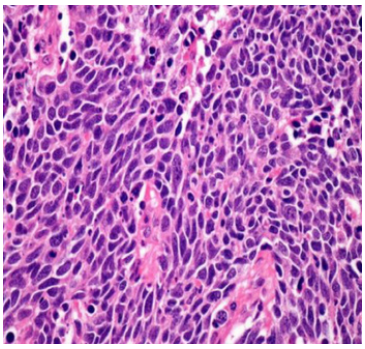


1	3	5	6
C	A	A	



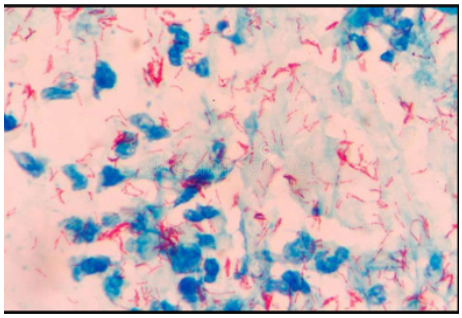
Final REPSA

7- Small cell carcinoma

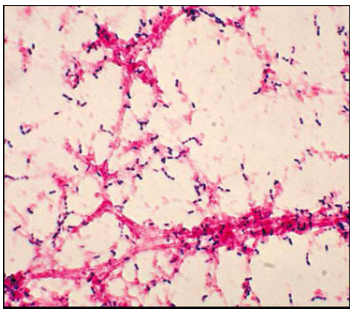


8-Centroacinar emphysema

9-Mycobacterium Tuberculosis

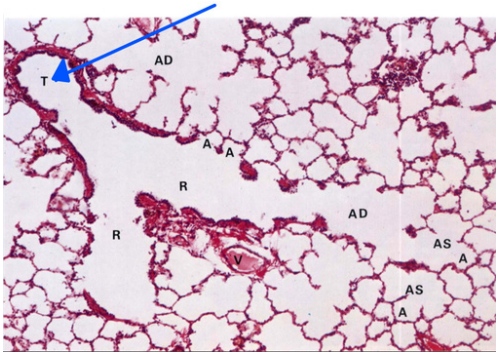


10-Streptococcus Pneumoniae

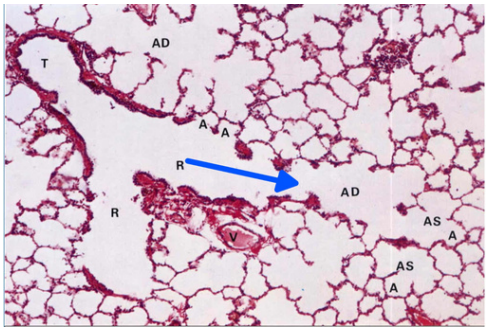


Final REPSA

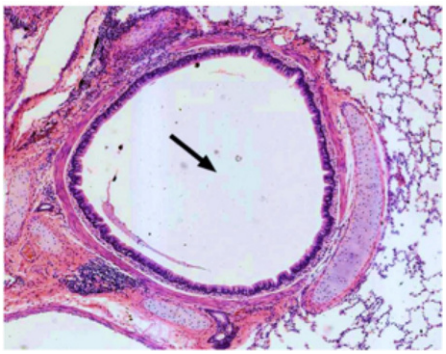
11- Terminal bronchi



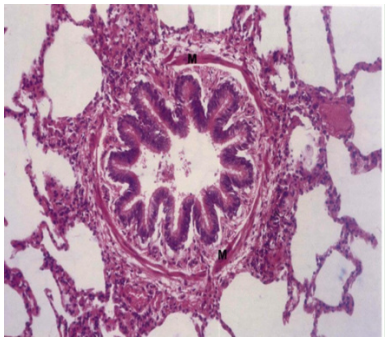
13-
A) Respiratory bronchiole
B) Alveolar duct ✅



14- Bronchus

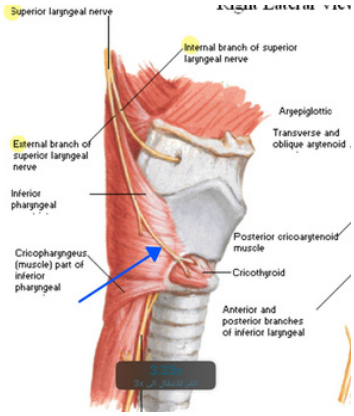


15- Bronchioles

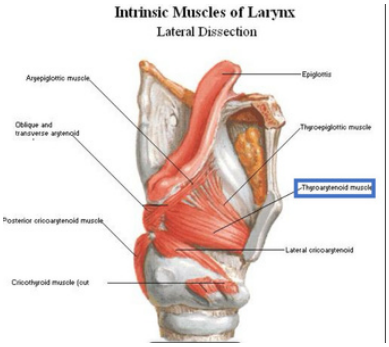


Final REPSA

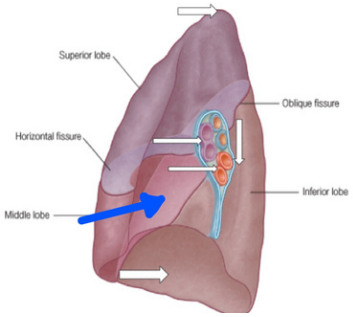
16- What is the action of this muscle that is supplied by this nerve?
A) Tension and elongation



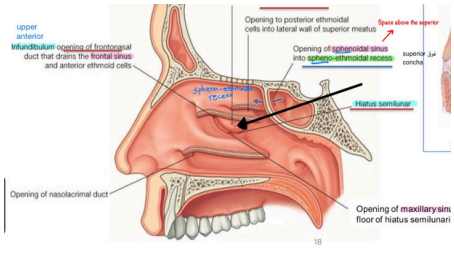
17- what is the action of this muscle
A) Relax and shortening



18- Right atrium:



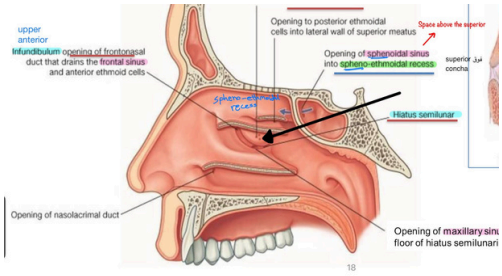
19-
1-identify the structure:
bulla ethmoidalis
2-what paranasal sinus forms it:
middle ethmoidal sinuses



Final REPSA

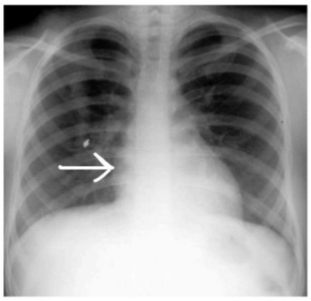
21-identify the bone related to this structure:

A) ethmoid bone



22)

2- Identify the marked structure?



✓A. Right atrium

25 - purulent exudate and low PH and glucose

A) Empyema

B) Exudative effusion

C) Transudative effusion

26- which of the following can not be measures by spirometer?

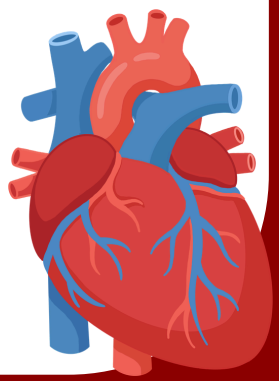
A-RV

B-IV

C-EV

D-TV

21	22	25	26
A	A	A	A



Final REPSA

27- centered x-ray?
A) Equal distance between the medial end of the clavicle and midline

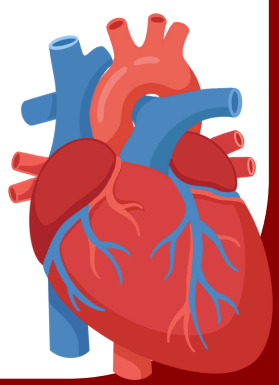
28. Question about TLC in the graph

30. Patient has TB and was prescribed a drug that causes orange discoloration:
A) Rifampin

31. most common sinus that infected?
A) Maxillary

32- What prevents fluid from moving from the pulmonary capillaries into the alveoli?
A. The repelling forces of the surfactant
B. Negative intrapleural pressure
C. Negative transthoracic pressure
D. Pulmonary Hydrostatic pressure

33 - Why arterial blood shows increased oxygen saturation than venous blood ?
A. Due to increased Pa02 in arterial blood more than venous blood
B. Due to increased PaCO2 in arterial more than venous blood
C. Due to decreased amount of hemoglobin in venous blood
D. Due to increased temperature in arterial blood increase oxygen saturation
of hemoglobin



27	28	30	31	32	33
A	-	A	A	A	A

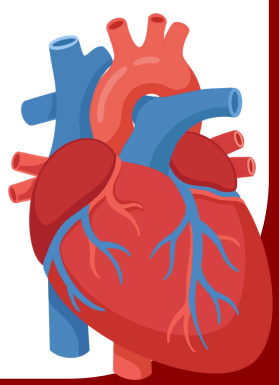
Final REPSA

34- Which of the following neurons limit the extent of lung inflation?
A.Hering-Breuer (Stretch) lung receptor
B. Irritant receptor
C. J receptors
D. Chemoreceptors

35- A patient with a stroke results in haemorrhage the medulla center, which of the following will be the outcome ?
A. Stoppage breathing
B. Apneustic breathing
C. Ataxia breathing
D. Irregular breathing

36- A patient sustains a stab wound to the plura, resulting in trauma to the mediastinum and chest wall. Several major vessels are injured, and the patient suddenly becomes short of breath. And increased air in plura by 1cm. Which of the following is the most likely cause of his respiratory difficulty?
A. Pluritis
B. atelectasis
C. Pneumoconiosis

37- Redix index?
A. The ratio of thickness of submucosal gland to bronchial wall
B. The ratio of thickness of cartilage to bronchial wall
C. The ratio of thickness of mucosa to bronchial wall
D. The ratio of thickness of muscular part to bronchial wall



34	35	36	37
A	A	B	A

Final REPSA

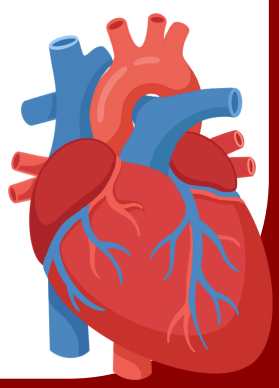
38- What is the mode of transmission of rhinovirus ?
A. Aerosol
B. Food
C. Contact
D. Blood

39- Which of the following is the most causative organism to community acquired pneumonia ?
A. Streptococcal pneumonia
B. Mycoplasma pneumonia
C. Respiratory syncytial virus
D. Candida

40- What is the preventative measure for Echinococcus?
A. Removal of snails
B. Avoid contamination of hands, food, and water with dog feces
C. Wear preventive clothes (gloves - shoes)
D. Proper washing of raw vegetables

41- Patient ate green salad and got a parasitic infection with increased eosinophilia?
A. Ascariasis
B. Schistosomes
C. Toxocariasis
D. Entamoeba histolytica

38	39	40	41
A	A	B	A



Final REPSA

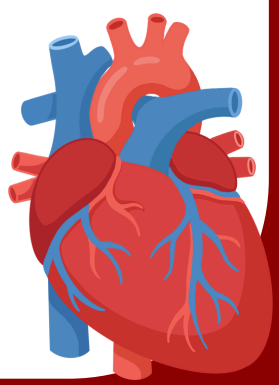
42- A 32-year-old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?
A. Levofloxacin
B. Ampicillin
C. Cefuxime
D. Clavulanic acid

43- A 35 years old pregnant lady visited the clinic with a mild fever and scanty sputum, CXR showed diffuse bilateral infiltration. What is the suggested drug for this patient?
A. Azithromycin
B. Doxycycline
C. Levofloxacin
D. Cephalexin

44- Hormone produced in excess immediately after climbing a mountain?
A. Tyroxin
B. Natruetic peptide
c.Epinephrine
D. Erythropoietin

note: (erythropoietin does increase, but not immediately)

42	43	44
A	A	C

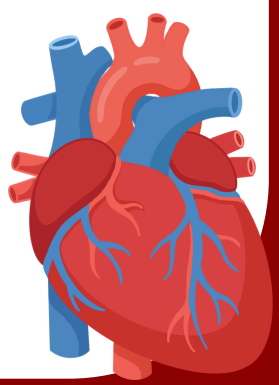


Final REPSA

45- A patient develops cyanosis with bluish discoloration of the lips and fingers. Which of the following is the most likely cause?
A) Cyanide poisoning
B) Heart failure
C) Carbon monoxide poisoning

46-A 50-year old female african amirecan patient came in with Dry cough , Bilateral hilar lymphadenopathy with Non-caseating granuloma. Which of the following is the most likely diagnosis?
A. Sarcoidosis
B. Pneumoconiosis
C. Emphysema
D. Asthma

46-A 50-year old female african amirecan patient came in with Dry cough , Bilateral hilar lymphadenopathy with Non-caseating granuloma. Which of the following is the most likely?
A) Lymphocytes
B) CD8 t cytotoxic
C) CD4 t helper



45	46.1	46.2
B	A	C

Final REPSA

47- A 56-year-old alcoholic man presents to the clinic with. An X-ray reveals in the posterior segment of the upper lobe. What could be the cause of this ?

A. Aspiration of gastric content
B. Pulmonary embolism
C. Viral pneumonia
D. Tuberculosis

48-Tuberculosis which type :

A)Hypersensitivity type 1
B) Hypersensitivity type 4
C)Hypersensitivity type 3
D)Hypersensitivity type 2

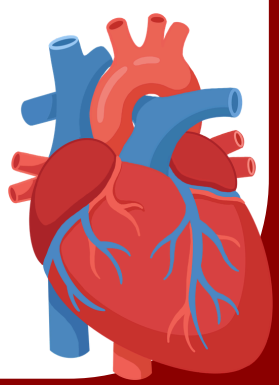
49-Deoxycycline adverse side effect:

A. Prolonged QT interval
B. Gastrointestinal problems
C. Ototoxicity
D. Photosensitivity

50- Which of the following reduces the effectiveness of salbutamol?

A. Propranolol
B. Theophylline
C. Corticosteroid
D. Tiotropium

47	48	49	50
A	B	D	A



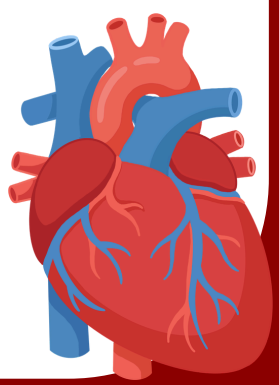
Final REPSA

51- A 70 kg woman came to the ER with acute exacerbated asthma. After setting an oxygen cannula, what is the first line drug to be used for treatment?
A. Salbutamol nebulization
B. Slow IV Cortisone
C. Aminophylline slow IV infusion
D. Ipratropium nebulization

52-patient take a corticosteroid drug ?
A) wash mouth after use

53- Ali is a 65 years-old man with COPD. He is currently using albuterol inhaler PRN (when required), Salmeterol inhaler twice a day, and tiotropium once a day. His COPD is still uncontrolled with frequent symptoms and recurrent exacerbation. What recommendation would you make to his regimen?
A. Add on oral corticosteroids once daily
B. Add on theophylline once daily
C. Change the ipratropium inhaler to ipratropium in scheduled four times a day
D. Change salmeterol to fluticasone/salmeterol combination in scheduled two times a day

51	52	53
A	A	D



Final REPSA

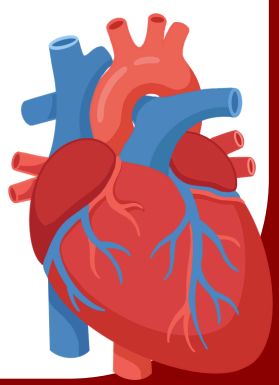
54- Which of the following anti-TB drugs causes visual disturbances, including slight color blindness and vision impairment?
A. Isoniazid
B. Ethambutol
C. Pyrazinamide
D. Rifampicin

55-A patient presents with fever and features of lower respiratory tract infection. He gives the history of keeping birds and parrots. What is the causative organism?
A) Chlamydia psittaci
B) Legionella pneumophila
) Mycoplasma pneumoniae
Respiratory syncytial virus (RSV)

56-Drug cause organ-red color of secretions
A-Rifampicin

57-Which of the following lung tissues lack smooth muscle?
A) Alveoli
B) Trachea
C) Bronchus
D) Bronchioles

54	55	56	57
B	A	A	A



Final REPSA

59- Surface area of alveoli continues to increase until:
A. Adult stage
B. Before birth
C. 8 years
D. Childhood

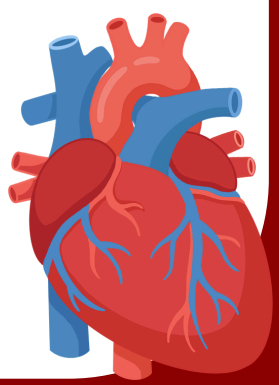
60-Contributes to the irreversible airflow obstruction (asthma)?
A)Sub-basement membrane fibrosis and smmoth muscle haypertrophy

61-About bacteria cause -Rheumatic fever
A-Streptococcus pyogenes
B-Streptococcus agalactiae

62 -(هو مأشر على صورة) Superior & middle concha parts:
A-Ethmoid bone
B-Sphenoid bone

64-Which of the following is found in high concentration and is therefore the most effective buffer system at physiological pH?
A. Phosphate buffer
B. Bicarbonate buffer
C. Hemoglobin
D. Plasma protein buffer

59	60	61	62	64
A	A	A	A	B



Final REPSA

65- Which of the following shows the amount of Oxygen in the blood?
A. Oxygen content (Ca 02)
B. Oxygen Saturation
C. PO2
D. None

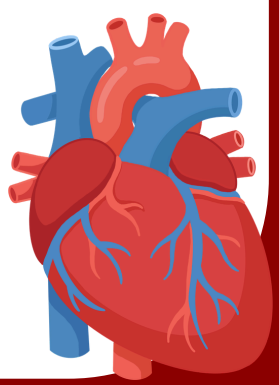
66- A 68-year-old man with COPD developed community-acquired pneumonia.The most causative agent is:
A) Mycoplasma pneumoniae
B) Legionella pneumophila

67-Patient has bull neck appearance what is treatment :
A) Penicillin and toxoid
B) Penicillin and anti-toxoic
C) Penicillin and gentamicin

68- what is the cause of pneumonia post influenza infection :
A) staph aureus

69- A 68-year-old man developed community-acquired pneumonia after undergoing hip surgery. What is the most likely causative agent?
A) Pseudomonas aeruginosa

65	66	67	68	69
A	A	B	A	A



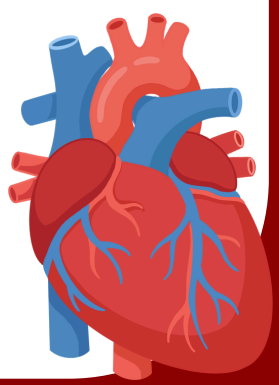
Final REPSA

70. Which of the following is the best investigation to diagnose ?
A) Chest CT angiography
B) Chest X-ray
C) blood count

71. A patient with chronic hypercapnia is being evaluated for respiratory drive. Respa drive .... replaced by .... drive?
A)hypercapnia - hypoxia
B)hypoxia-hypercapnia
C)apnustic-pnumotacsic

72-What is the role of respiratory bronchioles in gas exchange?
A) Complete gas exchange
B) moderate gas exchange
C) No gas exchange at all

73-A 40-year-old patient presents with progressive dyspnea and weight loss. Thoracentesis yields a white, milky effusion.What condition is indicated by this type of effusion?
A) Transudate from heart failure
B) Malignancy with bloody effusion
C) Chylothorax
D) Infected effusion



70	71	72	73
A	A	B	C

Final REPSA

74-Patient has TB and starts treatment of drugs and causes tingling in hand and feet. What is the drug that will prevent this ?
A)Isoniazid
B)Rifampicin
C)Ethambutol
D) B6

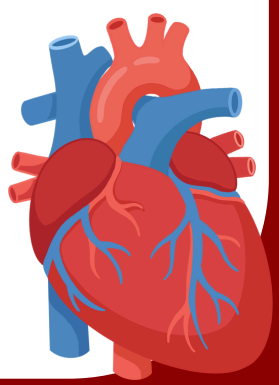
75-parasite penetrate the patient with filiform larvae. What the disease ?
A-Ancylostomiasis
B-SCHISTOSOMES

76- TH will recruit eosinophil by
A)IL-5
B)IL-4
C)IL-10
D)TNF-a

77- rapid test for TB?
A)Polymerase Chain Reaction (PCR)
B)BACTEC

78- female non smoker ... gland like ... EGFR mutation?
A)Adenocarcinoma

74	75	76	77	78
D	A	A	A	A



Final REPSA

79- male smoker... keratin like ...cavity inside tumour ?
A)Squamous Cell Carcinoma

80- case about boy has situs inversus and infertility
A) defective dyenein arm
note: Bronchiectasis (immotile cilia syndrome)

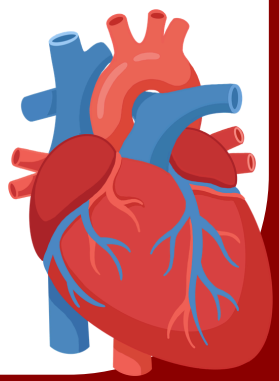
81- Cystic Fibrosis مو متذكره بس في سؤال له علاقه ب

82- Case about stony dull (بس جابوه بفاينل الريسبا 🤡 ECE)
A)plural effusion

83- case about BMPR2 mutation what is the mechanism of pulmonary arterial hypertension?
A)dysfunction of endothelium and proliferation of vascular smooth muscle cells
B)left atrial hypertension
C)thromboembolism

84- patient on ventilation develope infection what could be the causative organism
A)Pseudomonas

79	80	81	82	83	84
A	A	-	A	A	A



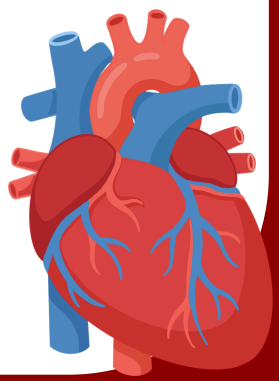
RS Midterm Locker21

1) Which of paranasal is more liable to get infected?
a- anterior ethmoidal
b- frontal
c- maxillary
d- sphenoidal

2) Which of the following is the main source of CO2 production?
a- incomplete oxidation of CHO and Fats
b- oxidative metabolism
c- oxidation of sulfa containing amino acids
d- Anaerobic respiration of glucose

3) which of the following drugs have tolerance to its own therapeutic effects?
a- Short acting beta2 agonists
b- montelukast
c- theophylline
d- cromolyn

4) a 42 year old man returned two weeks ago from a five month trip to Ghana . During his stay he had frequent visits to Lake which includes swimming in the lake. He present with fever, cough wheeze myalgia headache and urticaria.stool analysis revealed 2-3 eggs of schistosoma mansoni. What is the stage?
a- chronic schistosomiasis
b- Cercarial penetration (invasion stage)
c- cercaria (migration stage)
d- acute Schistosomiasis (Katayama)



1	2	3	4
C	B	A	D

RS Midterm Locker21

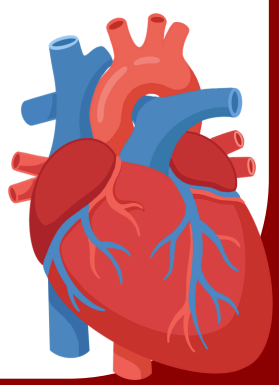
5) A 40 year old male has an infection in the nasal cavity and also in the maxillary air sinus. This infection is making a mass in the hiatus and blocking it. Where is the block located ?
a- superior meatus
b- inferior meatus
c- middle meatus
d- sphenoethmoidal recess

6) A 35yo pregnant lady has visited the clinic with a mild fever and scanty sputum, CXR showed diffused bilateral infiltration, what is the suggested drug for this patient?
a- Azithromycin
b- Doxycycline
c- Levofloxacin
d- Cephalexin

7) what is the most common form of oxygen carried in the blood?
a- physically dissolved in plasma
b- bound to hemoglobin
c- bound to plasma protein
d- as bicarbonate

8) What is the mode of transmission of rhinovirus ?
a- aerosol
b- Food
c- Contact
d- Blood

5	6	7	8
C	A	B	A



RS Midterm Locker21

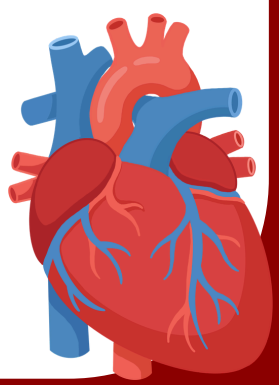
9) Which of the following reduces the effectiveness of salbutamol?
a- Propranolol
b- Theophylline
c- Corticosteroid
d- Tiotropium

10) 17 years old boy with strong family history of asthma with increase esionophil, what the advice that you will give him?
a- stop smoking
b- Avoid pollen grains
c- Avoid exercise and swimming
d- Avoid stress and anxiety

11) CO2 is mainly transported in the blood in the form of:
a- dissolved in plasma
b- bind to proteins
c- Carboxy-Hb
d- Bicarbonate

12) A 54 year old man with a history of hypertension he missed to receive his anti-hypertensive for days , but there was a decrease in the pressure noticed . What cause this decrease in pressure?
a- Cerebral cortex
b- Carotid body
c- Arterial baroreceptor
d- Proprioreceptor

9	10	11	12
A	B	D	C



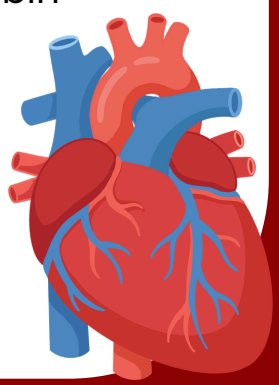
RS Midterm Locker21

13) Contraction of which of the following muscles responsible of the major lung compliance that increase lung volume ?
a- sternocleidomastoide muscle
b- External intercostal muscles
c- Diaphragm
d- Internal intercostal muscles

14) What is the common extrahepatic site of ambiosis?
a- Peritoneum
b- kidney
c- spleen
d- lung

15) A 68 year old diabetic man presented to the ER with productive cough, purulent sputum, and shortness of breath. Chest X-ray shows consolidation in the lobe of the lung. What organism is most likely the cause of his symptoms?
a- Chlamydia Psittacosis
b- Haemophilus Influenzae
c- Streptococcus Pneumonia
d- Staphylococcus Aureus

16) Why is the normal adult hemoglobin considered an efficient O2 carrier?
a- Hb has changeable affinity to O2 according to bioavailability
b- Due to highest affinity of binding to O2 compared to other gasses
c- Due to linear relationship between PaO2 and % hemoglobin saturation
d- Due to the stability of Hb-O2 binding



13	14	15	16
C	D	C	A

RS Midterm Locker21

17) A 56 year old man came to the clinic complaining of weight loss and progressive dyspnea , the doctor noticed that the patient shows pursed lip breathing. What is the histological feature of the underlying disease?

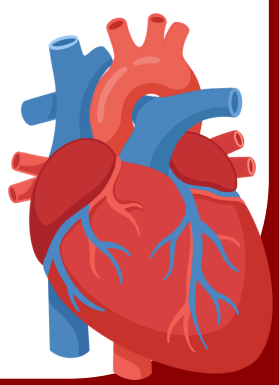
a-dilation of bronchi with destruction of their wall
b- Bronchial mucus glands hyperplasia
c- Alveolar damage with proteinaceous fluid in alveolar space
d- Destruction and enlargement of alveolar walls

18) Which of the following lymph nodes drains the carina of the trachea?

a- bronchopulmonary
b- Tracheobronchial
c- Hilar lymph nodes
d- Bronchopulmonary

19) A 68 year-old-male has COPD with moderate airway obstruction. Despite using salmeterol twice daily as prescribed, he reports continued symptoms of shortness of breath. Which one of the following agents would be an appropriate addition to his current therapy?

a- Systemic Corticosteroids
b- Albuterol
c- Tiotropium
d- Theophylline



17	18	19
D	B	C

RS Midterm Locker21

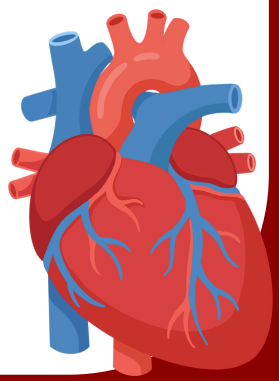
20) A 70 kg woman came to the ER with acute exacerbated asthma. After setting an oxygen cannula, what is the first appropriate drug to be used for treatment?
a- Salbutamol nebulization
b- Slow IV Cortisone
c- Aminophylline slow IV infusion
d- Ipratropium nebulization

21) A full term newborn came to the hospital he had pale skin peripheral cyanosis, a genetic analysis was and discovered that there a hydrophobic protein deficiency causing respiratory distress whats the messing protein?
a-SP-B
b-SP-D
c-Surfactant
d-dipalmitoyl phosphatidyl choline

22) Which of the following structures is seen in pseudoglandular phase?
a- terminal bronchioles
b- respiratory bronchioles
c- alveolar ducts
d- alveolar sacs

23) Which of the following structures is related to the medial side of the left lung?
a- Left atrium
b- arch of aorta
c- SVC
d- azygos

20	21	22	23
A	A	A	B



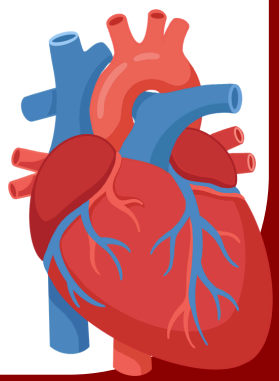
RS Midterm Locker21

24) The disease that has reduced FEV1/FVC ratio:
a- pneumoconiosis
b- sarcoidosis
c- chronic bronchitis
d- idiopathic pulmonary fibrosis

25) Why are smokers at higher risk of infection than non-smoker?
a- impaired epiglottic reflex
b- impaired mucociliary clearance
c- impaired cough reflex
d- cause aspiration

26) A patient came with Flu-like symptoms and cough and little amount of sputum , no consolidation . The X -ray showed Ground glass opacities under microscope the alveolar sample showed hyalinemembranes. What is the cause?
a- atypical viral pneumonia
b- lung abscess
c- Acute bacterial pneumoni
d- Nosocomial pneumonia.

27) a patient is in a car accident and suffers major trauma in the mediastinum and chest wall that leads to hemothorax. Many major arteries are damaged and the patient is unable to catch his breath. What is the cause of this?
a- Resorption Atelectasis
b- Contraction Atelectasis
c- Compression Atelectasis



24	25	26	27
C	B	A	C

RS Midterm Locker21

28) Patient comes to the clinic and has inflammation in the pharynx, tonsils, and gums with necrotic debris. Gram stains reveal a gram-negative spiral organism and gram-negative long tapering bacilli. What is the disease?

a- Streptococcal tonsillitis
b- Epiglottitis
c- Fever
d- Vincent angina

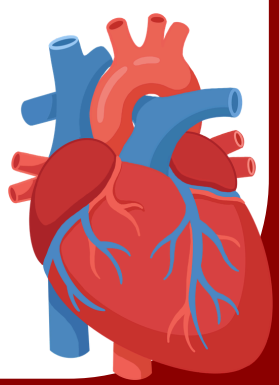
29) What type of emphysema associated with antitrypsin deficiency?

a- Distal Acinar
b- Pan-acinar
c- Centri-acinar
d- Para-septal acinar

30) The capillaries in inter-alveolar septa are:

a- continuous non-fenestrated
b- discontinuous fenestrated
c- Separated from air only by its own epithelium and lamina
d- Fenestrated to allow the air from the alveoli

28	29	30
D	B	A



RS Midterm Locker21

31) A 61male presented to emergency with high convulsions and high productive cough he has chorionic obstructive pulmonary disease ,he smokes 1.5 pack a day for past 35 years he has hypertension and it's Ph:7.2 Pco2:99 Po2:51 what's the most likely diagnosis?

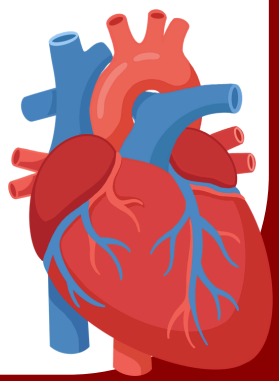
a- Nosocomial, Respiratory alkalosis
b- Hypoxia , Respiratory alkalosis
c- Hypoxia , Respiratory acidosis
d- nosocomial , Respiratory acidosis

32) A 53 year old man under went thyroid surgery and had a hoarseness in his voice due to nerve damage. Which of the following nerves most likely caused this damage?

a- External larnygeal
b- Internal laryngeal
c- Recurrent laryngeal
d- inferior laryngeal

33) A 35 year old diabetic female came in with abdominal pain and vomiting after attending a party. Her blood sugar was 600=mg/dl (normal <140 mg/dl). The values obtained from the ABG were, pH=7.03 (normal 7.35-7.45) PCO2= 36 mm Hg (normal 35-45mm Hg) HCO3-= 14mEq/L (normal 22-26 mEq/L). How will the body compensate?

a- increase H+ to buffer HCO3-
b- hypoventilate
c- decrease HCO3- reabsorption
d- hyperventilate



31	32	33
C	C	D

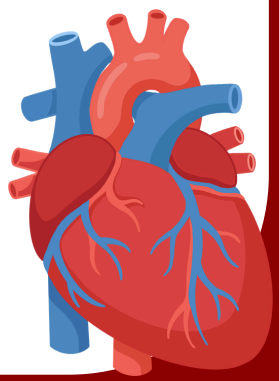
RS Midterm Locker21

34) Which of the following if reduced rate of gas diffusion decreases?
a- molecular size
b- fibrosis
c- surface area of alveoli
d- thickness of alveolar membrane

35) 60y old male go to Alulaa , after 2-3 days he start to complain of runny nose, cough, sore throat, and fever , also its progressed to acute pneumonia and diarrhea . What is the organism that he affected with after contact with camels?
a- SARS
b-MERS-COV
c-Chlamydia pneumonia
d-Staphylococcus aureus

36) patient with hyperventilation which of the following receptor that not activate under normal quit breathing and it's become active to decrease the ventilation?
a- ventral respiratory group
b- dorsal respiratory group
c- Apneustic center
d- Pneumotaxic Center

37) carboxyhemoglobin in blood is mean Hb and:
a- Carbon dioxide
b- oxygen
c- carbon monoxide



34	35	36	37
C	B	A	C

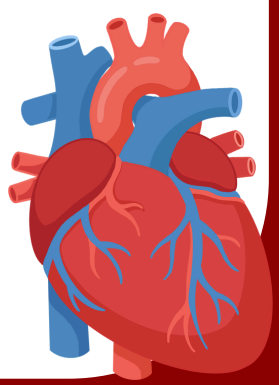
RS Midterm Locker21

38) Patient go trip in alolaa and after two weeks he have upeer right quadrant pain , jaundice, and abdominal mass with lung abscess. During surgery they find liver cyst. What the parasite that he infected with:

a- Echinococcus granulosus
b- Schistosomae
c- Entamoeba histolytica
d- Paragonimus westermani

39) A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing.
The attacks are particularly common in the spring. During an episode of acute respiratory difficulty, a physical examination shows that she is afebrile. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. A sputum sample examined microscopically also has increased numbers of eosinophils. Which of the following histologic features is most likely to characterize the lung in her condition?

a- Remodeling of airways with smooth muscleHyperplasia
b- Patchy areas of consolidation with leukocytic exudates in alveoli
c- Dilation of respiratory bronchioles with loss of elastic fibers
d- Inflammatory destruction of bronchial walls



38	39
A	A

RS Midterm Locker21

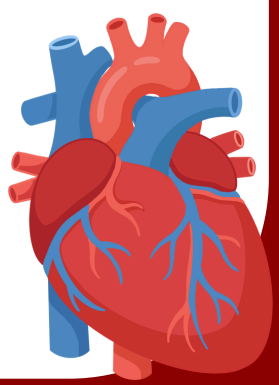
40) 50-year old female patient came with Dry cough , Bilateral hilar lymphadenopathy with Non-caseating granuloma . He had:
a- SARCOIDOSIS
b- PNEUMOCONIOSES
c- Emphysema
d- Asthma

41) which of these structure piercing thyrohyoid membrane:
a- superior laryngeal
b- external laryngeal
c- internal laryngeal
d- recurrent laryngeal

(they don't mention if it's an artery or nerve)

42) main component of surfactant:
a- dipalmitoyl phosphatidyl choline
b- Phosphatidylinositol
c- Phosphatidylglycerol
d- Protein

40	41	42
A	C	A

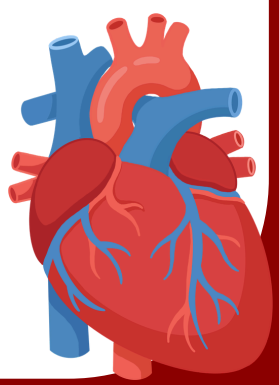


RS Final Locker21

1) which of the following drug affects growth because it's have been taken a lot during childhood?
a- Corticosteroids
b- Ampicillin
c- thyiophyline
d-macrolides

2) A 33-year-old businesswoman presented to the ER with productive sputum, fever after staying in Japan for 4 months she ate raw fish and crab. The doctor sent her abnormal X-ray and laboratory tests to the lab, and it showed an egg, what is the parasite causing her symptoms?
a- entamoeba histolytica
b- Echinococcus granulosus
c- P.westermani
d- ascaris lumbricoides

3) A 64 year old male known case of copd and type 2 respiratory failure, PaCo2 : 95. Placed on mechanical ventailation, which of the following breathing pattern will be seen upon his initaial presentation?
a-irregular RRirregular RR
b-Pursed lips
c-deep breathing (kussmaul breathing
d-Breathing with apnea



1	2	3
A	C	B

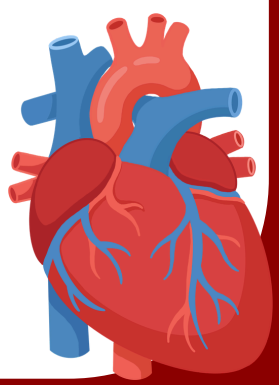
RS Final Locker21

4) which signal is released to limit inflation of lung?
a-Lung stretch receptor
b-Lung irritant receptor
c-arterial baroreceptor
d-atrial receptor

5) Which of the following related to the medial surface of the right lung?
a-Right ventricle
b-Left atrium
c-Arch of azygous vein
d-Descending aorta

6) why arterial blood shows increased oxygen saturation than venous blood?
a-due to increased PaO2 in arterial blood more than venous blood
b-due to increased PaCO2 in arterial more than venous blood
c-due to decreased amount of hemoglobin in venous blood
d-due to increased temperature in arterial blood increase oxygen saturation of
hemoglobin

7) what is the sign of respiratory failure type 1?
a- cynosis
b- flabbing tremor
c- lower limb edema
d- chest pain



4	5	6	7
A	C	A	A

RS Final Locker21

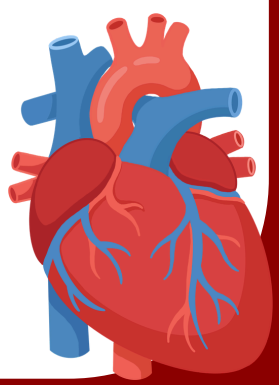
8) Which of the following is immediate of high altitude climbing
a-hyperventilation
b-Polycythemia
c-acidosis
d-increase 2,3 DPG

9) Which of the following is the egg that cause ascaris infection to human by raw vegetables and contaminated water?
a-Fertilized egg
b-unfertilized egg
c-mature egg
d-immature egg

10) female african american has a non-ceasiting granuloma with skin lesion , what type of her erythema?
a-Erythema migrans
b-Erythema multiform
c-Erythema nodosum
d-Erythema toxicum

11) case about HIV patient on microscope they found cup-shaped cysts in silver stain , most common likely organism?
a-Histoplasma
b-Coccioides
c-Pneumocystis jiroveci
d-candiases

8	9	10	11
A	C	C	C



RS Final Locker21

12) 26 year old With history of bronchial asthma maintained by Salbutamol had an asthma attack without response to Salbutamol called the emergency in the situation what should be done?

a-start on Theophylline and to go to the emergency room
b-Discontinue Salbutamol and take Ipratropium
c-Take corticosteroids and go to the emergency room
d-Double the Salbutamol and go to the emergency room

13) Hypothetically if drug X blocks the alveolar cells ATP synthesis what will happen to the diffusion rate?

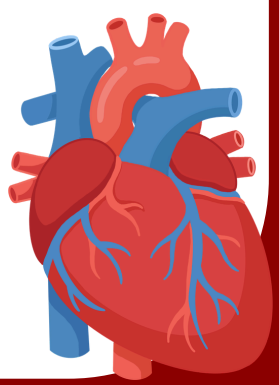
a-It will affect the diffusion on CO2 rather than O2
b-No effect on the diffusion rate
c-Decrease the diffusion rate
d-Increase the diffusion rate

14) Which of the following true regarding central chemoreceptor

a-maximum discharge responding to H in CSF
b-respond directly to increase CO2
c-respond to changes in ph
d-sensitive to hypoxia

15) Patient presented with lower respiratory tract symptoms, he give history of bird keeping, what is the caustive organism?

a-Bordetella
b-Diphtheria
c-SARS
d-Chlamydia psittaci



12	13	14	15
C	B	A	D

RS Final Locker21

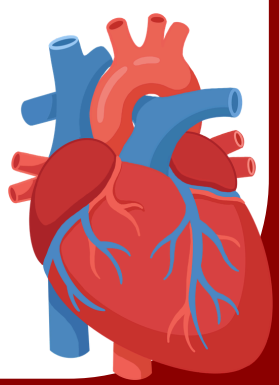
16) patient with stroke result in blocking blood supply from medulla center , which of the following will be the outcome?
a- Immediate cessation of breath
b-apneustic breathing
c-ataxia breathing
d-irregular breathing

17) Which of the following prevent leakge of flood from blood capillaries to Alveoli?
a-repull surfactant force
b-negativity of intrathorsic pressure
c-negativity of intrapleural pressure
d-Hydrostatic pressure

19) which of the following prevent lung collapse during expiration?
a-surfactant
b-increase negativity of intrapleural pressure during inspiration
c-increase negativity of intralviolar pressure during inspiration
d-tranmural pressure

20) regarding the embryonic origin of respiratory system , the epithelial lining it originate from?
a- splenic layer
b- mesoderm
c- ectoderm
d- endoderm

16	17	19	20
A	A	A	D



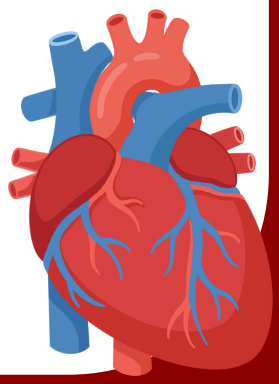
RS Final Locker21

21) most common cause of acute bronchitis?
a- bacterial infection
b-viral infection
c-fungul infection
d-non-infectious cause

22) The percentage of lung cancer, because of smoking?
a-80-90%
b-50-60%
c-10%
d-100%

24) A person arrives to the ER with tachycardia and tachypnea. It was noted that the patient was hypoxic and was given O2 therapy. The patient did not respond to therapy. Which of the following defects could contribute to this?
a- congenital anomalies (AI answer)
b- Histotoxic (testbank answer)
c-high altitude
d- ventilation \ perfusion ration dismatch

25) A patient comes to the clinic and confused, tired, with tachypnea after overdosing on Aspirin. Her blood pH = 7.44 CO2 = 12 mmHg HCO3- = 9. What does this indicate?
a-Metabolic Acidosis
b- Metabolic Alkalosis
c- Respiratory Acidosis
d- Respiratory Alkalosis



21	22	24	25
B	A	?	D

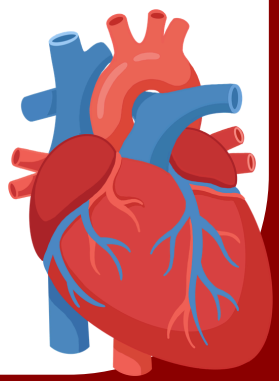
RS Final Locker21

26) why do aspirated bodies go into the right main bronchus?
a- beacuse the right bronchus is more vertical than the left bronchus
b- because the right bronchus is narrower than the left bronchus
c- because the right bronchus is more oblique than the left bronchus
d- because the right bronchus is longer than the left bronchus

28) an athletic goes to gem after training the lactic acid in his body elevate which of the following can be stimulus in response to lactic acid?
a- Central chemoreceptors
b- peripheral chemoreceptors
c- lung irritate receptors
d- arital barreceptors

29) A 6 year old child aspirates a peanut and comes into the ER with shortness of breath, dyspnea, and light headedness. What is the cause of his symptoms?
a-Compression Atelectasis
b- Contraction Atelectasis
c- Resorption Atelectasis

30) patient is taking a drug to control his TB and this drug results in Hyperuricemia. What drug is responsible for this side effect?
a-Isoniazid
b-Rifampin
c- Pyrazinamide
d- Streptomycin



26	28	29	30
A	B	C	C

RS Final Locker21

31) A 35y male,presents with fever and cough.3days ago he suffered the onset of nasal stuffiness, mild sore throat, and productive cough (clear sputum) . Today, he decided to seek physician assistance because of fever (38.3C) and mild bronchospasms . The patient has no history of familial illness, hospitalizations, or trauma. What is the empirical therapy?

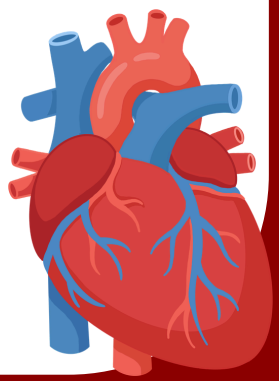
a- gentammicin
b- Levofloxacin
c- macolides
d- amoxicillin ,clavulanic acid

32) A man comes into the hospital with a chronic cough and take anti hypertension drug.(amlodapine). The patient has no history of cardiac problems or chronic Disease. He feels a burning feeling in the anterior aspect of his chest. What could be the cause of his cough?

a- GERD
b- drug induce cough
c- Chronic Bronchitis
d- Asthma

33) A 32 year old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?

a- Levofloxacin
b- Ampicillin with Sulbactam
c- Cefuxime
d- Penicillin



31	32	33
D	A	A

RS Final Locker21

34) The percentage of oxygenated Hb to total Hb?
a- PaO2
b- dissolve oxygen
c- oxygen saturation

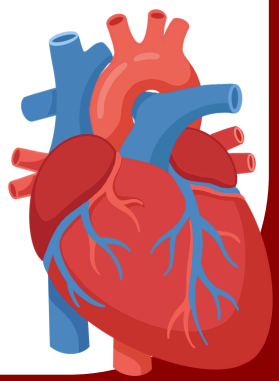
35) What is the mechanism of the hemoglobin buffer system?
a-chloride shift
b- Neutralize excess acidity by imadozole histidine ring
c- Neutralize excess alkalinity by imadozole histidine ring

36) A baby born at 37 weeks has a good respiratory function, which of the following can indicate that?
a-L/S ratio < 2
b-surfactant contains phosphatidylglycerol.
c- surfactant contains shpeniogel
d- L\s ratio =1

37) Larval form of ecchonococcus granuloses present in which host
a-Dog
b- wolf
c- man
d-sheep

38) Patient presented with 3 weeks of cough, dyspnea, CXR revealed bilateral zone patch consolidation, esoinphilia and ascaris larva in sputum, what is this patient have
a- Katamya fever
b- Liver abcess
c-lung cysct
d- Loffler syndrome

34	35	36	37	38
C	B	B	C	D



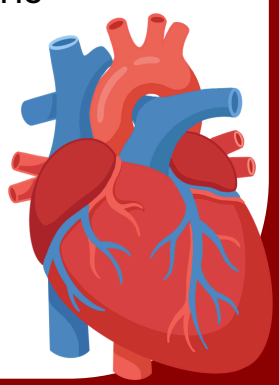
RS Final Locker21

39) 4yrs child presented with fever, runny nose and inspiratory whoop follows bout of coughing, what is the drug of choice ?
a-Erythromycin
b-Ciprofloxcin
c- Cephalosporin

40) Patient presented with cough, dyspnea, chest pain, CXR/investigation revealed small amount of pleural effusion and abscess in lung, what is the most likely caustive parasite?
a- Entoambea histolytica (Testbank answer)
b- Ecchonococus granuloses
c- Paragoniumas westermani (AI answer)
d- Schistosomiasis

41) cause of capillary don't infiltrate fluid to alveoli?
a- Hydrostatic pressure
b- Surfactant
c- Intra-pulmonary pressure
d- Intra-thoracic pressure

42) An arterial pressure has measured in a patient resulted with PaO2 = 80 mmHg, SaO2 = 80% and CaO2 = 20 ml O2 /dl, what does that meaning?
a- Oxygenated hemoglobin is 80 mmHg
b- O2 in dissolved plasma is 20 ml O2 /dl
c- O2 that bind to hemoglobin and dissolved in plasma equals 20 ml O2 /dl
d- Deoxygenated hemoglobin equals 80% of all hemoglobins



39	40	41	42
A	A	B	C

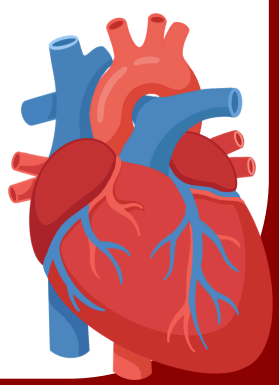
RS Final Locker21

43) Which of the following is characteristic histologic feature in atypical pneumonia?
a- Alveolar spaces are filled with fibrin and blood
b- Alveolar spaces are clear from inflammation
c- Alveolar septa are clear from inflammation
d- Alveolar spaces are filled with neutrophils

44) A 6 year old child aspirates a peanut and comes into the ER with shortness of breath.after 3 days he had a fever and diarrhea. What is the complication that may happen to him?
a- bronchopneumonia (AI answer)
b- bronchiectasis
c-resorption (testbank)
d-acute bronchitis

45) A 25 year old male presents to the medical OPD with complaint of cough and low grade fever since the last 3 months. Laboratory tests show sputum positive for acid fast bacteria. Which of the following combination of drugs is most likely to be administered at the start to this patient?
a- Pyrazinamide, Ethambutol and Isoniazid
b- Streptomycin, Isoniazid, Rifampicin and Pyrazinamide
c- PAS, Pyrazinamide and Rifampicin
d- Streptomycin and Rifampicin

43	44	45
B	C	B



RS Final Locker21

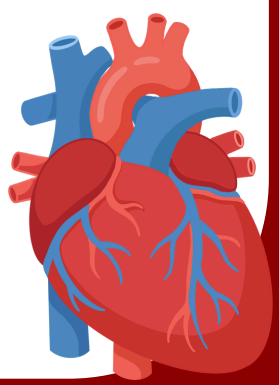
46) What is the function of respiratory epithelium?
a- transport of the oxygen from capillary to alveolai
b- Air trapping the particle and clear it
c- warming the air

47) the most common congenital anomalies in development of lung
a- esophageal atresia
b-hypoplasia
c-agenesis
d- Tracheo-esophageal Fistula

48) a smoker came to clinic with tumor in central lung tissue , under microscope small round, cells with scanty cytoplasm ,finely granular chromatin (salt & pepper) . what is the correct about this tumor?
a- neuroendocrine origin
b- late local invasion.
c- easy to prognosis
d- takes time to metastasis lymph node at time of diagnosis.

49) when you used the maximum adeno triphosphate in respiration?
a-tidal inspiration
b-tidal expiration
c-forced inspiration

46	47	48	49
B	D	A	C

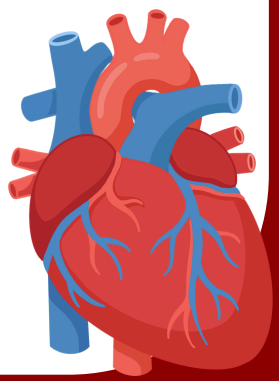


RS Final Locker21

50)An 18-year-old man presents to the emergency department after a stab wound to the chest. He is combative and with severe shortness of breath. What will happen to him?
a- Intrapleural pressure increased with possible lung collapse (AI answer)
b- no healing
c- increase of intrathoracic pressure
d- Interalveolar pressure increased with possible lung collapse (Testbank answer)

51) a female came to ER with insomnia and seizors after take an overdose of direct bronchodilatation drug which used orally usually for asthma attacks ?
a- Ipratropium
b- Prednisone
c- Theophylline
d-cromolyn

52) a 24 year old women came to ER with SOB , the result of ABG shows ph=7.27, pco2= 60, po2= 65, HCO3=26, base excess +1 . what is the diagnosis?
a- uncompensated respiratory acidosis
b- uncompensated respiratory alkalosis
c- compensated respiratory acidosis
d- uncompensated metabolic acidosis



50	51	52
D	C	A

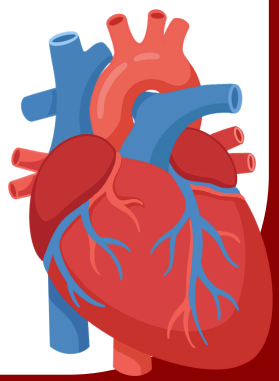
RS Final Locker21

53) after taking a biopsy of patient undergo attack lasts for weeks without response to treatment followed by hypercapnia and death after few days . the microscope show
a- Asteroid bodies
b- hypertrophy of smooth muscle
c-fibrosis

54) a middle age men present to clinic with chronic bronchitis. after the spirometry result show
a- increased FEV1/ FVC AND increased TLC
b- Decreased FEV1/ FVC AND decreased TLC
c- Decreased FEV1/ FVC AND increased TLC
d- increased FEV1/ FVC AND normal TLC

55)A 45 obesity male came to ER with chest pain and sob . after measure the vital sign his pressure was high which significant contributor to the development of cor-pulmonale.the cause
a-Recurrent thromboemboli
b- Congenital or acquired heart disease
c- interstitial lung diseases
d- Obstructive sleep apnea

56) a 24 year old women came to ER with SOB , the result of ABG shows ph=7.31, pco2= 50, po2= 65, HCO3=26, base excess >2 .
a- HCO3 + base uncompensated respiratory acidosis
b- HCO3 + base compensated respiratory acidosis
c- the HCO3 + base indicated metabolic acidosis



53	54	55	56
B	C	D	A

RS Final Locker21

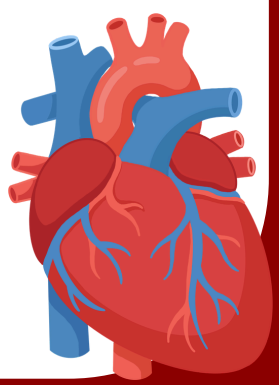
57) why venous blood is more acidic than arterial blood?
a- Deoxygenated hemoglobin have more affinity to H
b-no have carbonmyl enzyme
c- oxygenated hemoglobin have more affinity to H
d-less oxygen

58) patient smoking what will happen that make the diffusion hard?
a- iron deficiency anemia
b- increasing thickness of respiratory membrane
c- increase red blood cell

59) excerbation of chronic bronchitis and take oxygen but he need drug what is initial therapy?
a- nebulization albutamol 2.5mg every 20 second
b- nebulization Ipratropium 0.5mg every 20 second
c- inhaled corticosteroids
d- Antibiotics

60) a 57 patient come to ER with pulmonary hypertension. what is the changes in the artery
a- fibrous thickness in the wall
b- medial hypertrophy
c- widening of the artery

61) a long case with patient have a productive cough with purulent septum in situs versus in the respiratory epithelium
a- bronchectasis
b- emphysema
c- atelectasis



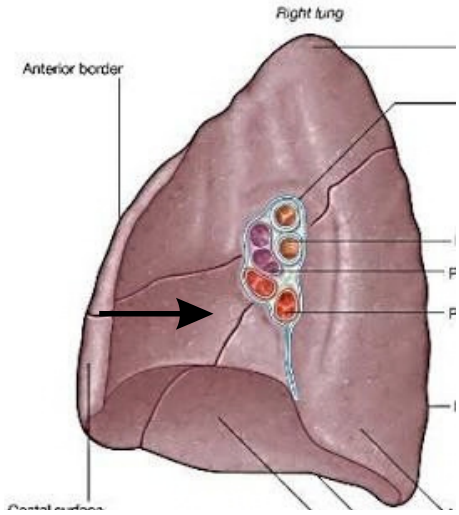
57	58	59	60	61
A	B	B	B	A

RS Final Locker21

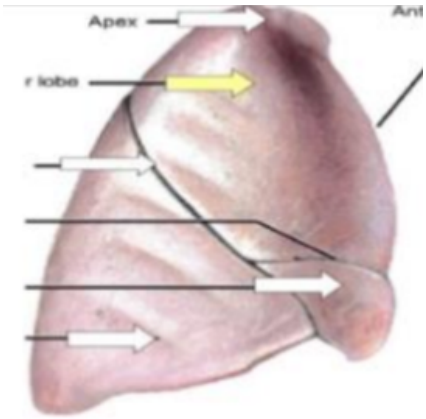
62) patient have a cough and wheezing. he take inhaling corticosteroid what is the instruction that he should followed
a- Rinsing the mouth after inhalation with water

OSPE:

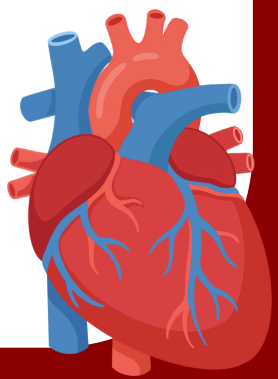
63)this groove for
a-trachea
b-SVC
c-azygous
d-ascending aorta



64)How many bronchosegmental lobe (yellow arrow)
a- 3
b-4
c-5
d-2

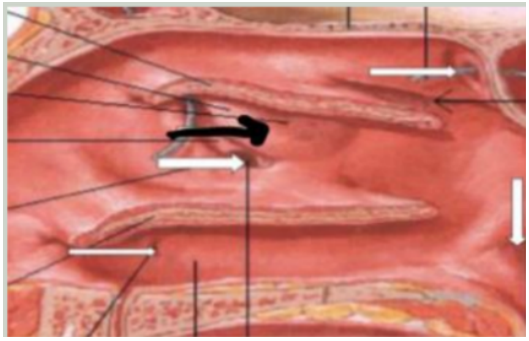


62	63	64
A	B	A

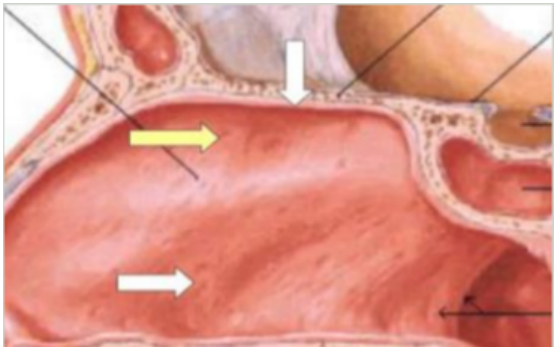


RS Final Locker21

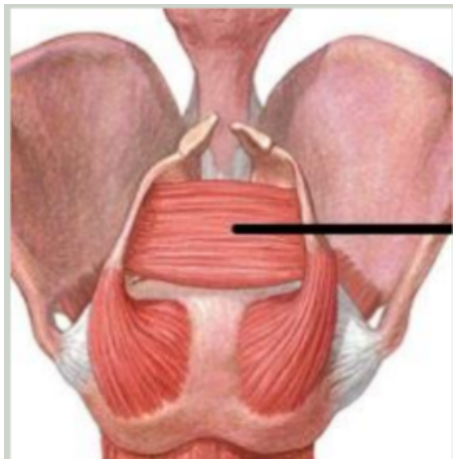
65) opining sinus
a-middle ethmoidal
b-maxillary ethmoidal
c-posterior ethmoidal
d-nasal lacrimal sinus



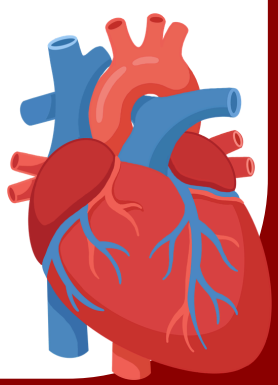
66) For special sensation
a- optic nerve
b- recurrent larengeal
c- facial nerve
d- olfactory nerve



67) What the action of this muscle on the vocal cords?
a- Abduction
b- Adduction
c- Tension
d- Relaxing

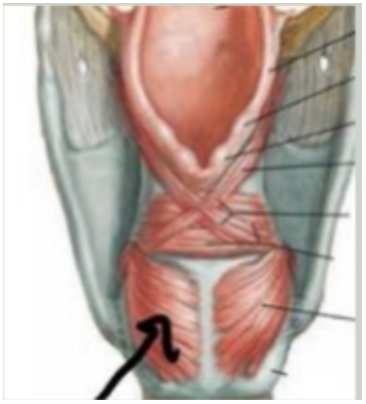


65	66	67
A	D	B

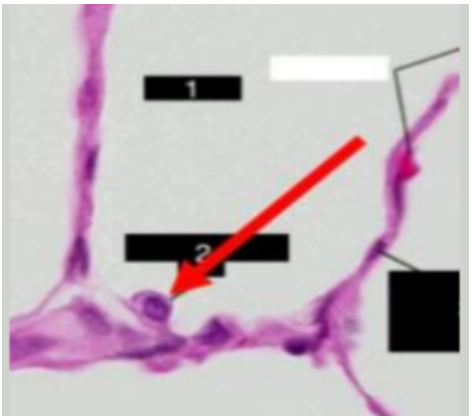


RS Final Locker21

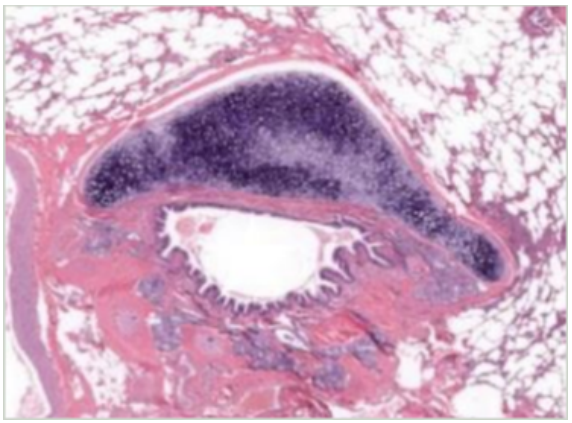
68) nerve supply
a- recurrent laryngeal
b- external laryngeal
c-internal laryngeal



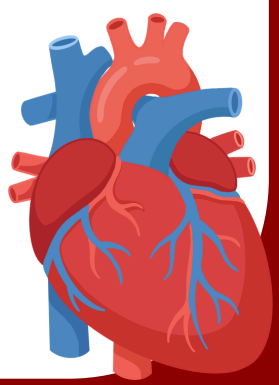
69)identity the cell type
a- type 1 alveolar
b-clara cell
c- macrophages
d- type 2 alveolar cell



70) the structure in this picture lining
a-ciliated simple columnar epithelium
b-ciliated pseudostratified columnar with goblet cells
c-ciliated pseudostratified columnar no goblet cells
d-simple squamous epithelium

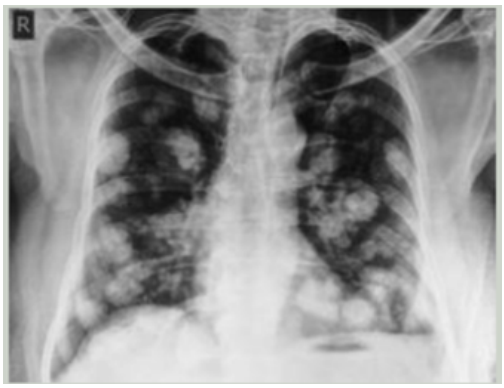


68	69	70
A	D	B

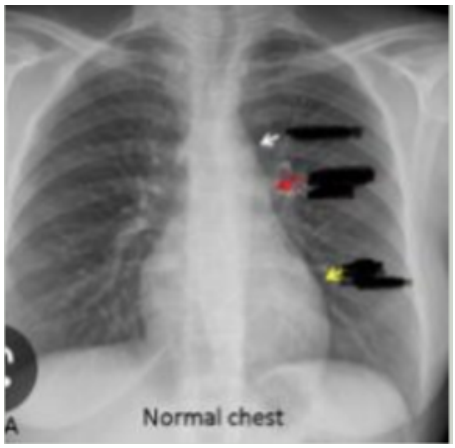


RS Final Locker21

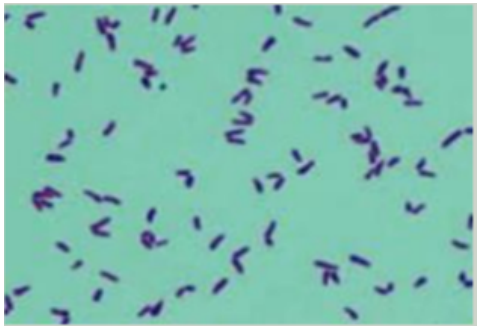
71) This x-ray shows ?
a- Millary TB
b- Cannon ball metastasis
c- Cavities

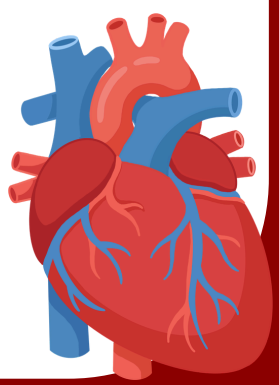


72) what is the structure (red narrow)
a-pulmonary artery
b-pulmonary vein
c-bronchial artery
d-left atrium



73) The organism causing infection shown in the slide:
a-streptococcus
b-clostridium diphtheria
c-strept pyogenes
d-Haemophilus influenzae



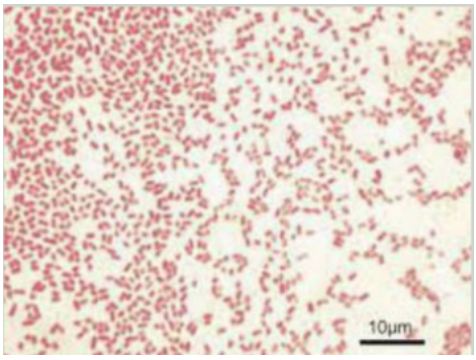


71	72	73
B	A	B

RS Final Locker21

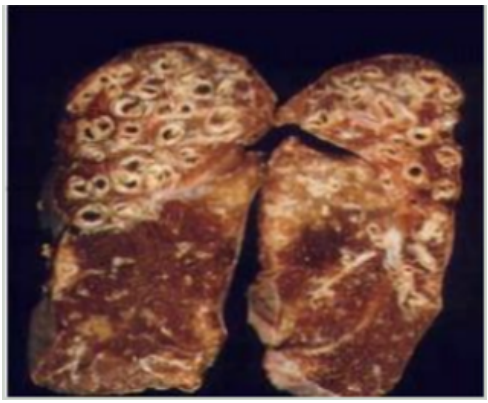
74) The organism causing infection shown in the slide:

a-streptococcus
b-clostridium diphtheria
c-strept pyogenes
d-Haemophilus influenzae



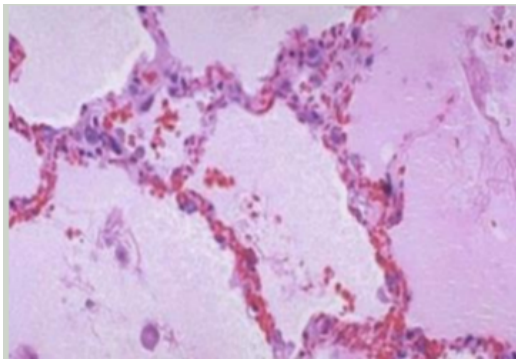
75) the cause of this picture is

a-localized bronchiectasis due to aspirated foreign bodies
b-inflammation can result in scarring
c-cystic fibrosis

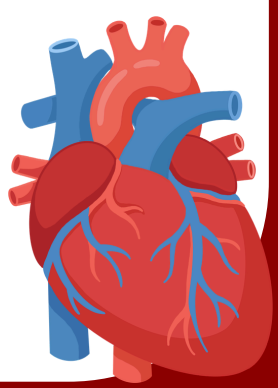


76) in this stage of lobar pneumonia

a-intra-alveolar fluid with numerous bacteria.
b-alveolar exudates composed mainly of neutrophils
c-the septal dilated and filled with RBCs
d-persistence of fibrinosuppurative exudate



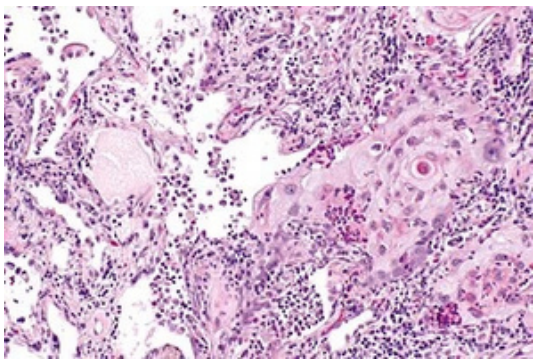
74	75	76
D	C	A



RS Final Locker21

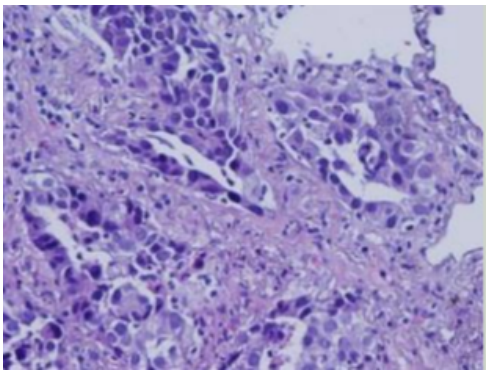
77) A 50 year old smoker patient complained of cough, shortness of breath, and weight loss. His radiological findings showed a centrally located irregular mass that prompted biopsy. Which of the following describes his histological findings ?

a-diffuse sheets of lymphocytes and histocytes
b- sheets of small round blue cells
c- infiltrate of malignant squamous cells and keratin pearls
d-infiltrate of malignant glands



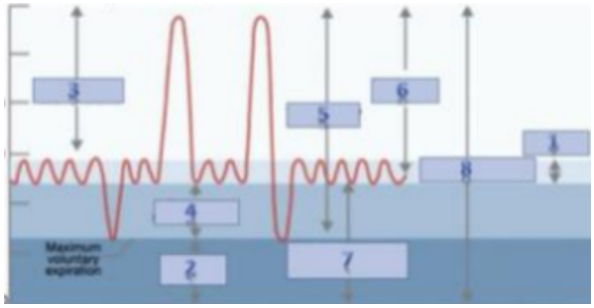
78) what is correct about this picture

a- usually non-smoker patient
b- present in central of the lung
c-arise from mesothelium cells

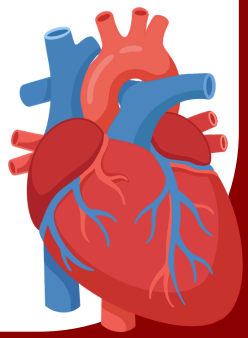


79)The following marked segment of spirogram represents(5):

a-Functional residual capacity
b-Expiratory reserve volume
c-Inspiratory capacity
d-Inspiratory reserve volume



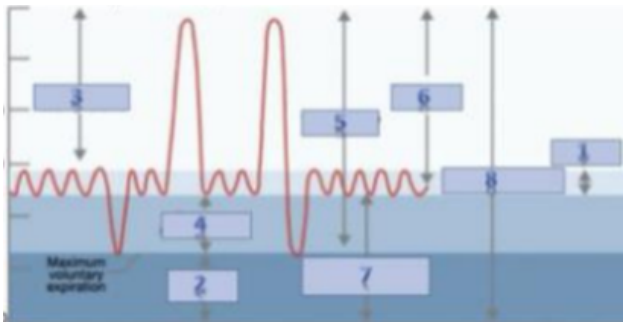
77	78	79
C	A	A



RS Final Locker21

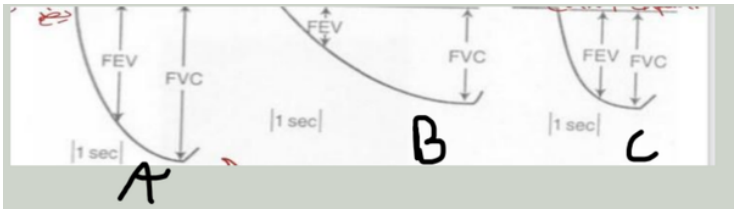
80) The following marked segment of spirogram represents(6):

a-Functional residual capacity
b-Expiratory reserve volume
c-Inspiratory capacity
d-Inspiratory reserve volume



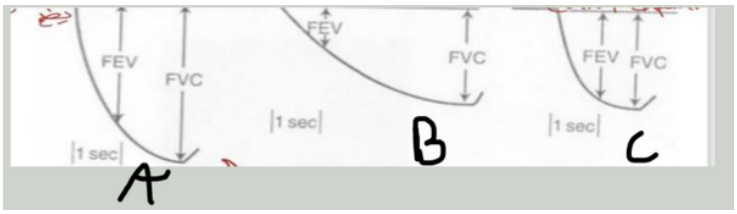
81) the letter A represent?

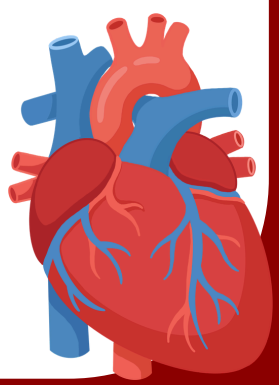
a- normal
b- obstructive
c-restrictive



82) the letter C represent?

a- normal
b- obstructive
c-restrictive





80	81	82
C	A	C

NetflixMidterm

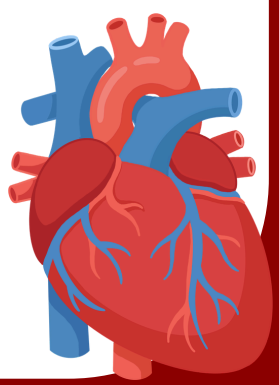
1- Which of the following factors causes increased alveolar PCO2?
A) Increased respiratory membrane thickness
B) Increased oxygen concentration
C) Severe bronchoconstriction
D) Decreased lung compliance

2- Which of the following antibiotics is impaired with calcium?
A) tetracycline
B) Macrolides
C) Amoxicillin
D) Cephalosporin

3- What is related to the lower medial surface of the left lung?
A) Azygos arch
B) Left atrium
C) Esophagus
D) Trachea

4- How is croup transmitted?
A) Blood
B) Aerosol
C) Contact
D) Food

1	2	3	4
C	A	C	B



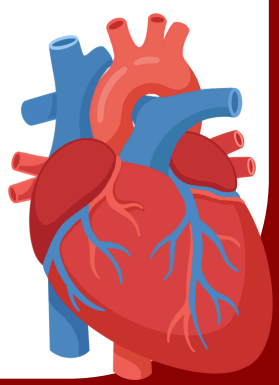
NetflixMidterm

5- What is the virulence factor of Corynebacterium diphtheriae?
A) Enterotoxin
B) Exotoxin
C) Enzyme
D) Capsule

6- (case) a patient came with fever, pharyngeal edema, bull neck appearance, and gray membrane coating the tonsils. What is the organism?
A) Corynebacterium diphtheriae
B) Streptococcal tonsillitis
C) Staphylococcal aureus
D) Streptococcus pneumonia

7- (case) a patient came with inflammation of the pharynx, tonsils, and gums and offensive halitosis caused by borrelia vincentii and fusobacterium. What is the diagnosis?
A) Vincent's Angina
B) Epiglottitis
C) Croup
D) Viral pharyngitis

5	6	7
B	A	A



NetflixMidterm

8- (case) a patient came with high fever, purulent exudates, and enlarged cervical lymph node. What is the immunological complication of this disease?

A) Rheumatic fever
B) Scarlet fever
C) Septicemia
D) Glandular fever

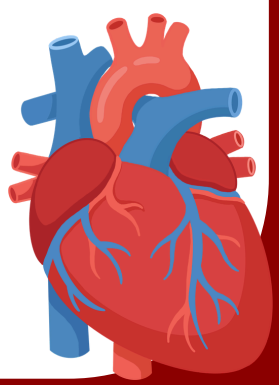
9- A 75 years old patient with a history of diabetes and chronic heart disease suffered from severe pneumonia .What is the first line drug used for empirical treatment ?

A) Ceftriaxone
B) Ciprofloxacin
C) Amoxicillin
D) Azithromycin

10- Which of the following bones is present in the nasal septum?

A) Frontal
B) Palatine
C) Vomer
D) Nasal

8	9	10
A	A	C



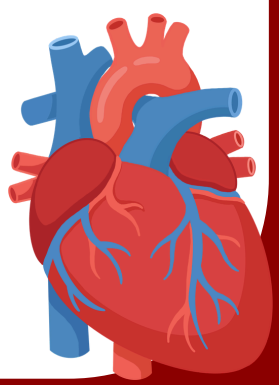
NetflixMidterm

11- Which of the following paranasal sinuses open in the sphenoethmoidal recess?
A) Sphenoid
B) Frontal
C) Maxillary
D) Posterior ethmoidal

12- During surfactant synthesis, which of the following is found extra-cellular in alveolar lumen and repair SP-A & SP-B for its formation and stabilization?
A) DPPC
B) Type II alveolar cells
C) Lamellar body
D) Tubular myelin

13- A 53 year old patient has difficulty breathing through nose. On examination, his physician finds that he has swelling of mucous membrane of superior nasal meatus. Which of the following paranasal sinuses is affected?
A) Middle ethmoidal
B) Posterior ethmoidal
C) Frontal
D) Maxillary

11	12	13
A	D	B



NetflixMidterm

14- During the lung maturity test a pregnant woman's L/S ratio is determined to be 1:1, what can we conclude from this result?

A) decreased risk of respiratory distress syndrome
B) No specific significance is concluded
C) Insufficient production of surfactant
D) Adequate lung maturity

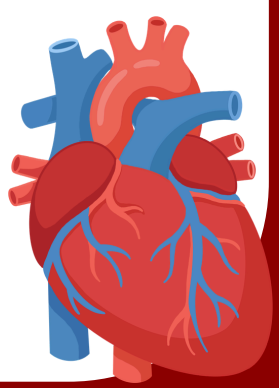
15- What is the role of surfactant in breathing in a healthy subject?

A) enhance lung immunity
B) Prevent alveolar collapse
C) Facilitate gas diffusion
D) Reduce lung expansion

16- (case) a patient with a benign thyroid tumor underwent thyroidectomy then he lost the ability to abduct his vocal cords . Which of the following muscles is affected ?

A) Posterior Cricoarytenoid
B) Lateral Cricoarytenoid
C) Cricothyroid
D) Transverse arytenoid

14	15	16
C	B	A



NetflixMidterm

17- A 26 year old pregnant woman came in with high fever and scanty sputum ,CXR showed diffuse bilateral infiltration. Which drug is recommended?

A) Levofloxacin
B) Azithromycin
C) Doxycycline
D) Cephalexin

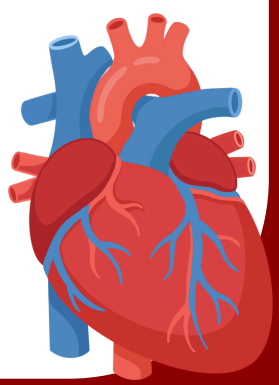
18- Which of the following can lead to cor pulmonale?

A) schistosomiasis
B) T oxocariasis
C) Echinococcosis
D) Ascariasis

19- A Patient came to the ER with a cough and fever, x-rays showed elevated right diaphragm and lower right pleural effusion and abscess. What is the causative agent?

A) Entamoeba histolytica
B) Ascariasis
C) Hydatid cyst
D) T oxocariasis

17	18	19
B	A	A



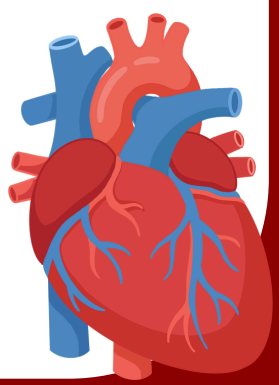
NetflixMidterm

20- A patient came in with skin flushed, they found a ruptured hepatic cyst, what is the causative agent?
A) echinococcus
B) Ascariasis
C) Toxocariasis
D) Entamoeba histolytica

21- What is the preventative measure for echinococcus?
A) Removal of snails
B) Avoid contamination of hands, food and water with dog feces
C) Wear preventive clothes (gloves - shoes)
D) Proper washing of raw vegetables

22- A 6y/o boy played with dogs ina school trip, later he came in with chest tightness,and bronchospasm , what is the possible organism?
A) Toxocara
B) Schistosoma
C) Ascaris
D) Ancylostoma

20	21	22
A	B	A



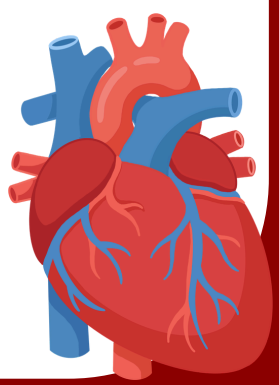
NetflixMidterm

23- Which of the following have simple ciliated columnar epithelium and is devoid of cartilage and glands?
A) alveolar ducts
B) Bronchioles
C) 2ry bronchus
D) External bronchi

24- The lymph nodes present at the carina of the trachea are?
A) bronchopulmonary
B) Tracheobronchial
C) Paratracheal
D) Pulmonary

25- A patient came in from a car accident and had a crush injury, he is presented with shallow rapid breathing,and chest pain .What is the most likely situation?
A) Increased intrapleural pressure and right lung collapse
B) Increased intrapleural pressure and no effect on the right lung
C) Increased intra alveolar pressure
D) Overinflation of the lung

23	24	25
B	B	B



NetflixMidterm

26- What prevents fluid from moving from the pulmonary capillaries into the alveoli?

A) The repelling forces of the surfactant
B) Negative intrapleural pressure
C) Negative transthoracic pressure
D) Pulmonary Hydrostatic pressure

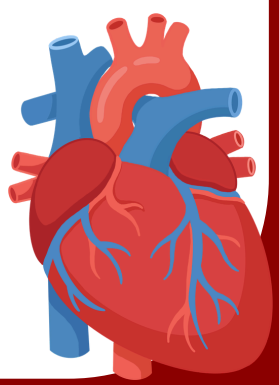
27- A baby is born at 37 weeks with normal pulmonary function, a pulmonary test was taken, what is the likely finding?

A) Phosphatidylglycerol is going to make up 90% of surfactant
B) Presence of Phosphatidylglycerol
C) L/S ratio is below 2
D) Sphingomyelin is the prominent surfactant

28- Which muscle is responsible for widening the laryngeal inlet ?

A) Transverse Arytenoid
B) Thyroepiglottic
C) Oblique arytenoid
D) Aryepiglottic

26	27	28
A	B	B



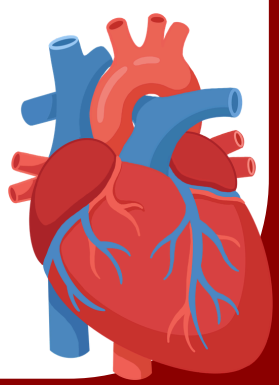
NetflixMidterm

29- (case) A smoker came to the clinic with dyspnea and chronic productive cough that lasted 3 months over the last 2 years. What is true about the laboratory findings?
A) reduced FEV1/FVC ratio
B) Normal FEV1/FVC ratio
C) Decreased total lung capacity
D) Reduced FVC

30- Regarding the bronchopulmonary segment, which of the following is intersegmental?
A) Bronchial artery
B) Segmental bronchi
C) Tributary pulmonary vein
D) Pulmonary artery

31- (case) A child came into the ER, after aspiring a strawberry that has lodged into his right
bronchi, what is the likely result?
A) Contraction atelectasis
B) Constrictive atelectasis
C) Resorption atelectasis
D) Pneumothorax

29	30	31
A	C	C



NetflixMidterm

32- Which of the following will most likely result in contraction atelectasis?

A) Pleural fibrosis
B) Pneumothorax
C) Bronchial asthma
D) Bronchiectasis

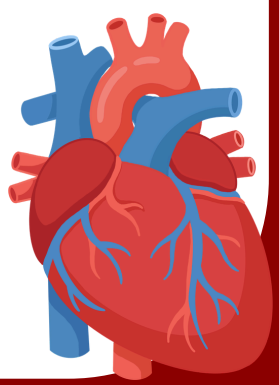
33- The epithelium of the respiratory tract comes from which of the following?

A) Endoderm
B) Ectoderm
C) Splanchnic mesoderm
D) Somatic mesoderm

34- increased acidity (PH) in blood is likely to have which of the following effect?

A) Decreased oxygen delivery to that tissue
B) Increased affinity of HB to oxygen
C) Increased oxygen delivery to that tissue
D) Decreased affinity of HB to Co2

32	33	34
A	A	C



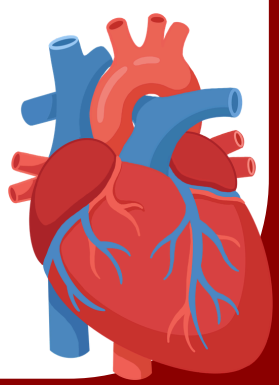
NetflixMidterm

35- A young adult came in with community acquired pneumonia, they have no history of illness, they described a history of resistance to penicillin, what is the best drug to give them?
A) Levofloxacin
B) Cefuramine
C) Ceftriaxone
D) Ampicillin

36- Which of the following limits inspiration?
A) Ventral respiratory group
B) Lung stretch receptors
C) Irritant receptor

37- Which sensory nerve supplies the area above the vocal cords?
a) Internal laryngeal
b) recurrent laryngeal
c) External laryngeal
d) Glossopharyngeal

38- (case) A patient came in with interstitial fibrosis. What may be the result of the pulmonary function test?
A) FVC reduced
B) TLC normal
C) FEV1/FVC decreased
D) TLC increased



35	36	37	38
A	B	A	A

NetflixMidterm

39- (case) A 24-year old male was playing American football and went head-to-head with another player. He came to the ER with cyanosis and no respiration and had to be on ventilation . Which of the following spinal level is mostly affected?

A) C1
B) C5
C) C8
D) T2

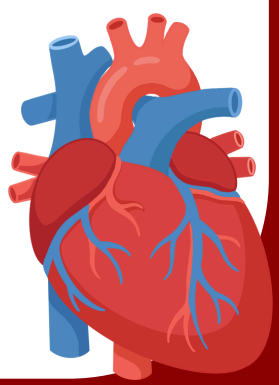
40- which indicates the Percentage of oxygenated Hb on total hemoglobin in plasma?

A) hematocrit
B) oxygen saturation
C) oxygen content
D) partial pressure O2

41- (case) a man was stabbed in the right 5th intercostal space at the midaxillary level. X-ray showed air in the pleura. what's the most probable complication of the underlying case?

A) atelectasis
B) Pneumonia
C) Congestive heart failure
D) Pneumoconiosis

39	40	41
A	B	A



NetflixMidterm

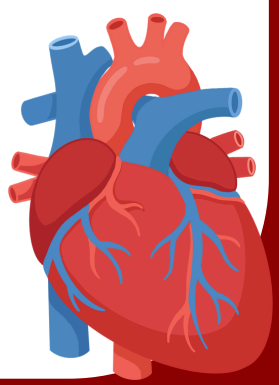
42- Which of the following is the main source of CO2 production ?
A) incomplete oxidation of CHO and Fats
B) oxidative metabolism
C) oxidation of sulfa containing amino acids
D) Anaerobic respiration of glucose

43- Which of the following initiates the tidal expiration?
A- elastic recoil of the lung
B- intercostal muscle contraction
C - contraction of the diaphragm
D- forced blowing to excrete air from lung

44- Which drug binds to penicillin binding protein ?
A) macrolides
B) T etracycline
C) Beta lactam
D) Fluoroquinolones

45- Which drug is given to overcome the resistance of beta lactamases producing bacteria?
A) ciprofloxacin
B) Clavulanic acid
C) Doxycycline
D) Pipercellin

42	43	44	45
B	A	C	B



Mid respiratory

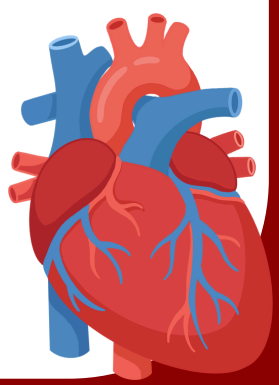
1) In an arterial blood gas (ABG) sample, which of the following is used in the syringe?
a) Heparin
b) Warfarin
c) Coagulase
d) Potassium

2) A 45-year-old male with a past history of smoking complains of fever, mild diarrhea, and a nonproductive cough. Gram staining of sputum shows no microorganisms. A urine test was performed, and a bacterial antigen was found. The MOST recommended treatment for this infection is expected to be:
A) Macrolide
B) Penicillin
C) Aminoglycoside
D) Tetracycline

3) Nerve supply to mediastinal pleura:
a) sympathetic
b) phrenic
c) broncho something i dont remember
d) intercostal

4) A 20-year-old girl ate green salad and, weeks later, developed a cough, dyspnea, and asthma. What is the most likely parasite?
A) Ascaris lumbricoides
B) Ancylostoma duodenale
C) Toxocara species
D) Enterobius vermicularis

1	2	3	4
A	A	B	A



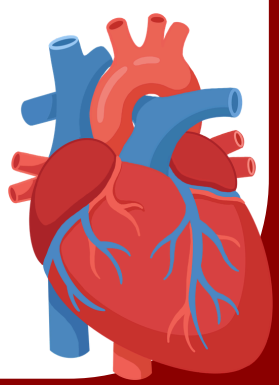
Mid respiratory

5) Nerve supply above the vocal cords?
a) internal laryngeal
b) Recurrent laryngeal
c) External laryngeal
d) Glossopharyngeal

6) Which of the following nerves pierces the thyrohyoid membrane?
A) Recurrent laryngeal nerve
B) External laryngeal nerve
C) Glossopharyngeal nerve
D) Internal laryngeal nerve

7) What is the first-line treatment for streptococcal pharyngitis?
A) Ceftriaxone
B) Ampicillin
C) Penicillin
D) Metronidazole

8) A 53 year old patient has difficulty breathing through nose. On examination, his physician finds that he has swelling of the mucous membrane of the superior nasal meatus. Which of the following paranasal sinuses is affected?
A)Middle ethmoidal
B)Posterior ethmoidal
C)Frontal
D)Maxillary



5	6	7	8
A	D	C	B

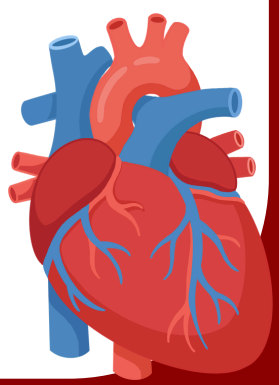
Mid respiratory

9) Which of the following bones is present in the nasal septum?
A) Frontal
B) Palatine
C) Vomer
D) Nasal

10) Which of the following muscles widens the laryngeal inlet?
A) Thyro-epiglottic muscle
B) Oblique arytenoid muscle
C) Aryepiglottic muscle
D) Lateral cricoarytenoid muscle

11) Horizontal fissure pierces the anterior border of the lung at;
A)Fifth
B)forth
C)sixth
D) third

12) Which of the following structures form impressions on the lower medial side of left lungs?
A)Esophagus
B)Superior vena cava
C)azygos arch
D) left atrium



9	10	11	12
C	A	B	A

Mid respiratory

13) In a project representing lung ventilation, where the lung is depicted as an elastic material, the diaphragm as a dome-shaped rubber sheet, and the pleura as a balloon, what happens during inspiration?

A) The diaphragm moves upward
B) The thoracic volume decreases
C) The rubber sheet stretches and flattens
D) The intrapulmonary pressure increases

14) A woman undergoes abdominal surgery and returns two weeks later with shortness of breath and chest pain. Her left leg is swollen, and the overlying skin is reddish-blue. What is the physiological compensatory response?

A) Contraction of local pulmonary arterioles
B) Relaxation of local arteriolar smooth muscle
C) Relaxation of bronchiolar smooth muscle
D) Decrease in airway resistance

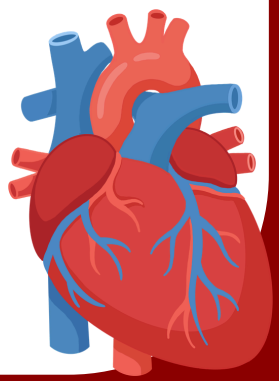
15) Which parasite penetrates the skin and causes infection through the filariform larval stage?

A) Taenia saginata
B) Ancylostoma duodenale
C) Enterobius vermicularis
D) Ascaris lumbricoides

16) A patient with a severe urinary tract infection is prescribed amoxicillin. Given the pharmacokinetics of this drug, which of the following should the doctor be aware of regarding the patient's condition?

A) History of penicillin allergy
B) Renal impairment
C) Increased hepatic metabolism
D) Increased gastric acid secretion

13	14	15	16
C	A	B	B



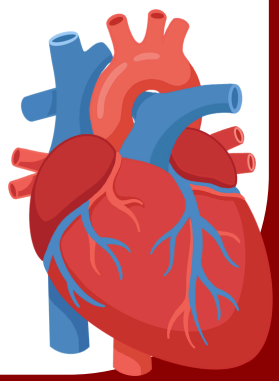
Mid respiratory

17) Which effect describes how an increase in CO_2 and acidity lowers the affinity of hemoglobin for oxygen?
A) Haldane effect
B) Bohr effect
C) Hering–Breuer effect
D) Hamburger phenomenon

18) Coxiella burnetii can cause which type of fever?
A) Q fever
B) Scarlet fever
C) Rheumatic fever
D) Typhoid fever

19) Cellular respiration is:
A) The transport of O_2 in the blood circulation
B) The exchange of O_2 and CO_2 at the tissue level
C) The movement of air into and out of the lungs
D) The use of O_2 by mitochondria to produce energy from food metabolism

20) CO2 is mainly transported in the blood in the form of:
A- Dissolved in plasma
B- Bind to proteins
C- Carboxy-Hb
D- Bicarbonate



17	18	19	20
B	A	D	D

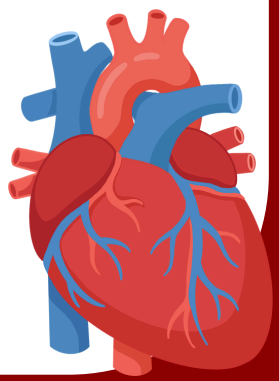
Mid respiratory

21) Which amino acid in hemoglobin helps buffer blood through its imidazole ring?
A) Glycine
B) Histidine
C) Alanine
D) Valine

22) Identification of eggs in stool can be beneficial in diagnosing which of the following diseases?
A) Pulmonary amoebiasis
B) Liver amoebiasis
C) Intestinal amoebiasis
D) Hydatid cyst disease

23) Which property of pulmonary surfactant helps prevent pulmonary edema?
A) Increasing alveolar surface tension
B) Repelling force of surfactant
C) Increasing pulmonary capillary pressure
D) Increasing alveolar membrane permeability

24) What are the main components of pulmonary surfactant?
A) Proteins and phospholipids
B) Proteins and carbohydrates
C) Nucleic acids and proteins
D) Cholesterol and nucleic acids



21	22	23	24
B	C	B	A

Mid respiratory

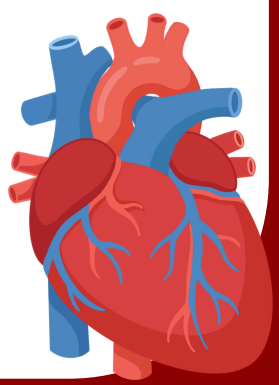
25) A 3-year-old child presents with a barking cough. Which of the following organisms is the most likely cause?
A) Parainfluenza virus
B) Respiratory syncytial virus (RSV)
C) Parvovirus
D) Influenza virus

26) A pregnant woman is expected to deliver her baby prematurely. How do corticosteroids help in the development of the infant's lungs?
A) By promoting the production of surfactant
B) By decreasing surfactant production
C) By inhibiting the maturation of type II pneumocytes
D) By increasing alveolar surface tension

27) The epithelium of the respiratory tract comes from which of the following?
A. Endoderm
B. Ectoderm
C. Splanchnic mesoderm
D. Somatic mesoderm

28) During thyroidectomy, a surgeon accidentally damages a nerve supplying a muscle that opens the vocal cords. Which muscle is affected?
A) Lateral cricoarytenoid
B) Cricothyroid
C) Posterior cricoarytenoid
D) Thyroarytenoid

25	26	27	28
A	A	A	C



Mid respiratory

29) A 60-year-old male with a history of heavy smoking presents with persistent cough and shortness of breath for 2 months. Chest X-ray shows a right upper lobe mass, and the surrounding lung appears collapsed. The patient denies fever or recent respiratory infections. What is the most likely type of atelectasis?

A) Compression atelectasis
B) Resorption atelectasis
C) Constrictive atelectasis
D) Adhesive atelectasis

30) What makes hemoglobin an effective carrier of oxygen in the body?

A) It binds reversibly to O_2
B) It has a higher affinity for CO_2 and does not significantly affect pH
C) It binds oxygen irreversibly
D) It primarily transports carbon dioxide

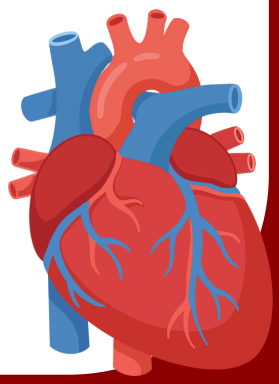
31) Which of the following is an adverse effect of doxycycline?

A) Respiratory depression
B) Arrhythmias
C) Gastrointestinal disturbances
D) Photosensitivity

32) A patient with renal failure is prescribed levofloxacin. Considering the drug's pharmacokinetics, what adjustment should be made to the treatment plan?

A) Change the drug
B) Increase the frequency of the dosage
C) Increase the dosage
D) Decrease the frequency of the dosage

29	30	31	32
B	A	D	D



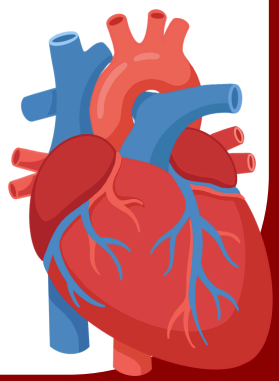
Mid respiratory

33) Which cell type is primarily responsible for gas diffusion in the alveoli?
A) Type 1 pneumocytes
B) Type 2 pneumocytes
C) Alveolar macrophages
D) Club cells

34) Which of the following is a recognized adverse effect of azithromycin?
A) Ototoxicity
B) QT interval prolongation
C) Nephrotoxicity
D) Photosensitivity

35) Which of the following is a recognized adverse effect of azithromycin?
A) Hepatotoxicity
B) Nephrotoxicity
C) QT interval prolongation
D) Bone marrow suppression

36) A smoker came to the clinic with dyspnea and chronic productive cough that lasted 3 months over the last 2 years. What is true about the laboratory findings?
A) reduced FEV1/FVC ratio
B) Normal FEV1/FVC ratio
C) Decreased total lung capacity
D) Reduced FVC



33	34	35	36
A	B	C	A

Mid respiratory

37) Patient comes to the clinic with a chronic cough, fever, and right-sided chest pain. Imaging shows an elevated right diaphragm, What is a likely parasitic cause?

A) Entamoeba histolytica
B) Schistosomiasis
C) Toxocariasis
D) Ascaris lumbricoides

38) A patient's arterial blood gas results show a pH of 7.22, a PCO_2 of 55 mmHg, and a normal HCO_3^- level. What is the most likely diagnosis?

A) Metabolic alkalosis
B) Respiratory alkalosis
C) Metabolic acidosis
D) Respiratory acidosis

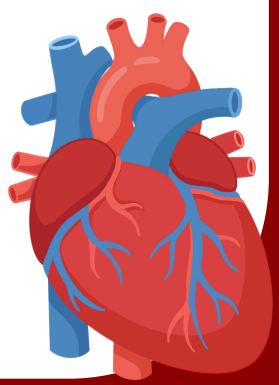
39) Which of the following types of emphysema is associated with smoking?

A) Panacinar emphysema
B) Centriacinar emphysema
C) Irregular emphysema
D) Paraseptal emphysema

40) A 20-year-old male died. At the age of 15, he started experiencing progressive shortness of breath and later developed hepatic cirrhosis. What is the underlying cause of his condition?

A) Decreased elastase
B) Decreased alpha-1 antitrypsin
C) Decreased protease
D) Increased alpha-1 antitrypsin

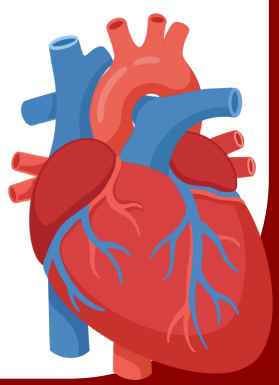
37	38	39	40
A	D	C	B



Mid respiratory

41) what is the source of volatile H+?
A)lactic acid
B) phosphorinc acid
C)carbonic acid
D)sulfuric acid

41
C



Med10 midterm

1- regarding Nasal cavity, all correct Except:

A) The roof is formed by cribriform plate of ethmoid bone.
B) The medial wall has chonchae and meatuses opening
C) The floor is formed by entirely soft palates.
D) The lateral wall contains superior, middle, and inferior conchae.

2- Hyaline membrane disease is due to deficient surfactant, which is secreted by:

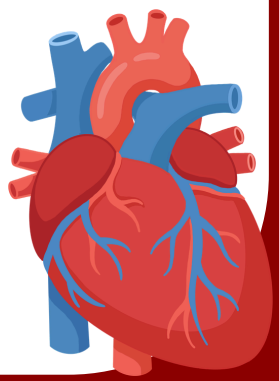
A) Clara cells
B) Pulmonary alveolar macrophages (Dust cells)
C) Alveolar epithelium (pneumocyte) type II
D) Alveolar epithelium (pneumocyte) type I

3- The muscle that abducts the vocal cords is:

A) Posterior-Cricoarytenoids
B) Transverse arytenoid
C) Thyroarytenoid
D) Lateral cricoarytenoid

4- The commonest congenital anomaly of trachea and esophagus is:

A) Isolated tracheoesophageal fistula without atresia
B) Esophageal atresia alone
C) Congenital tracheal stenosis
D) Esophageal atresia and tracheo-esophageal fistula



1	2	3	4
B	C	A	D

Med10 midterm

5- The Nasolacrimal duct opens to:

A) The Inferior meatus
B) The Superior meatus
C) The Middle meatus
D) The Sphenoethmoidal recess

6- Which of the following is not related to right lung:

A) Azygos vein
B) Arch of aorta
C) Superior vena cava
D) Esophagus

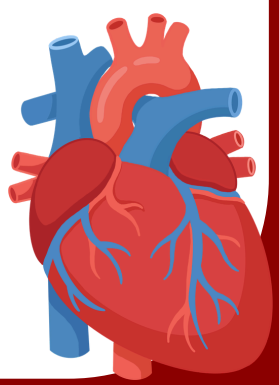
7- The following structures are related to the medial surface of the left lung, except:

A) Descending aorta
B) Arch of aorta
C) The left atrium

8- The diaphragmatic pleura is supplied by:

A) Intercostal nerves only
B) Phrenic and intercostal nerves
C) Phrenic nerve only
D) Vagus nerve

5	6	7	8
A	B	C	B



Med10 midterm

9- As regard to trachea & bronchi, the following are true except:

A) Right bronchus is wider than left
B) Right bronchus is longer then left
C) Right bronchus is shorter and more vertical
D) Trachea bifurcates at the level of sternal angle

10- Vital capacity:

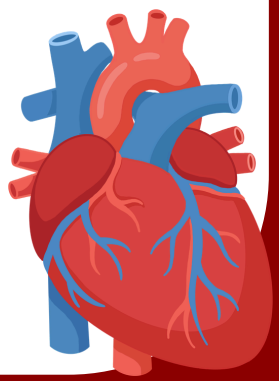
A) Volume of air remaining in the lungs after maximum expiration
B) Maximum volume of air that can be inhaled after a normal tidal expiration
C) Volume of air maximally expired after maximum inspiration
D) Volume of air in a normal tidal breath

11- Oxygen dissociation curve is shifted to the left by:

A) Increased temperature
B) Alkalosis
C) Increased 2,3-DPG
D) Acidosis

12- The most effective stimulus for central and peripheral chemoreceptors is:

A) Decrease in arterial PO2
B) PCO2
C) Increase in arterial blood pH
D) Decrease in plasma bicarbonate concentration



9	10	11	12
B	C	B	B

Med10 midterm

13- Intrapleural pressure:

A) Is generally less than Atmospheric Pressure
B) Is generally equal to Atmospheric Pressure
C) Is always greater than Atmospheric Pressure during rest
D) Becomes positive during normal quiet inspiration

14- Surfactant is decreased in:

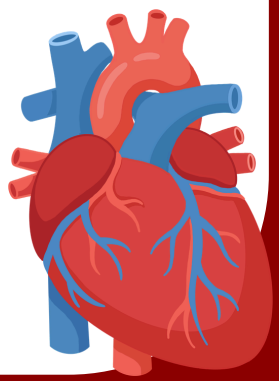
A) Bronchial asthma
B) Respiratory Distress Syndrome of Newly-Born
C) Chronic obstructive pulmonary disease (COPD)
D) Pulmonary emphysema

15- The Rate of Diffusion of a gas is decreased by an increase in the:

A) Surface area of the respiratory membrane
B) Pressure gradient of the gas across the membrane
C) Thickness of Respiratory Membrane
D) Solubility of the gas in the membrane

16- Shift of Oxy-Hemoglobin Dissociation Curve to the right is caused in:

A) Decreased hydrogen ion concentration (Alkalosis)
B) Decreased 2,3-DPG levels
C) Fever
D) Hypothermia



13	14	15	16
A	B	C	C

Med10 midterm

17- Chloride Ion moves into the RBC when:

A) Chloride shift does not occur
B) Oxygen binds to hemoglobin in the lungs
C) Carbon dioxide diffuses out of the tissue into plasma
D) BiCarbonate (HCO_3) moves out of the RBC

18- Midpontin + bilateral vagi transection produces:

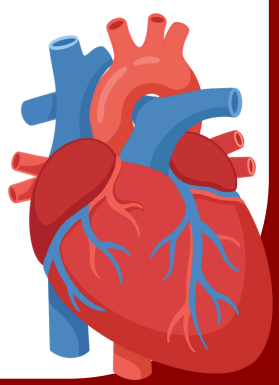
A) Apneustic breathing (apneusis)
B) Normal eupnea
C) Ataxic breathing
D) Cheyne-Stokes respiration

19- The Intra-pulmonary pressure:

A) drops to -5 mmHg throughout the entire cycle
B) is about 760 mmHg at the end of expiration
C) remains constantly higher than atmospheric pressure
D) drops to zero only during active coughing

20- Increase in Venous PO2 result in which hypoxia?

A) Hypoxic hypoxia
B) Histotoxic hypoxia
C) Anemic hypoxia
D) Stagnant hypoxia



17	18	19	20
D	A	B	B

Med10 midterm

21- Hering-Breuer reflex from:

A) J-receptors in pulmonary capillaries
B) Irritant receptors in airway epithelium
C) Lung stretch receptor
D) Central chemoreceptors in the medulla oblongata

22- Both peripheral & central chemoreceptors response to:

A) Decrease in arterial PO_2 below 60 mmHg
B) Direct drop in arterial blood pressure
C) Increase arterial PCO_2
D) Significant drop in plasma insulin levels

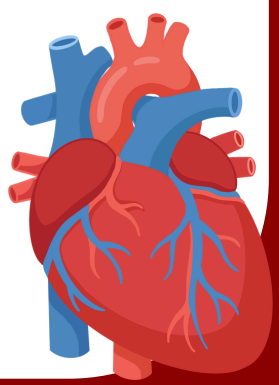
23- Opportunistic parasite:

A) Plasmodium falciparum
B) Strongyloides Stercoralis
C) Enterobius vermicularis
D) Ascaris lumbricoides

24- Loffler's syndrome manifested with:

A) Hookworm infection
B) Trichuris trichiura
C) Ascaris
D) Enterobius infection

21	22	23	24
C	C	B	C



Med10 midterm

25- Hydatid disease through:

A) Direct skin penetration by filariform larvae
B) Eating raw vegetables contaminated by eggs
C) Drinking contaminated unboiled river water directly
D) Vector-borne transmission via mosquito bites

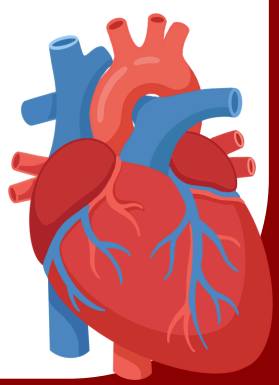
26- Regarding viruses, all correct Except:

A) Obligate intracellular parasites
B) Sensitive to antibiotic
C) Can contain either DNA or RNA, but not both
D) Lack independent metabolic machinery

29- Compensatory changes in respiratory acidosis include:

A) Decrease renal H^+ excretion
B) Increase excretion of bicarbonate by kidneys
C) Increase renal H^+ excretion
D) Decreased reabsorption of filtered bicarbonate

25	26	29
B	B	C



RESPA - FINAL

1- the transverse fissure of the right lung ends anteriorly at the level of the costal cartilage number

A) Fourth
B) Fifth
C) Sixth
D) Third

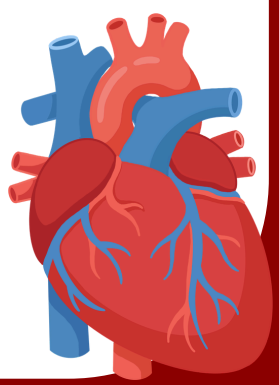
2-what is found on the medial side of the left lung?

A) azygos vein
B) arch of the aorta
C) left atrium
D) SVC

3- (case) about someone who didn't travel and had dyspnea , cough, malaise , blood work appears something (hemoptysis) , leukocytosis, andabundant neutrophils. On chest x-ray there's a consolidation in the right middle lobe.

A) primary tuberclosis
B) ...pneumonia

1	2	3
A	B	A



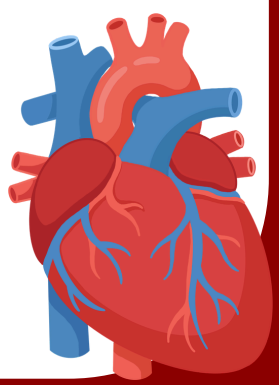
RESPA - FINAL

4- BCG vaccine is given:
A) Intradermal
B) Intramuscular
C) Subcutaneous
D) Ora

5- (case) about a child having paroxysmal cough, cyanosis, eye congestion, high pitched sound during intaking air between bouts of coughing. He was infected with gram negative coccobacilli. What is the diagnosis?
A) Whooping cough
B) Epiglottis

6- (case) 5-month infant with bronchiolitis ?
A) Respiratory syncytial virus
B) Adenovirus

7- Which of the following diseases is caused by incomplete separation of tracheoesophageal septa ?
A) Esophageal atresia
B) Trachea atresia
C) Tracheoesophageal fistula



4	5	6	7
A	A	A	C

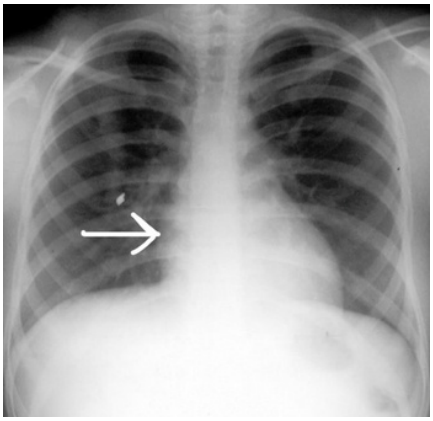
RESPA - FINAL

8- a 30 y/o woman represents with signs and symptoms of plural effusion. You decided to do diagnostic thoracocentesis. For what —- if the effusion is transudative or exudative?

A) RBCs content
B) protien and LDH level
C) cell count

9- What is this structure?

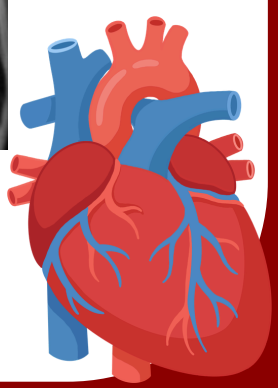
A) right atrium



10- What is the abnormality in this picture?

A) right side pleural effusion
B) Right side pneumothorax
C) Right side collapse
D) Left side pneumonectomy

8	9	10
B	A	C



RESPA - FINAL

11- a 44 year old patient came to you complaining of a persistent cough for three weeks, night sweats, fever and weight loss. Ziel Neelsen stain showed red beaded bacilli in a blue background. what is the virulence factor of its pathogenesis?

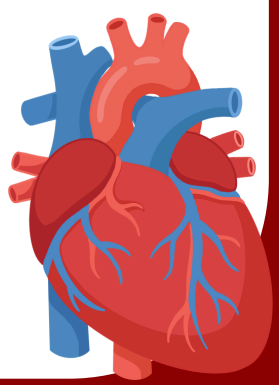
A) bacteria can destroy macrophages
B) bacteria can survive in macrophages
C) bacteria can live in bronchi
D) bacteria can destroy pulmonary tissue

12- which of the following nerves pierces thyrohyoid membrane?

A) internal laryngeal
B) External laryngeal
C) Recurrent laryngeal
D) Superior laryngeal

13- 50 year old female , 4cm apical malignancy

A) horner's syndrome
B) Hyperparathyroidism
C) Turner syndrome
D) Hyperthyroidism



11	12	13
B	A	A

RESPA - FINAL

14- 86-years old woman has difficulty and excreting the air all the time. She has no history of smoking or being exposed to fumes. There is no history of weight loss and no persistent cough. The most common reason for her condition is?

A) lose the elasticity of the lung
B) Bronchial asthma
C) Lung cancer
D) Decrease surfactant

15-which of the following does not contain smooth muscle?

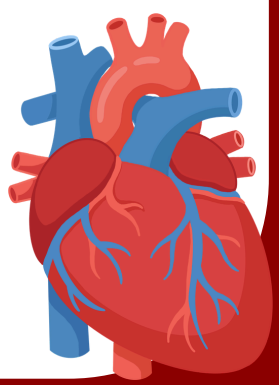
A) alveoli
B) Trachea
C) Bronchi
D) Bronchioles

16- emphysema present in case of deficiency of antitrypsin 1

A) central-acinar
B) Pan-acinar
C) Para-acinar

17-African female has high level of calcium and angiotensin converting enzyme + bilateral hilar lymphadenopathy. What is the diagnosis?

A) Sarcoidosis
B) Silicosis



14	15	16	17
A	A	B	A

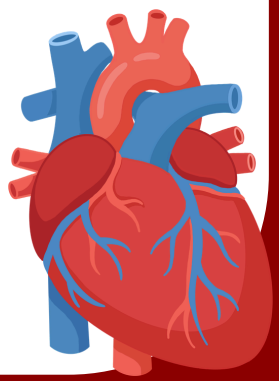
RESPA - FINAL

18- What nerve supplies the mediastinal pleura?
A) Phrenic nerve
B) Intercostal nerve

19- (pointing to the roof of the nose), what is the special nerve?
A) Olfactory nerve**

20- 58 patient has obesity, loud snoring at night, deveolped cor pumlonae what is the most likely cause of his condition?
A) Emphysema
B) Lung tumor
C) Hypoxia
D) Infection

21- Someone has an obstruction in the bronchus, what is the expected PO2 in the capillaries?
A) Equal to the partial pressure of atmospheric
B) Equal to normal arterial PO2
C) More the PO2 of inspired air
D) Equal to venous PO2 maybe



18	19	20	21
A	A	D	D

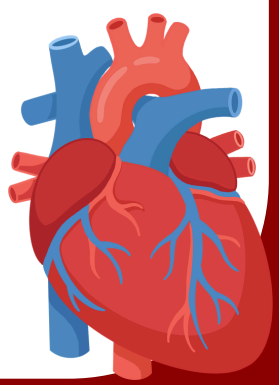
RESPA - FINAL

22- Which reflex protects the respiratory tract during coughing?
A) pharyngeal receptor
B) Irritant receptor
C) Central receptor
D) Peripheral receptor

23- What is true about central receptors?
A) CO2 directly activates them
B) Maximum stimulation by increased H+ in CSF
C) Affected by arterial pH

25- A surgery was performed on the superior thyroid artery and it is suspected that nerve
damage was made, what would result from this never damage?
A) decreased tension

26- A surgery was performed on the inferior thyroid artery and it was suspected that nerve damage was made, what would this nerve damage cause?
A) Loss of sense above the vocal cord
B) Paralysis of vocalis muscle
C) Decrease the tension of vocal cords



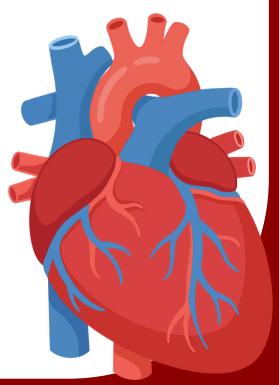
22	23	25	26
B	B	A	B

RESPA - FINAL

27- Which antibiotic is used before exercise and does not cause bronchodilation?
A) inhaled cromolyn
B) Inhaled beta agonist

28- An infant aged six months admitted to paediatric emergency , with dehydration due severe attack of gastroenteritis , after clinical examination, the physician described your urgent intravenous fluid only EJV was available to give , after few minutes the physician noted severe dyspnoea and respiratory distress of infant , x ray showed pneumothorax of same side of injury, which part of pleura was suspected??
A- costal
B- visceral
C- cervical
D- mediastinal

29- Which time will we expect to have the maximum anticoagulant effect of warfarin ?
A) one day
B) three days
C) 6 hours
D) one week



27	28	29
A	C	B

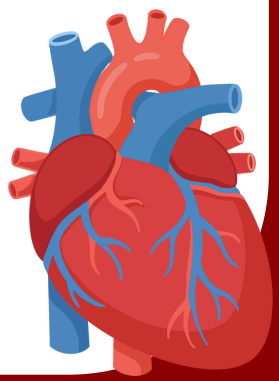
RESPA - FINAL

30- Which of the following will be used in case of heparin overdose?
A) heparin sulfate
B)Protamine sulfate

31- A male Shepard from Al Ulla came complaining with hepatomegaly, upper right quadrant pain, jaundice, and a non painful hepatic cysts was found, what is the likely causative agent?
A) echinococcus
B) Entamoeba histolytica
C) Schistosomiasis

32- Patient ate green salad and got a parasitic infection with increased eosinophilia?
A) Ascariasis
B) Schistosomas
C) Toxocariasis
D) Entamoeba histolytica

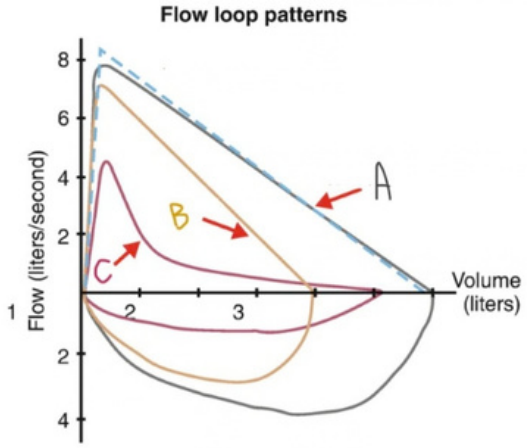
33- Where are the larva forms of echinococcus found?
A) man
B) Dog
C) Cat
D) Wolf



30	31	32	33
B	A	A	A

RESPA - FINAL

34-35:



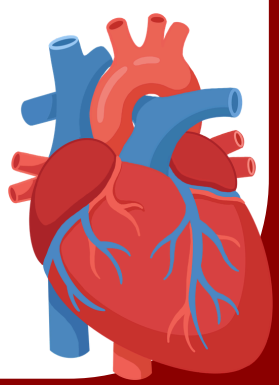
34- what type of lung disease is labeled as (A)?

A) obstructive
B) Restrictive
C) Normal

35- What type of lung disease is labeled as (B)?

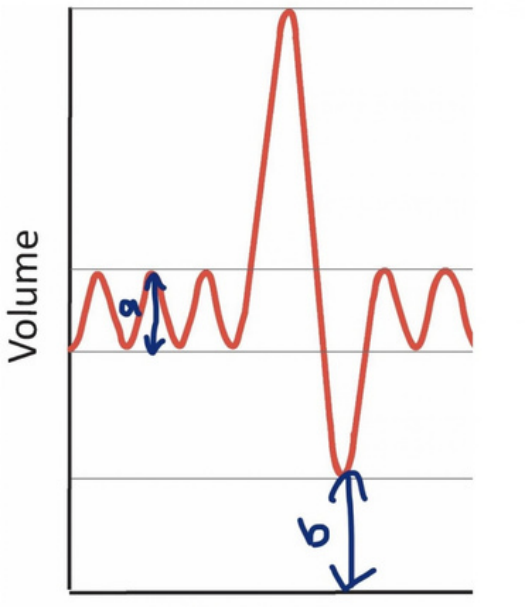
A) obstructive
B) Restrictive
C) Normal

34	35
C	B



RESPA - FINAL

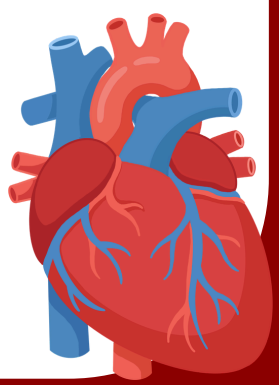
36-37:



36- What is (a) in the diagram?
A) tidal volume
B) Residual volume

37- what is (b) on the diagram?
A) residual volume
B) Vital capacity

36	37
A	A



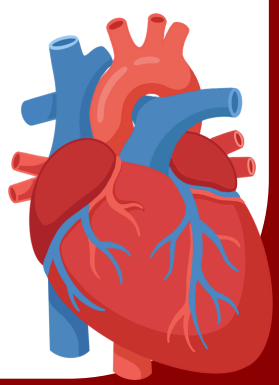
RESPA - FINAL

38- Ali is a 65 years-old man with COPD. He is currently using albuterol inhaler PRN, Salmeterol inhaler twice a day, and tiotropium once a day. His COPD is still uncontrolled with frequent symptoms and recurrent exacerbation. What recommendation would you make to his regimen?

A) Add on oral corticosteroids once daily
B) Add on theophylline once daily
C) Change the ipratropium inhaler to ipratropium in scheduled four times a day
D) Change salmeterol to fluticasone/salmeterol combination in scheduled two times a
day

39- 69 years-old man has COPD with moderate airway obstruction. Despite using salmeterol two times daily, he reports continued symptoms of shortness of breath with mild exertion. Which agent is an appropriate addition to his current therapy?

A) Salbutamol
B) Tiotropium
C) Systemic corticosteroids
D) Theophylline



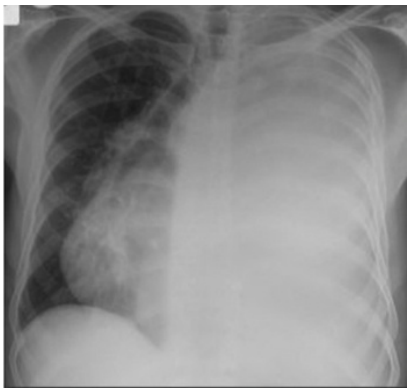
38	39
D	B

RESPA - FINAL

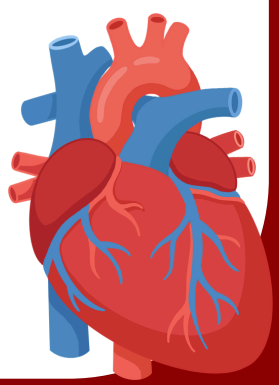
40- which of the following is responsible for increased lymphatic return during inspiration?
A) Decreased transpulmonary pressure
B) Negative intraalveolar pressure
C) Abdominal muscle contraction
D) Increased negativity of intrathoracic pressure

41- a 70 year old man who is a heavy smoker for nearly 50 years with possible harm to type II alveolar cells. Which of the following would probably decrease?
A) lung compliance
B) Filtration into the alveoli
C) Work of breathing
D) Surface tension in the alveoli

42- What is the abnormality in this picture?
A) Left side pleural effusion
B) Left side pneumothorax
C) Left side collapse
D) Left side pneumonectomy



40	41	42
D	A	A

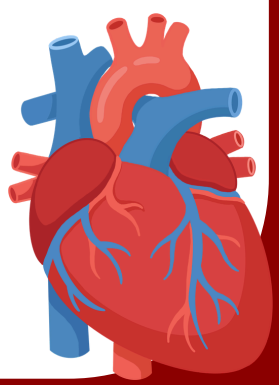


RESPA - FINAL

43- 56 year old alcoholic man present to the clinic with foul smelling sputum later on an x- ray was ordered which revealed a lung abscess what could be the cause of this abscess?
A) aspiration of gastric content
B) Inhaled carious teeth

44- restrictive lung diseases are characterized by what?
A) wheeze
B) Fever
C) Hemopytsis
D) Dry cough

45- 62 year old man who has a history of asthma presented to ER with difficulty breathing and ABG test done was done which revealed the following pH = 7.24, pCO2 = 55 mmHg, HCO3= 26 what the interpretation of these parameters?
A) respiratory alkalosis
B) Respiratory acidosis
C) Metabolic alkalosis
D) Metabolic acidosis



43	44	45
A	D	B

FinalNight RS - MED23

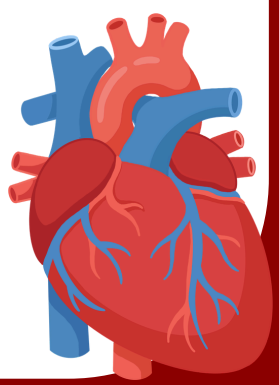
1.Peripheral chemoreceptors are most sensitive when partial pressure of oxygen falls below?
A. 100
B. 60
C. 0
D. 30

2. Peripheral chemoreceptors?
A. React to hypocapnia more rapid than central chemoreceptors
B. Don't react to hypocapnia
C. Don't react to hypercapnia
D. react to increased pH

3. Which of the following can be stimulus in response to lactic acid?
A. Central chemoreceptors
B. Peripheral chemoreceptors
C. Lung irritate receptors
D. Arterial baroreceptors

4. Which of the following true regarding central chemoreceptor?
A. Maximum discharge responding to H in CSF
B. Respond directly to increase CO2
C. Respond to changes in ph
D. Sensitive to hypoxia

1	2	3	4
B	B	B	A



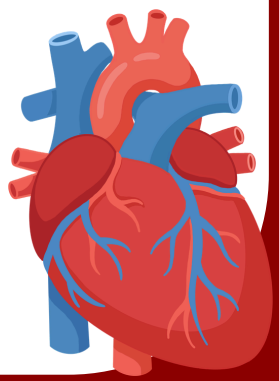
FinalNight RS - MED23

5. An athletic goes to gym after training the lactic acid in his body elevate. Which of the following can be stimulus in response to lactic acid?
A. Central chemoreceptors
B. Peripheral chemoreceptors
C. Lung irritant receptors
D. Arterial baroreceptors

6. What is true about central receptors?
A. CO2 directly activates them
B. Maximum stimulation by increased H+ in CSF
C. Affected by arterial pH
D. Increase H+ in plasma stimulate it

7. Hormone produced in excess one month after climbing a mountain?
A. Thyroxin
B. Natriuretic peptide
C. Adrenaline
D. Erythropoietin

8. Patient was brought to emergency because he has hypoxia and distress, his oxygen pressure decreases markedly and not improved by oxygen therapy, what's the cause?
A. Ventilation-perfusion mismatch
B. Diffusion defect
C. Hypoventilation
D. Toxicity of mitochondrial enzyme



5	6	7	8
B	B	D	D

FinalNight RS - MED23

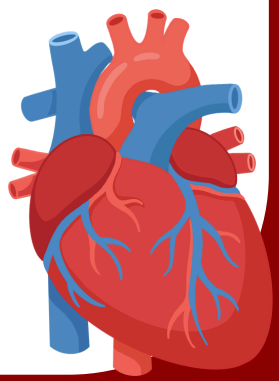
9. A person arrives to the ER with tachycardia and tachypnea. It was noted that the patient was hypoxic and was given O2 therapy. The patient did not respond to therapy. Which of the following defects could contribute to this?
A. Congenital anomalies
B. Histotoxic
C. High altitude
D. Ventilation / perfusion ratio mismatch

10. Which of the following will not increase within a day of climbing a mountain?
A. Heart rate
B. Hematocrit
C. 2,3- DPG
D. pH

11. A person arrives to the ER with tachycardia and tachypnea. It was noted that the patient was hypoxic and was given O2 therapy. The patient did not respond to therapy. Which of the following defects could contribute to this?
A. Ventricular septal defect
B. Mitochondrial enzyme defect
C. CO poisoning
D. V/Q imbalance

12. What does NOT complicate Bronchial asthma?
A. Pneumonia
B. Recurrent infections
C. Panacinar emphysema
D. Hyperplasia of goblet cells

9	10	11	12
B	B	B	C



FinalNight RS - MED23

13. Status asthmaticus?
A. Severe attacks with no response to therapy
B. Late stage
C. Early stage
D. Drugs induced asthma

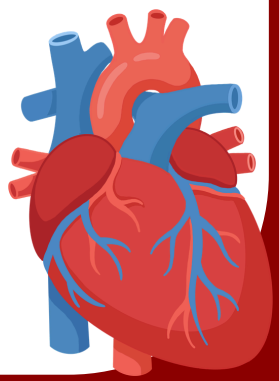
14. What do you advise a patient that has had bronchial asthma since childhood and positive family history?
A. Avoid exercise
B. Stop smoking
C. Avoid pollen grain
D. Avoid stress and anxiety

15. Choose the best correlation?
A. Status asthmaticus / severe respiratory attack for days
B. Status asthmaticus / caused by Aspirin
C. Drug-induced / caused by workplace triggers
D. Intrinsic asthma / type I IgE-mediated hypersensitivity

16. Which is the best state that describes an asthmatic attack?
A. Induce drug asthma
B. Early developed asthma
C. Rhinitis
D. Severe attack with no response to the drugs

17. 17 years old boy with a strong family history of asthma with increased eosinophil, what advice would you give him?
A. Stop smoking
B. Avoid pollen grains
C. Avoid exercise and swimming
D. Avoid stress and anxiety

13	14	15	16	17
A	C	A	D	B



FinalNight RS - MED23

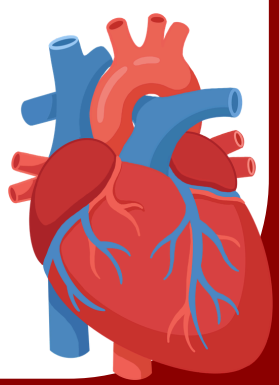
18. A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing. The attacks are particularly common in the spring. During an episode of acute respiratory difficulty, a physical examination shows that she is afebrile. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. A sputum sample examined microscopically also has increased numbers of eosinophils. Which of the following histologic features is most likely to characterize the lung in her condition?

A. Remodeling of airways with smooth muscle hyperplasia
B. Patchy areas of consolidation with leukocytic exudate in alveoli
C. Dilation of respiratory bronchioles with loss of elastic fibers
D. Inflammatory destruction of bronchial walls

19. 11 years old boy complains of cough, wheezing and difficulty of breathing, hyper resonance (increased air in lung), laboratory finding showed high IgE and eosinophil levels, what is the histological appearance of this acute condition?

A. Remodeling of airways with smooth muscle hyperplasia
B. Patchy areas of consolidation with leukocytic exudate in alveoli
C. Dilation of respiratory bronchioles with loss of elastic fibers
D. Inflammatory destruction of bronchial walls

18	19
A	A



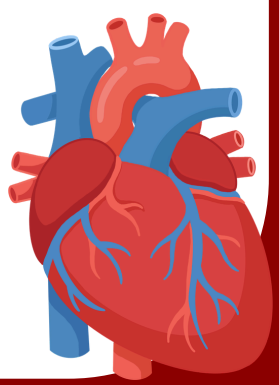
FinalNight RS - MED23

20. After taking a biopsy from a patient that undergoes an attack that lasts for weeks without response to treatment followed by hypercapnia and death after a few days. The microscope show?
A. Asteroid bodies
B. Hypertrophy of smooth muscle
C. Fibrosis
D. Goblet cells atrophy

21. 11 years old boy complains of cough, wheezing and difficulty of breathing, hyper resonance (increased air in lung), laboratory finding showed high IgE and eosinophil levels, what is the histological appearance of this acute condition?
A. Remodeling of airways with smooth muscle hyperplasia
B. Patchy areas of consolidation with leukocytic exudate in alveoli
C. Dilation of respiratory bronchioles with loss of elastic fibers
D. Inflammatory destruction of bronchial walls

22. Regarding pneumoconiosis, which of the following are correct?
A. It is a restrictive lung disease
B. It is a disease of smokers
C. It is forced vital capacity is normal
D. It is forced expiratory volume markedly reduced

23. What is seen in Late stage idiopathic pulmonary fibrosis?
A. Capillaries are filled with neutrophils
B. Alveoli are lined by type II pneumocytes and separated by collagen
C. Fibrinous exudate in pleura
D. Alveolar spaces are filled with foamy macrophages



20	21	22	23
B	A	A	B

FinalNight RS - MED23

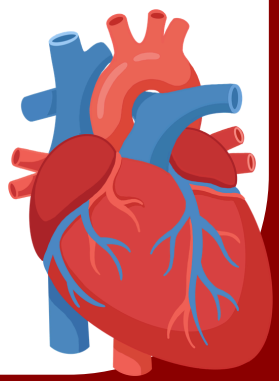
24. A 50-year old female patient came in with Dry cough, Bilateral hilar lymphadenopathy with Non-caseating granuloma. Which of the following is the most likely diagnosis?
A. Sarcoidosis
B. Pneumoconiosis
C. Emphysema
D. Asthma

25. A 60 year old man has dyspnea, nonproductive cough, he is a nonsmoker, he works as a shipbuilder and was exposed to asbestos 20 years ago. Which of the following is the most likely diagnosis?
A. Acute respiratory disaster syndrome
B. Good pasture syndrome
C. Malignant mesothelioma
D. Idiopathic

26. Female african american has a non-caseating granuloma with skin lesion, what type of her erythema?
A. Erythema migrans
B. Erythema multiform
C. Erythema nodosum
D. Erythema toxicum

27. African females have high levels of calcium and ACE + bilateral hilar lymphadenopathy. What is the diagnosis?
A. Sarcoidosis
B. Silicosis
C. Berylliosis
D. Asbestosis

24	25	26	27
A	C	C	A



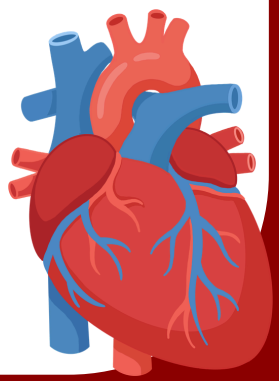
FinalNight RS - MED23

28. A patient's lung autopsy showed liver-like lung and histologically the alveoli was filled with fibrin, red cells and neutrophils. What stage of pneumonia is that?
A. Congestion
B. Red hepatization
C. Grey hepatization
D. Resolution

29. An alcoholic patient, vomited multiple times a day. Presented with cough with foul sputum. The cause of this could be?
A. Viral pneumonia
B. Pulmonary infarct
C. Aspiration pneumonia
D. Nosocomial pneumonia

30. Lysis of RBC's and fibrin exudate in alveoli are seen in which stage of acute bacterial pneumonia?
A. Red hepatization
B. Grey hepatization
C. Resolution
D. Congestion

31. A patient came with Flu-like symptoms and cough and little amount of sputum, no consolidation. The X-ray showed Ground glass opacities under microscope the alveolar sample showed hyaline membranes. What is the cause?
A. Atypical viral pneumonia
B. Lung abscess
C. Acute bacterial pneumonia
D. Nosocomial pneumonia



28	29	30	31
B	C	B	A

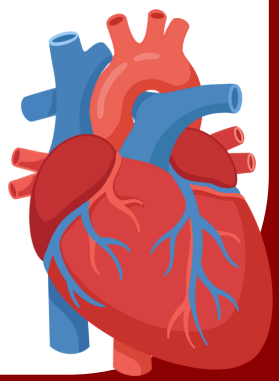
FinalNight RS - MED23

32. 56 year old alcoholic man present to the clinic with foul smelling sputum later on an x ray was ordered which revealed a lung abscess what could be the cause of this abscess?
A. Aspiration of gastric content
B. Pulmonary embolism
C. Viral pneumonia
D. Tuberculosis

33. Why are smokers at higher risk of infection than non-smoker?
A. Impaired epiglottic reflex
B. Impaired mucociliary clearance
C. Impaired cough reflex
D. Cause aspiration

34. A 72-year-old man presents with severe shortness of breath and has been diagnosed with acute bacterial pneumonia. Which of the following underlying conditions most likely increased his risk for developing this acute infection?
A. Pulmonary Fibrosis
B. Congestive Heart failure
C. Lung cancer
D. Pulmonary hypertension

35. Which of the following is characteristic histologic feature in atypical pneumonia?
A. Alveolar spaces are filled with fibrin and blood
B. Alveolar spaces are clear from inflammation
C. Alveolar septa are clear from inflammation
D. Alveolar spaces are filled with neutrophils



32	33	34	35
A	B	B	B

FinalNight RS 🍿 - MED23

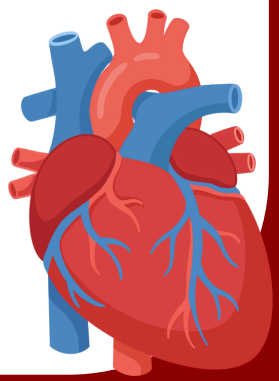
36. A 74 years old male presented to the emergency with lethargy, fever, productive cough, worsening dyspnea for a week duration. Diagnosis was demonstrate severe pneumonia. Which of the following is complication of bacterial pneumonia?
A. Lung cancer
B. Heart failure
C. Lung fibrosis
D. Empyema

37. Case about an HIV patient; on microscope they found cup-shaped cysts in silver stain, most common likely organism?
A. Histoplasma
B. Coccidioides
C. Pneumocystis jiroveci
D. Candidiasis

38. (Case) A patient presents with symptoms of dyspnea, cough, and malaise. She has not traveled recently. Laboratory tests indicate leukocytosis with an abundance of neutrophils and hemoptysis. A chest X-ray reveals consolidation in the right middle lobe.
A. Primary tuberculosis
B. Pneumonia
C. Bronchitis
D. Asthma

39. Lines of Zahn are seen in:
A. Pulmonary congestion
B. Arterial thrombi
C. Postmortem clot
D. Venous thrombi

36	37	38	39
D	C	B	B



FinalNight RS 🍿 - MED23

40. A 25-year-old male presented to the ER with a 2-day history of cough, fever, shortness of breath, and weight loss. He had visited a patient with similar symptoms. What's the most likely diagnosis?

A. Tuberculosis
B. Pneumonia
C. COPD
D. Bronchial asthma

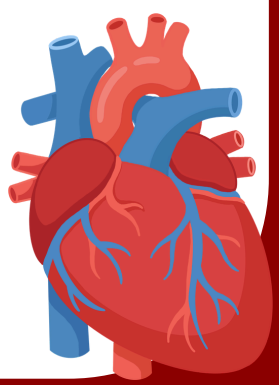
41. Pulmonary embolism will cause hypoxemia by which of the following ways?

A. Increase V/Q ratio around embolus
B. Decrease V/Q ratio distal from embolus
C. Decreased shunting
D. Edema and defect diffusion

42. A 30-year-old female with a history of renal transplant and chronic coughs presented with dyspnea. Upon examination, the lung had nodules. On biopsy, intact spherules within multinucleated giant cells were found. What is the causative microorganism?

A. Tuberculosis
B. Histoplasma
C. Coccidioides
D. Blastomyces dermatitidis

40	41	42
A	A	C



FinalNight RS - MED23

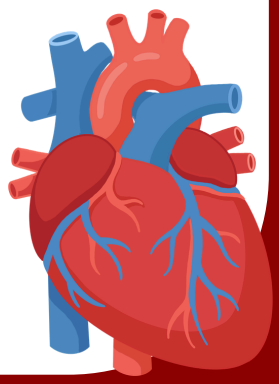
43. A 36-year-old woman came with shortness of breath. She had a history of a clot and was not taking any preventative measures. If she develops pulmonary embolism, what physiological change will happen?
A. Decrease respiratory rate
B. Decrease heart rate
C. Hypoventilation
D. Increase alveolar dead space

44. What microorganism causes pulmonary mycosis characterized by broad budding on a silver stain?
A. Histoplasma
B. Coccidioides
C. Blastomyces
D. Actinomycosis

45. A 45-year-old obese male came to the ER with chest pain and shortness of breath. After measuring the vital signs, his blood pressure was high. What is a significant contributor to the development of cor pulmonale?
A. Recurrent thromboemboli
B. Congenital or acquired heart disease
C. Interstitial lung diseases
D. Obstructive sleep apnea

46. A 57-year-old patient came to the ER with pulmonary hypertension. What is the most common change that occurs in his arteries?
A. Elastic degeneration
B. Medial hypertrophy
C. Thinning of the vessel wall
D. Arterial dilation

43	44	45	46
D	C	D	B



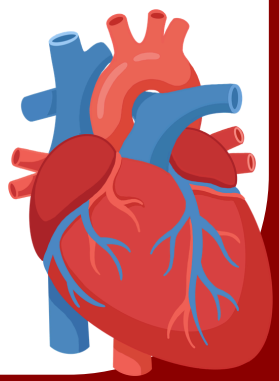
FinalNight RS - MED23

47. A 25-year-old Indian male presented to the hospital with a 2-day history of fever, cough, and hemoptysis. He stated that he was previously healthy. A CXR shows a cavity lesion in his lung. What is the cause of his condition?
A. Pulmonary embolism
B. Pulmonary tuberculosis
C. Lung abscess
D. Blastomyces dermatitidis

48. What microorganism causes pulmonary mycosis characterized by broad budding on a silver stain?
A. Histoplasma
B. Coccidioides
C. Blastomyces
D. Actinomycosis

49. A group of large cancer cells with bizarre anaplastic cells:
A. Large cell carcinoma
B. Small cell carcinoma
C. Squamous cell carcinoma
D. Adenocarcinoma

50. A 45 year old Female patient presented with a peripheral tumor Microscopy showed malignant cells forming glands:
A. Active mutation in EGFR gene
B. Active mutation in BAP1 gene
C. Active mutation in Tp53 gene
D. Deletion in 3p



47	48	49	50
B	C	A	A

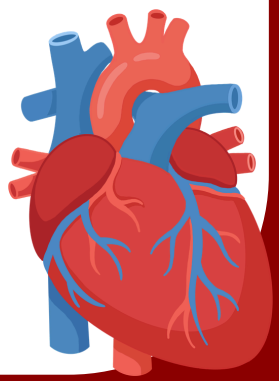
FinalNight RS - MED23

51. A smoker came to clinic with tumor in central lung tissue, under microscope small round, cells with scanty cytoplasm, finely granular chromatin (salt & pepper). What is the correct about this tumor?
A. Neuroendocrine origin
B. Late local invasion
C. Easy to prognosis
D. Takes time to metastasis lymph node at time of diagnosis

52. 50 year old female, 4cm apical malignancy with enophthalmos, ptosis, meiosis, and anhidrosis she has?
A. Horner's syndrome
B. Hyperparathyroidism
C. Turner syndrome
D. Hyperthyroidism

53. The percentage of lung cancer, because of smoking:
A. 80-90%
B. 50-60%
C. 10%
D. 100%

54. Which of the following drug affects growth because it's have been taken a lot during childhood?
A. Corticosteroids
B. Ampicillin
C. Theophylline
D. Macrolides



51	52	53	54
A	A	A	A

FinalNight RS - MED23

55. A 66-year old man, resident of Mumbai who used to work in Shipyard, was referred to the outpatient department complaining of predominantly dry cough and dyspnea on exertion. Chest computed tomography (CT) revealed heterogeneously enhancing nodular pleural thickening. Histopathology revealed malignant biphasic neoplasm showing both Epithelial and Sarcomatoid components. Which of the following is most likely involved in the pathogenesis of this tumor?

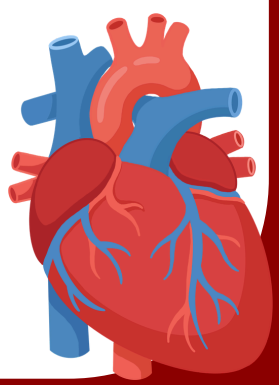
A. Arsenic
B. Carbon dust
C. Chromium
D. Asbestos

56. A 26 year old With history of bronchial asthma maintained by Salbutamol had an asthma attack without response to Salbutamol called the emergency in this situation. What should be done?

A. Start on Theophylline and to go to the emergency room
B. Discontinue Salbutamol and take Ipratropium
C. Take corticosteroids and go to the emergency room
D. Double the Salbutamol and go to the emergency room

57. A female came to the ER with insomnia and seizures after taking an overdose of a direct bronchodilation drug which is used orally usually for asthma attacks?

A. Ipratropium
B. Prednisone
C. Theophylline
D. Cromolyn



55	56	57
D	C	C

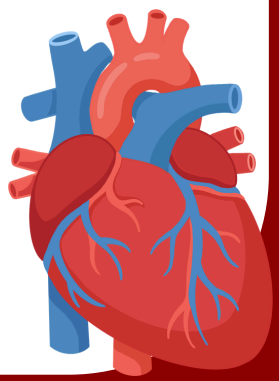
FinalNight RS - MED23

58. A patient who has a cough and wheezing. He take inhaling corticosteroid what is the instruction that he should follow?
A. Rinsing the mouth after inhalation with water
B. Holding the breath for 2 minutes after inhaling
C. Inhaling rapidly and forcefully for maximum effect
D. Using the inhaler immediately before meals

59. Which of the following drugs have tolerance to its own therapeutic effects?
A. Short acting beta2 agonists
B. Montelukast
C. Theophylline
D. Cromolyn

60. Which of the following reduces the effectiveness of salbutamol?
A. Propranolol
B. Theophylline
C. Corticosteroid
D. Tiotropium

61. A 70 kg woman came to the ER with acute exacerbated asthma. After setting an oxygen cannula, what is the first appropriate drug to be used for treatment?
A. Salbutamol nebulization
B. Slow IV Cortisone
C. Aminophylline slow IV infusion
D. Ipratropium nebulization



58	59	60	61
A	A	A	A

FinalNight RS - MED23

62. During pregnancy which of the following drug preferred in long term asthma control?
A. Albuterol
B. Theophylline
C. Theobromine
D. Inhaled corticosteroids

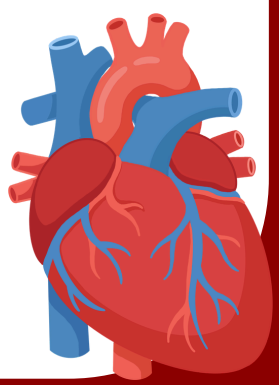
63. Which of the following drugs worsens the Asthma control?
A. Atorvastatin
B. Propranolol
C. Cephalosporin
D. Losartan

64. What happens when we use beta 2 agonist with regular use?
A. Tolerance
B. Increased risk of asthma attacks
C. Improved lung function permanently
D. Decreased heart rate

65. Which of the following drugs are used in asthma and cause hoarseness of voice and candidiasis?
A. Theophylline
B. Ipratropium bromide
C. Zileuton
D. Beclomethasone

66. Which of the following drugs are used in asthma and cause hoarseness of voice and candidiasis?
A. Theophylline
B. Ipratropium bromide
C. Zileuton
D. Beclomethasone

62	63	64	65	66
D	B	A	D	D



FinalNight RS - MED23

66. Work of breathing is decreased during?
A. B2 agonist
B. Anti inflammatory drugs
C. Pregnancy
D. Bronchial asthma

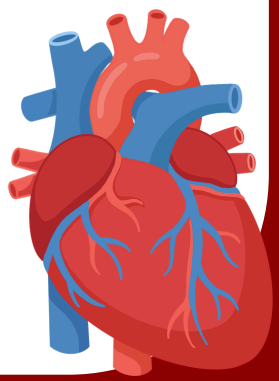
67. What is the pharma action of cromolyn?
A. Beta agonist
B. Bronchodilator
C. Inhibits Mast cell degranulation
D. Bronchoconstriction

68. What is the drug that is used mainly in acute asthmatic attack?
A. Salbutamol
B. Cromolyn
C. Omalizumab
D. Corticosteroids

69. Rinsing the mouth after inhalation of corticosteroids help to:
A. Increase the absorption of the medication
B. Enhance the bronchodilator effect
C. Decrease the possibility of oropharyngeal candidiasis
D. Prevent the development of tolerance to the medication

70. All the following cause asthma EXCEPT:
A. Beta blockers
B. Ibuprofen
C. Atropine
D. Exercise

66	67	68	69	70
A	C	A	C	C



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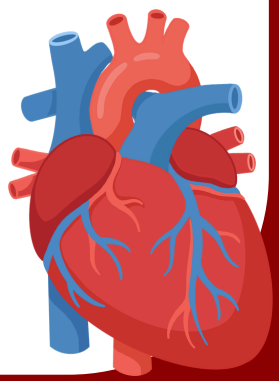
71. A 15 years old boy presented to ER with persistent, moderate asthma he uses Fluticasone 220 mcg (orally), 1 puff twice a day, Montelukast 10 mg daily, and Salbutamol (albuterol) when needed. After 3 months of well-controlled, the doctor will recommend:
A. Lower Montelukast dose
B. Lower Fluticasone dose
C. Increase Salbutamol frequency
D. Stop all medications

72. Omalizumab is best prescribed for:
A. Chronic asthma in patients >12 years as an add-on therapy with inhaled corticosteroids
B. Chronic asthma in patients >12 years as add-on therapy with LABA
C. Chronic asthma for patient >12 years and are uncontrolled with ICS (inhaled corticosteroid)
D. Acute exacerbation of chronic asthma

73. All of the following increase the work of breathing except:
A. Beta antagonist
B. Anti-inflammatory drug
C. Bronchial asthma
D. Lying down

74. Which of the following drugs can be used as a rescue drug for asthmatic attacks?
A. Ipratropium
B. Salbutamol
C. Cromolyn
D. Prednisolone

71	72	73	74
B	C	B	B



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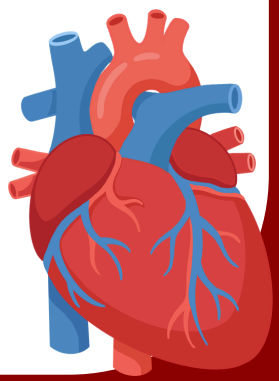
75. Which of the following is used for treating a hospitalized patient with severe acute asthma?
A. Aminophylline slow IV infusion
B. Albuterol
C. Cromolyn
D. Montelukast

76. Why is Theophylline considered a 3rd line of treatment in acute asthma?
A. Low bioavailability after oral
B. Narrow therapeutic index
C. A+B
D. None

77. Which antibiotic is used before exercise and does not cause bronchodilation?
A. Inhaled cromolyn
B. Inhaled beta agonist
C. Montelukast
D. Oral corticosteroid

78. Dosing for omalizumab is based on which of the following criteria:
A. Baseline severity of asthma
B. Renal function
C. Baseline of IgE level
D. Liver function



75	76	77	78
B	B	A	C

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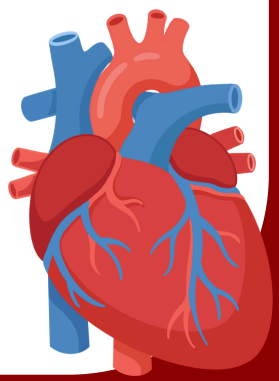
79. Ali is a 65 years-old man with COPD. He is currently using albuterol inhaler PRN (when required), Salmeterol inhaler twice a day, and tiotropium once a day. His COPD is still uncontrolled with frequent symptoms and recurrent exacerbation. What recommendation would you make to his regimen?
A. Add on oral corticosteroids once daily
B. Add on theophylline once daily
C. Change the ipratropium inhaler to ipratropium in scheduled four times a day
D. Change salmeterol to fluticasone/salmeterol combination in scheduled two times a day

80. A 69 years-old man has COPD with moderate airway obstruction. Despite using salmeterol two times daily, he reports continued symptoms of shortness of breath with mild exertion. Which agent is an appropriate addition to his current therapy?
A. Salbutamol
B. Tiotropium
C. Systemic corticosteroids
D. Theophylline

81. A 68 year-old-male has COPD with moderate airway obstruction. Despite using salmeterol twice daily as prescribed, he reports continued symptoms of shortness of breath. Which one of the following agents would be an appropriate addition to his current therapy?
A. Systemic Corticosteroids
B. Albuterol
C. Tiotropium
D. Theophylline



79	80	81
D	B	C

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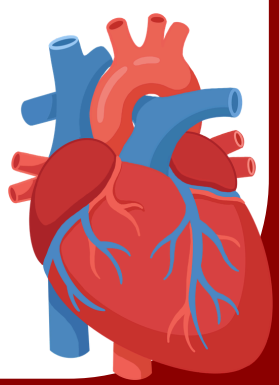
82. Which of the following choices of drugs are preferred in COPD?
A. Theophylline and antihistamine
B. Beta agonist and Muscarinic antagonist
C. Beta antagonist and inhaled corticosteroids
D. Cholinergic agonist and inhaled corticosteroids

83. Which of the following is considered a mucolytic drug?
A. Stimulates the mucociliary activity of the respiratory epithelium
B. Reduces the mucous secretions
C. Increases the mucous secretions
D. Makes the respiratory secretions more watery

84. Exacerbate chronic bronchitis and take oxygen but he needs drugs. What is initial therapy?
A. Nebulization salbutamol 2.5mg every 20 second
B. Nebulization Ipratropium 0.5mg every 20 second
C. Inhaled corticosteroids
D. Antibiotics

85. The toxic effects of INH (central and peripheral neuropathy) are reversed by:
A. Folate
B. Pyridoxine
C. Cyanocobalamin
D. Thiamine

82	83	84	85
B	D	B	B



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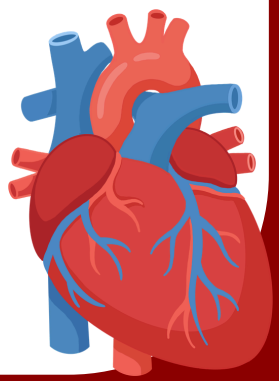
86. A female is on an anti-tuberculosis drug and oral contraceptives. After a while, she felt sick and had a positive pregnancy result. Which drug caused her pregnancy?
A. Rifampicin
B. Isoniazid
C. Streptomycin
D. Ethambutol

87. Which of the following anti-TB drugs causes visual disturbances, including slight color blindness and vision impairment?
A. Isoniazid
B. Ethambutol
C. Pyrazinamide
D. Rifampicin

88. Which of the following drugs can cause hepatotoxic effects in the treatment of TB?
A. para-Aminosalicylic Acid
B. Rifampicin
C. para-Aminosalicylic Acid + Rifampicin
D. Ethambutol

89. A patient is taking a drug to control his TB, which results in hyperuricemia. What drug is responsible for this side effect?
A. Isoniazid
B. Rifampin
C. Pyrazinamide
D. Streptomycin



86	87	88	89
A	B	C	C

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90. An HIV patient recently diagnosed with TB started using HAART therapy (saquinavir). Which drug is least preferred?
A. Rifampicin
B. Isoniazid
C. Pyrazinamide
D. Ethambutol

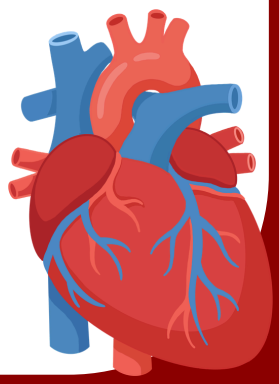
91. Choose the wrong statement:
A. Isoniazid causes decrease CYP3A4 activity
B. Rifampicin causes increased CYP3A4 activity
C. Pyrazinamide is an inducer for CYP3A4
D. Warfarin effect is decreased by Rifampicin

92. Which drug's metabolism is affected by genetic differentiation in acetylation?
A. Isoniazid
B. Pyrazinamide
C. Fluoroquinolones
D. Ethambutol

93. Which of the following is incorrect about warfarin?
A. Its overdose can be reversed by vitamin k
B. It cannot cross placenta
C. Broad spectrum antibiotics can increase its action
D. Phenobarbitone can decrease its action

94. Which of the following are direct thrombin inhibitors?
A. Argatroban
B. Dalteparin
C. Enoxaparin
D. Danaparoid

90	91	92	93	94
A	C	A	B	A



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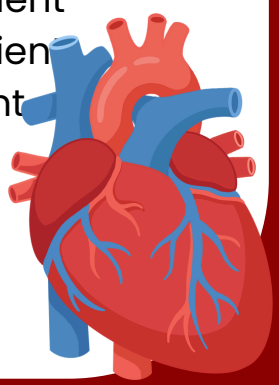
95. Which of the following is not an advantage of low molecular weight heparin?
A. It lowers the incidence of heparin induced thrombocytopenia
B. It enhances clot degradation
C. Lowers the frequent dosing
D. No need for routine monitoring of (aPTT)

96. LMWH is different from high molecular weight heparin in:
A. It inactivates factor Xa completely and only partially inactivate IIa (thrombin)
B. Completely inhibits factor IIa (Thrombin), IXa, and Xa
C. Inhibits the synthesis of vitamin K-dependent clotting factors
D. Independent from anti-thrombin III and directly binds to active site of factor IIa (Thrombin)

97. A 25-year-old male is brought to the ER due to excessive bleeding after an accident. It is found that he overdosed on warfarin. What is the antidote?
A. Vitamin K
B. Vitamin C
C. Vitamin B12
D. Vitamin D

98. A 32-year old female has a valvular heart disease and takes warfarin, She decided to follow a new diet with lots of green vegetables. How would that affect her?
A. Vitamin K is increased which makes warfarin less efficient
B. Vitamin C is decreased which makes warfarin more efficient
C. Vitamin K is decreased which makes warfarin more efficien
D. Vitamin C is increased which makes warfarin less efficient



95	96	97	98
B	A	A	A

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99. Which of the following is one of the advantages of LMWH?
A. Easily monitor using INT
B. Cheap
C. Safe in pregnancy
D. Fully response to sulfate

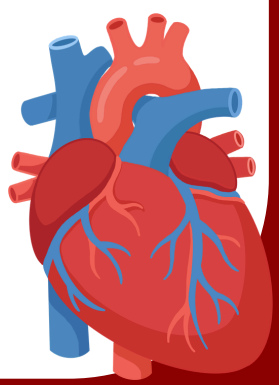
100. What is the antidote for warfarin overdose?
A. Vitamin K
B. Vitamin B12
C. Vitamin D
D. Vitamin

101. A 32-year old female has a valvular heart disease and takes warfarin, She decided to follow a new diet with lots of green vegetables. How would that affect her?
A. Vitamin K is increased which makes warfarin less efficient
B. Vitamin C is decreased which makes warfarin more efficient
C. Vitamin K is decreased which makes warfarin more efficient
D. Vitamin C is increased which makes warfarin less efficient

102. When would we expect to see the maximum anticoagulant effect of warfarin?
A. One day
B. Three days
C. Six hours
D. One week

103. Which of the following will be used in case of heparin overdose?
A. Heparin sulfate
B. Protamine sulfate
C. Fresh frozen plasma
D. N-acetylcysteine (NAC)

99	100	101	102	103
C	A	A	B	B



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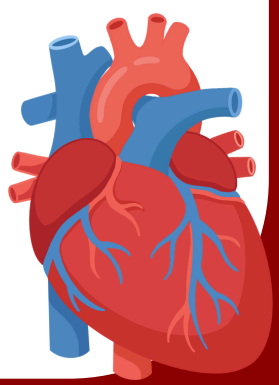
104. If you suspect pneumonia in a patient, auscultation would reveal which of the following?
A. Wheezing
B. Bronchial breath sounds
C. Egophony
D. Friction rub

105. Pneumothorax clinical signs are associated with all of the following except:
A. Hyper resonance to percussion
B. Decreased breath sounds
C. Decreased chest expansions
D. Crepitation's

106. A 30 y/o woman represents with signs and symptoms of pleural effusion. You decided to do diagnostic thoracocentesis. For what —- if the effusion is transudative or exudative?
A. RBCs content
B. Protein and LDH level
C. Cell count
D. WBCs content

107. A 45 years business woman came to ER with cough hemoptysis (tachycardia) tachypnea what is the diagnosis you will rule out?
A. Pleural effusion
B. Acute coronary syndrome
C. Pneumonia
D. Pulmonary embolism

104	105	106	107
B	D	B	A



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108. A man comes into the hospital with a chronic cough and takes an antihypertensive drug (Amlodipine). The patient has no history of cardiac problems or chronic Diseases. He feels a burning feeling in the anterior aspect of his chest. What could be the cause of his cough?

A. GERD
B. Drug-induced cough
C. Chronic bronchitis
D. Asthma

109. All of the following cause chronic cough except:

A. Pneumonia
B. GERD
C. Bronchial Asthma
D. Asthma

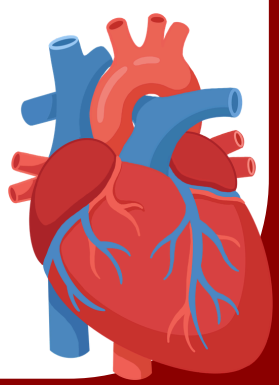
110. A 70 years old man comes to the clinic with fever, cough and dyspnea, no cardiac problems or chronic diseases. He feels burning sensation in his chest, What is the cause of his cough?

A. Lung cancer
B. Bronchial asthma
C. Gastric esophageal reflex disease
D. Chronic bronchitis

111. A man comes into the hospital with a fever, cough, and dyspnea. The patient has no history of cardiac problems or chronic diseases. He feels a burning feeling in the anterior aspect of his chest. What could be the cause of his cough?

A. Bronchial asthma
B. GERD
C. Chronic bronchitis
D. Lung cancer

108	109	110	111
A	A	C	B



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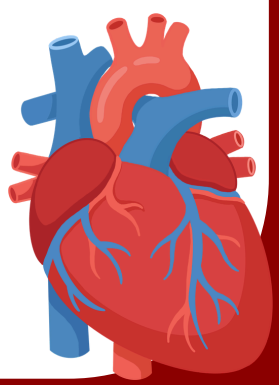
112. Which of the following causes type 2 respiratory failure? (Opiate drugs)
A. Pneumonia
B. Pulmonary embolism
C. Pulmonary edema

113. A 64-year-old male with a known case of COPD and type 2 respiratory failure, PaCO2: 95, was placed on mechanical ventilation. What breathing patterns will be observed upon initial presentation?
A. Irregular irregular RR
B. Pursed lips
C. Deep breathing (kussmaul breathing)
D. Breathing with apnea

114. What is the sign of respiratory failure type 1?
A. Cyanosis
B. Flapping tremor
C. Lower limb edema
D. Chest pain

115 .Posterioanterior in thorax radiogram means?
A. The patient laying on his back
B. The patient is in sitting position
C. The patient is slightly rotated
D. The patient's back is on the x Ray tube



112	113	114	115
A	B	A	D

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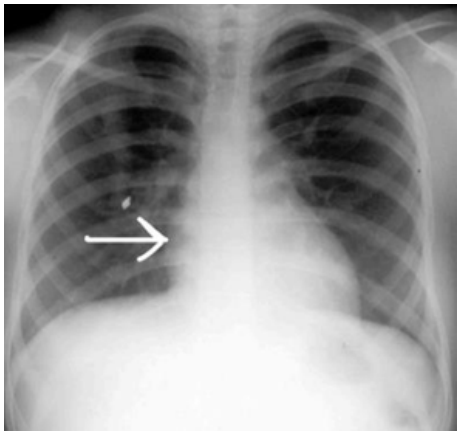
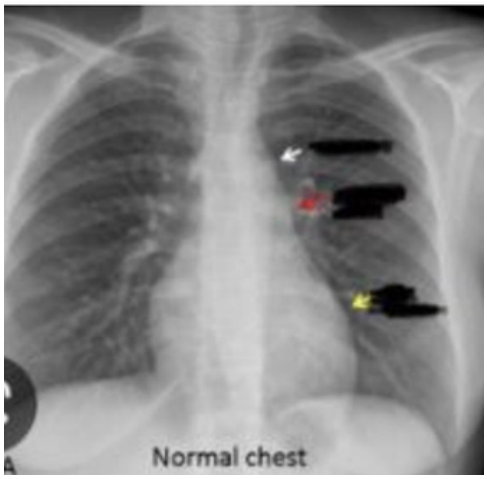
116. Identify the marked structure?

A. Right atrium
B. Pulmonary artery
C. Pulmonary vein
D. Left atrium



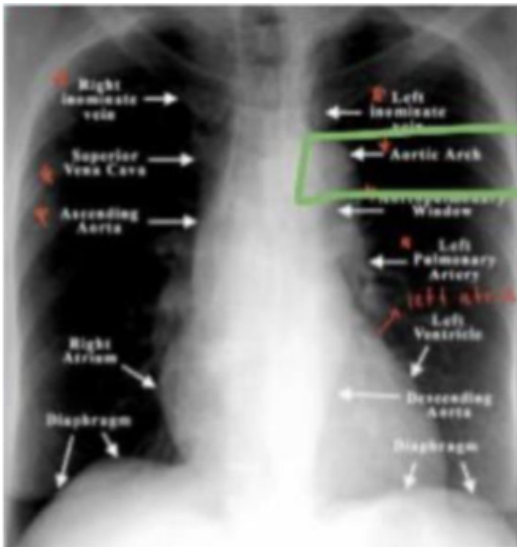
117. Identify the structure marked in the red arrow?

A. Pulmonary artery
B. Pulmonary vein
C. Left atrium
D. Right atrium



118. What is the structure?

A. Aortic arch
B. Right pulmonary artery
C. Right hilum
D. Right atrium



116	117	118
A	A	A

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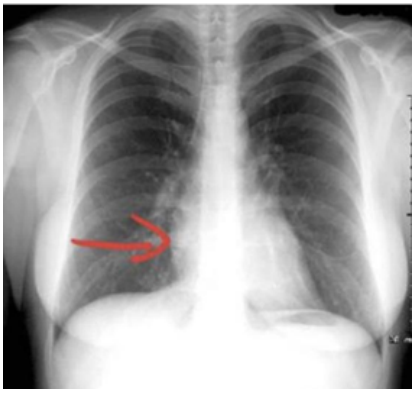
119. Identify the marked structure?

A. Right hilum
B. Right ventricle
C. Right pulmonary artery
D. Right atrium

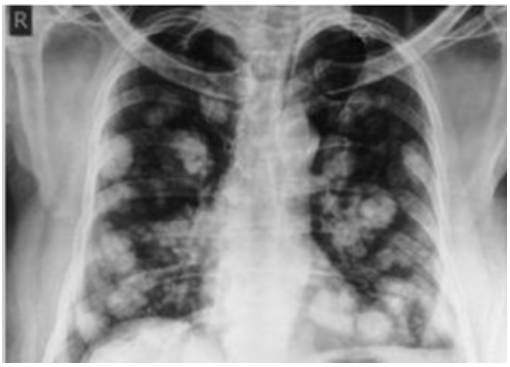


120 .What is the abnormality in this picture?

A. Right side pleural effusion
B. Right side pneumothorax
C. Right side collapse
D. Left side pneumonectomy

121 .This x-ray show?

A. Miliary TB
B. Cannon Ball Metastasis
C. Cavities
D. Lung consolidation



122.What is the abnormality in this picture?

A. Left pleural effusion
B. Left pneumothorax
C. Left collapse
D. Left pneumonectomy

119	120	121	122
D	C	B	A

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123.The hallmark of radiological appearance of lung collapse on chest radiograph:
A. Movement of normal anatomical structure
B. Air Bronchogram sign
C. Heterogeneous opacity with no significant mass effect
D. Contralateral deviation of the mediastinum

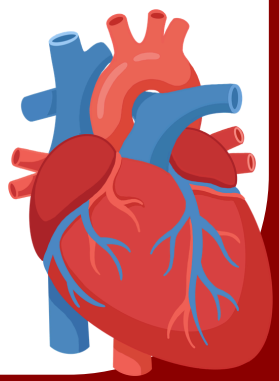
124. In Lung collapse:
A. Volume increases
B. Movement of anatomical structures
C. Air Broncho gram sign
D. Contralateral deviation of the mediastinum

125. In lung collapse, the radiological sign that can be seen is?
A. V shape costophrenic angle
B. Movement of normal anatomic
C. U shape costophrenic angle

126. The radiological sign that is seen with lobar pneumonia is:
A. Air-Broncho gram
B. Shifting of the mediastinal structures
C. U-shaped cost phrenic angle
D. V-shaped costophrenic angle

127. A patient with shortness of breath and, dyspnea visits the doctor. Upon Xray we find the trachea is deviated to the left and consolidation in the right lung. What does this indicate:
A. Collapsed Left Lung
B. Collapsed Right Lung
C. Massive Pneumothorax of the right lung
D. Massive Right lung pleural Effusion

123	124	125	126	127
A	B	B	A	D



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1. Olfactory mucosa is present in?
A. Roof of nasal wall
B. Floor of nasal wall
C. Over Inferior conchae
D. Lower part of medial wall

2. All the following innervate the nasal cavity EXCEPT?
A. Mandibular
B. Maxillary
C. Superior ethmoidal
D. Olfactory

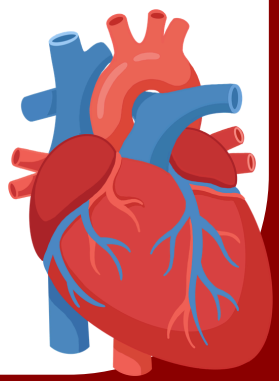
3. What is the most labile parasinus to infections?
A. Sphenoidal
B. Maxillary
C. Frontal
D. Anterior ethmoidal

4. Which of the paranasal sinuses is more liable to get infected?
A. Anterior ethmoidal
B. Frontal
C. Maxillary
D. Sphenoidal

5. A 40-year-old male has an infection in the nasal cavity and also in them maxillary air sinus. This infection is making a mass in the hiatus and blocking it. Where is the block located?
A. Superior meatus
B. Inferior meatus
C. Middle meatus
D. Sphenoethmoidal recess

6. Which of the following bones is present in the nasal septum?
A. Frontal
B. Palatine
C. Vomer
D. Nasal

1	2	3	4	5	6
A	A	B	C	C	c



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7. Which of the following paranasal sinuses open in the sphenoethmoidal recess?
A. Sphenoid
B. Frontal
C. Maxillary
D. Posterior ethmoidal

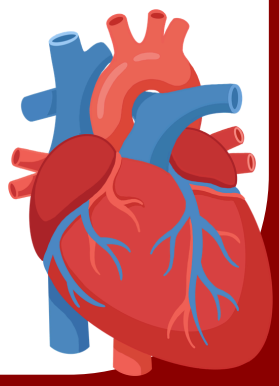
8. A 53-year-old patient has difficulty breathing through the nose. On examination, his physician finds that he has swelling of the mucous membrane of the superior nasal meatus. Which of the following paranasal sinuses is affected?
A. Middle ethmoidal
B. Posterior ethmoidal
C. Frontal
D. Maxillary

9. A 53-year-old patient has difficulty breathing through his nose. On examination, the physician finds increased mucosa in the middle meatus. There is a bulging in which sinus?
A. Maxillary sinus
B. sphenoid sinus
C. Posterior ethmoidal sinus
D. Anterior ethmoidal sinus

10. A young girl is complaining of a dry palate and nose. The lesion is most probably occurring in which ganglion?
A. Submandibular
B. Ciliary
C. Pterygopalatine
D. Otic

11. Pointing to the roof of the nose, what is the special nerve?
A. Olfactory nerve
B. Optic nerve
C. Trigeminal nerve
D. Vagus nerve

7	8	9	10	11
A	B	A	C	A



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1. Contraction of cricothyroid muscle lead to?
A. Increased tension of vocal cords
B. Relaxation of vocal cords
C. Adduction of vocal cords
D. Abduction of vocal cords

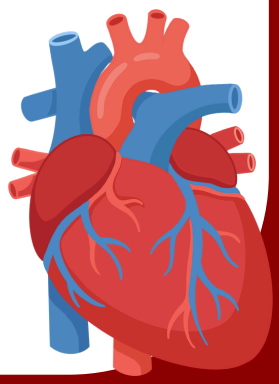
2. Which of the following muscles causes adduction of vocal cord?
A. Posterior coricoarytenoid
B. Lateral coricoarytenoid
C. Cricothyroid
D. All of the above

3. The laryngeal inlet is widened by?
A. Transverse arytenoid muscles
B. Oblique arytenoid muscles
C. Aryepiglottic muscles
D. Thyroepiglottic muscles

4. The narrowest part of the larynx is?
A. Rima epiglottis
B. Laryngeal inlet
C. Rima vestibule
D. Infraglottic cavity

5. During the surgery, if the surgeon accidentally cuts the accompanying nerve with the inferior thyroid artery, which of the following will be affected?
A. Paralysis of Vocalis Muscle
B. Paralysis of Cricothyroid Muscle
C. Decrease in Tension of Vocal Cords
D. Asphyxia and death

1	2	3	4	5
A	B	D	A	A



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6. Which of the following muscles close the laryngeal inlet?
A. Thyroepiglottic muscle
B. Aryepiglottic muscle
C. Cricothyroid muscle
D. Thyroarytenoid muscle

7. A 53-year-old man underwent thyroid surgery and developed hoarseness in his voice due to nerve damage. Which of the following nerves most likely caused this damage?
A. External Laryngeal
B. Internal Laryngeal
C. Recurrent Laryngeal
D. Inferior Laryngeal

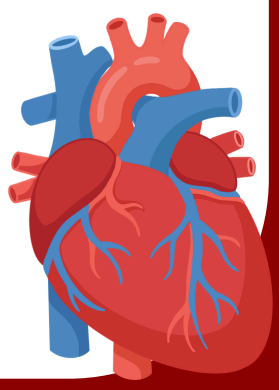
8. Which of these Nerves pierces the thyrohyoid membrane?
A. Superior Laryngeal
B. External Laryngeal
C. Internal Laryngeal
D. Recurrent Laryngeal

9. (Case) A patient with a benign thyroid tumor underwent thyroidectomy and then lost the ability to abduct his vocal cords. Which of the following muscles is affected?
A. Posterior Cricoarytenoid
B. Lateral Cricoarytenoid
C. Cricothyroid
D. Transverse Arytenoid

10. Which muscle is responsible for widening the laryngeal inlet?
A. Transverse Arytenoid
B. Thyroepiglottic
C. Oblique Arytenoid
D. Aryepiglottic

11. Which sensory nerve supplies the area above the vocal cords?
A. Internal laryngeal
B. Recurrent laryngeal
C. External laryngeal
D. Glossopharyngeal

6	7	8	9	10	11
B	C	C	A	B	A



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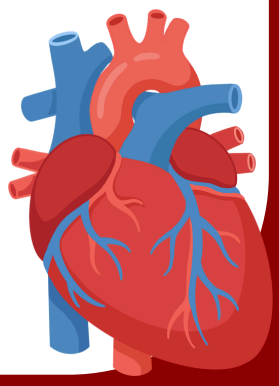
12. Which of the following nerves pierces thyrohyoid membrane?
A. Internal laryngeal
B. External laryngeal
C. Recurrent laryngeal
D. Superior laryngeal

13. A surgery was performed on the superior thyroid artery, and it is suspected that nerve damage occurred. What would result from this nerve damage?
A. Decreased Tension
B. Increased Vocal Cord Length
C. Enhanced Pitch Control
D. Complete Vocal Cord Paralysis

14. A surgery was performed on the inferior thyroid artery, and it is suspected that nerve damage occurred. What would this nerve damage cause?
A. Loss of Sensation Above the Vocal Cords
B. Paralysis of Vocalis Muscle
C. Decrease in Tension of Vocal Cords
D. Inability to Produce High-Pitched Sounds

15. A patient is undergoing surgery to remove a goitre. The surgeon should be cautious to avoid damaging which of the following nerves?
A. Internal laryngeal nerve
B. External laryngeal nerve
C. Superior laryngeal nerve
D. Recurrent laryngeal nerve



12	13	14	15
A	A	B	D

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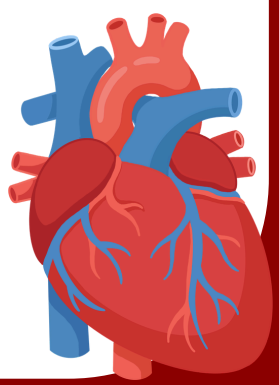
1. Left bronchus?
A. Wider than right
B. Vertical than right
C. It’s bronchi divide after it’s enter the lung
D. It’s bronchi divide before it’s enter the lung

2. Horizontal fissure begins at?
A. 3rd costal cartilage
B. 4th costal cartilage
C. 5th costal cartilage
D. 6th costal cartilage

3. Which lung lobe has 2 bronchopulmonary segment?
A. Middle lobe of right lung
B. Upper lobe of right lung
C. Middle lobe of left lung
D. Upper lobe of left lung

4. In Bronchopulmonary segments, which of the following structures is intersegmental?
A. Segmental branch of pulmonary artery
B. Tributary pulmonary vein
C. Segmental bronchus
D. Lymph vessels

5. The lower border of the plura cuts the mid-clavicular line at the level of?
A. 6th
B. 7th
C. 8th
D. 9th



1	2	3	4	5
C	B	A	B	C

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6. Regarding the bronchopulmonary segment. Which of the following structures is considered intersegmental?
A. Segmental artery
B. Segmental vessel
C. Segmental vein
D. Segmental bronchus

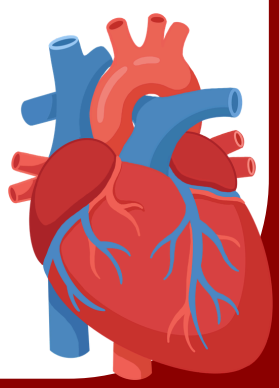
7. Which nerve supplies the diaphragmatic pleura?
A. Phrenic & intercostal
B. Intercostal
C. Vagus & phrenic
D. Recurrent laryngeal

8. At which thoracic spine the oblique fissure cut at the posterior of lung?
A. Second
B. Third
C. Fourth
D. Fifth

9. Which of the following lymph node drains the carnia of the trachea?
A. Tracheobronchial
B. Bronchopulmonary
C. Paratracheal
D. Pulmonry

10. Which of the following lymph nodes drains the carina of the trachea?
A. Bronchopulmonary
B. Tracheobronchial
C. Hilar lymph nodes
D. Bronchopulmonary

6	7	8	9	10
C	A	B	A	B



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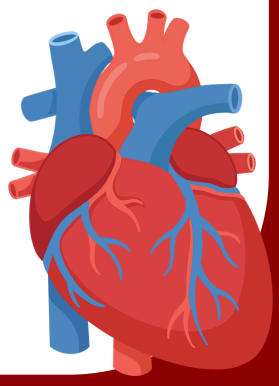
11. Why do aspirated bodies go into the right main bronchus ?
A. Beacuse the right bronchus is more vertical than the left bronchus
B. Beacuse the right bronchus is narrower than the left bronchus
C. Beacuse the right bronchus is more oblique than the left bronchus
D. Beacuse the right bronchus is longer than the left bronchus

12. The lymph nodes present at the carina of the trachea are?
A. Bronchopulmonary
B. Tracheobronchial
C. Paratracheal
D. Pulmonary

13. Regarding the bronchopulmonary segment, which of the following is intersegmental?
A. Bronchial artery
B. Segmental bronchi
C. Tributary pulmonary vein
D. Pulmonary artery

14. The transverse fissure of the right lung ends anteriorly at the level of the costal cartilage number?
A. Fourth
B. Fifth
C. Sixth
D. Third

15. What nerve supplies the mediastinal pleura?
A. Phrenic nerve
B. Intercostal nerve
C. vagus nerve
D. bronchial nerve



11	12	13	14	15
A	B	C	A	A

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16. An infant aged six months admitted to paediatric emergency , with dehydration due severe attack of gastroenteritis , after clinical examination, the physician described your urgent intravenous fluid only EJV was available to give , after few minutes the physician noted severe dyspnoea and respiratory distress of infant , X-ray showed pneumothorax of same side of injury, which part of pleura was suspected?

A. Costal
B. Visceral
C. Cervical
D. Mediastinal

1. Which one of the following is related to the medial side of LT lung?

A. Esophagus
B. IVC
C. Azygos arch
D. LT atrium

2. The trachea is in medial relations with?

A. Anterior superior lobe of left lung
B. Posterior inferior lobe of right lung
C. Posterior inferior lobe o left lung
D. Posterior superior lobe of right lung

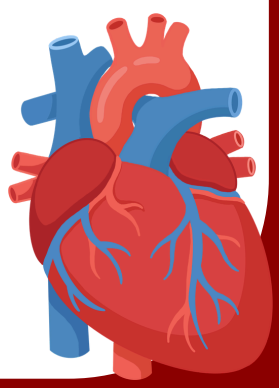
3. Which of the following is related to the medial side of the left lung?

A. Superior Vena Cava
B. Azygos arch
C. Descending aorta
D. Inferior Vena Cava

4. Which of the following structures is related to the medial side of the left lung ?

A. Left atrium
B. Arch of aorta
C. SVC
D. Azygos

16	1	2	3	4
C	A	D	C	B



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5. Which of the following is related to the medial surface of the right lung ?
A. Right ventricle
B. Left atrium
C. Arch of azygous vein
D. Descending aorta

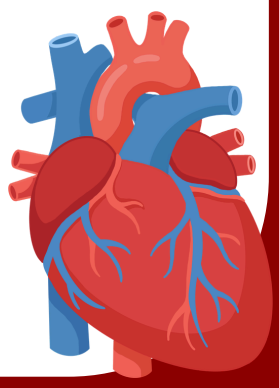
6. What is related to the lower medial surface of the left lung?
A. Azygos arch
B. Left atrium
C. Esophagus
D. Trachea

7. What is found on the medial side of the left lung?
A. Azygos vein
B. Arch of the aorta
C. Left atrium
D. SVC

1. Which of the following is a feature of the trachea?
A. Submucosa contains numerous seromucus glands
B. C-shaped fibrocartilage
C. Submucosa contains smooth muscles
D. None

2. Regarding the lung, which of the following statements is correct?
A. The intrapulmonary bronchi are deficient of cartilage
B. Alveolar sacs and ducts are parts of the conducting portions of respiratory system
C. The respiratory bronchioles are lined by pseudo stratified columnar ciliated epithelium
D. The alveoli intercommunicate through alveolar pores

3. Which of the following is CORRECT regarding the terminal bronchiole?
A. They contain Clara cells
B. They have alveoli on their walls
C. They are lined with stratified squamous epithelium
D. They are the site of gas exchange



5	6	7	1	2	3
C	C	B	A	D	A

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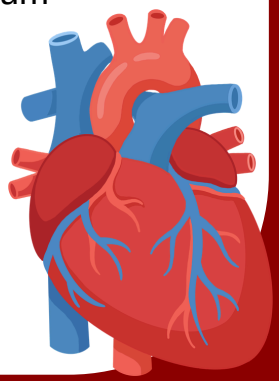
4. All of the following statements about the respiratory epithelium are true, except:
A. The respiratory epithelium has Type I pneumocytes which secrete surfactant
B. The respiratory epithelium has cilia
C. The respiratory epithelium covers some of the conducting parts of the respiratory system
D. The respiratory epithelium plays a role in filtering and humidifying inhaled air

5. The bronchioles differ from the bronchi in all of the following, EXCEPT?
A. Absence of cartilage
B. Presence of smooth muscles and elastic fibers
C. Epitheliumtype
D. Presence of Clara cells

6. Which of the following cells function as a blood-air barrier?
A. Pneumocytes I
B. PneumocytesII
C. Gobletcell
D. Clara cell

7. What are the changes in respiratory epithelium occur during the passage from the trachea to the terminal bronchioles?
A. Simple squamous epithelium changes to pseudostratified columnar ciliated epithelium
B. Simple cuboidal partially ciliated epithelium changes to simple columnar epithelium
C. Pseudostratified columnar ciliated epithelium changes to ciliated simple cuboidal epithelium
D. Simple columnar ciliated epithelium changes to squamous epithelium



4	5	6	7
A	B	A	C

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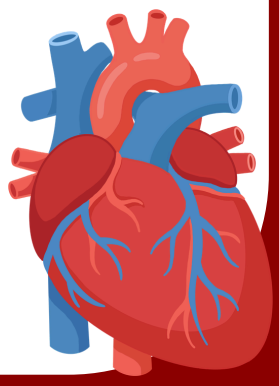
8. What is the function of respiratory epithelium?
A. Transport of the oxygen from capillary to alveoli
B. Trappingtheairparticlesandclearit
C. warmingtheair
D. Immune Response

9. Which of the following have ciliated simple columnar epithelium and are devoid of cartilage and glands?
A. Alveolar ducts
B. Bronchioles
C. Secondarybronchus
D. External bronchi

10. Which of the following does not contain smooth muscle?
A. Alveoli
B. Trachea
C. Bronchi
D. Bronchioles

11. The capillaries in inter-alveolar septa are?
A. Continuous non-fenestrated
B. Discontinuousfenestrated
C. Separatedfromaironlybyitsownepitheliumandlamina
D. Fenestrated to allow the air from the alveoli

12. Intrapulmonary bronchi differs from extra pulmonary bronchi in which of the following?
A. Type of epithelium
B. Contain shared cartilage
C. Havemuscularissmoothmuscle
D. Presence of submucosal glands



8	9	10	11	12
B	B	A	A	A

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1. Which of the following congenital anomalies causes diaphragm hernia?
A. Tracheo-esophageal fistula
B. Extra lung lobe
C. Lung cyst
D. Hypoplasia of the lung

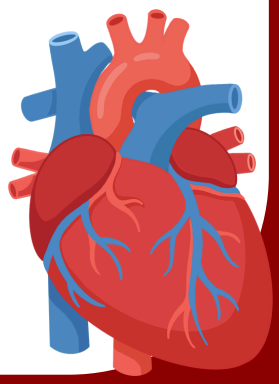
2. During lung development, repeated division of bronchi and alveoli continues till the age of?
A. 4 Years
B. 5 Years
C. 8 Years
D. 1 Year

3. During maturation of the lung, the pseudoglandular stage has?
A. Terminal bronchioles
B. Respiratory bronchioles
C. Alveolar sac
D. Alveolar duct

4. Surface area of alveoli continues to increase until?
A. Childhood
B. Adulthood
C. 8 Years
D. Before birth

5. Which of the following is the most common congenital anomaly of the trachea and esophagus?
A. Esophageal atresia and tracheoesophageal fistula
B. H-type tracheoesophageal fistula
C. Esophageal atresia
D. Isolated esophageal atresia without fistula

1	2	3	4	5
D	C	A	B	A



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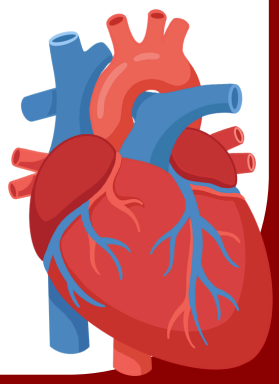
6. At the pseudoglandular stage, which of the following is present?
A. Alveolar ducts
B. Alveoli
C. Respiratory bronchioles
D. Terminal bronchioles

7. During lung growth, at which stage does the surface area of alveoli increases?
A. Adult stage
B. Before birth
C. 8 Years
D. Childhood

8. Regarding the embryonic origin of the respiratory system, the epithelial lining originates from?
A. Splanchnic layer
B. Mesoderm
C. Ectoderm
D. Endoderm

9. The most common congenital anomaly in the development of the lung is?
A. Esophageal atresia
B. Hypoplasia
C. Agenesis
D. Tracheo-esophageal fistula

10. The epithelium of the respiratory tract comes from which of the following?
A. Endoderm
B. Ectoderm
C. Splanchnic mesoderm
D. Somatic mesoderm



6	7	8	9	10
D	A	D	D	A

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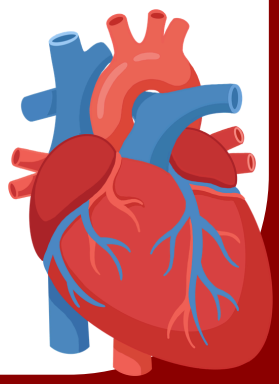
11. Which of the following diseases is caused by incomplete separation of tracheoesophageal septa ?
A. Esophageal atresia
B. Trachea atresia
C. Tracheoesophageal fistula
D. Agenesis

12. Which of the following structures is seen in pseudoglandular phase?
A. Terminal bronchioles
B. Respiratory bronchioles
C. Alveolar ducts
D. Alveolar sacs

13. During maturation of the lung the pseudoglandular stage contains?
A. Respiratory bronchioles
B. Terminal bronchioles
C. Alveolar sacs
D. Alveolar ducts

1. During surfactant synthesis, which of the following is found extra-cellular in alveolar lumen and repair SP-A & SP-B for its formation and stabilization?
A. DPPC
B. Type II alveolar cells
C. Lamellar body
D. Tubular myelin

2. During the lung maturity test a pregnant woman's L/S ratio is determined to be 1:1, what can we conclude from this result?
A. Decreased risk of respiratory distress syndrome
B. No specific significance is concluded
C. Insufficient production of surfactant
D. Adequate lung maturity



11	12	13	1	2
C	A	B	D	C

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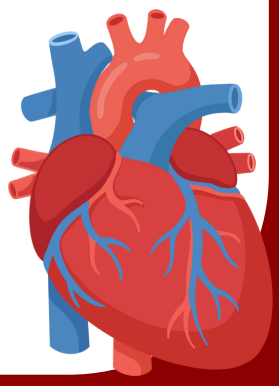
3. A baby is born at 37 weeks with normal pulmonary function. A pulmonary test was performed; what is the likely finding?
A. Phosphatidylglycerol is going to make up 90% of surfactant
B. Presence of Phosphatidylglycerol
C. L/S ratio is below 2
D. Sphingomyelin is the prominent surfactant

4. A baby born at 37 weeks has a good respiratory function, which of the following can indicate that ?
A. L/S ratio < 2
B. Surfactant contains phosphatidylglycerol
C. Surfactant contains sphingomyelin
D. L\S ratio = 1

5. A full-term newborn was brought to the hospital with pale skin and peripheral cyanosis. A genetic analysis was performed and revealed a hydrophobic protein deficiency causing respiratory distress. What is the missing protein?
A. SP-B
B. SP-D
C. Surfactant
D. Dipalmitoyl phosphatidyl choline

6. Which surfactant protein is responsible for the synthesis of tubular myelin and its deficiency causes infant disease?
A. SP B
B. SP D
C. SP A
D. SP C



3	4	5	6
B	B	A	A

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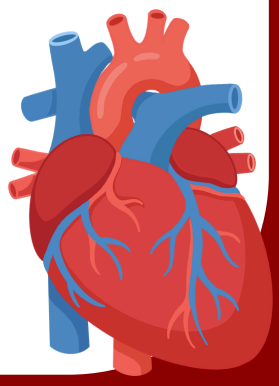
7. A 32 year old woman came to assess her L/S ratio to assess fetal lung maturity. Her L/S ratio was less than 1. What was the expected gestational age?
A. 38 weeks
B. 35 weeks
C. 32 weeks
D. 30 weeks

8. A sensitive lung maturity test that includes automated counting of phospholipid granules from the amniotic fluid is?
A. L/S
B. Lamellar body count
C. Phosphatidylglycerol
D. D.Surfactant/albumin ratio

9. During the synthesis of surfactant, it is secreted into the alveolar lumen and needs SP-A and SP-B for its synthesis and stabilization?
A. Lamellar bodies
B. Type II pneumocyte
C. Tubular myelin
D. DPPC

10. The main component of surfactant?
A. dipalmitoyl phosphatidyl choline
B. Phosphatidylinositol
C. Phosphatidylglycerol
D. Protein

11. A newborn baby was born 9 weeks prematurely. Her aspiration is low, and her lips and fingers are cyanotic (blue) from low oxygen levels in her arterial blood. Her condition is related to a problem with the synthesis of lipoprotein. What is the main constituent of this lipoprotein?
A. Dipalmitoyl phosphatidyl chloride
B. Phosphatidic acid
C. Angiotensin-converting enzyme
D. Cholesterol



7	8	9	10	11
D	B	C	A	A

MidNight RS - MED23

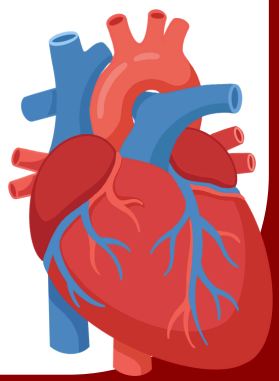
12. The lung has an ability to protect its systemic vasculature from vasoactive substances by?
A. Release of histamine
B. Synthesis of surfactant
C. Release of bradykinin and norepinephrine
D. Synthesis and release of norepinephrine

1. Which of the following is the main source of CO2 production?
A. Incomplete oxidation of CHO and Fats
B. Oxidative metabolism
C. Oxidation of sulfa-containing amino acids
D. Anaerobic respiration of glucose

2. (Case) PH: 7.48, CO2: Low?
A. Asthmatic patient
B. Anxiety hyperventilation
C. Severe diarrhea
D. Vomiting

3. Which of the following is found in high concentration and is therefore the most effective buffer system at physiological pH?
A. Phosphate buffer
B. Bicarbonate buffer
C. Hemoglobin
D. Plasma protein buffer

4. Which of the following factors shift the oxyhemoglobin curve to the left?
A. Acidosis
B. Hyperthermia
C. Exercise
D. Hyperventilation



12	1	2	3	4
A	B	B	B	D

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5. During cell metabolism, the cells produce CO2, which then combines with water to form carbonic acid. What is the role of hemoglobin in this situation?
A. Clear out excess acidosis by taking H in the imidazole ring
B. Clear excess alkalosis by releasing H
C. Clear out excess acidosis with the help of leucine
D. Clear excess acidosis by releasing H from its carboxyl terminal

6. A patient with hypoventilation is most likely to have which of the following?
A. High plasma bicarbonate
B. Hypocapnea
C. Cool extremities
D. Hypercapnea

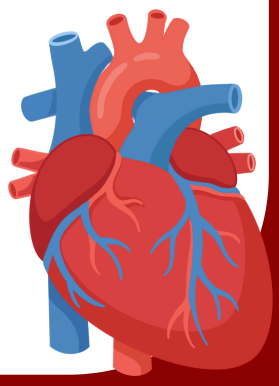
7. Which of the following is the main source of CO2 production?
A. Incomplete oxidation of CHO and Fats
B. Oxidative metabolism
C. Oxidation of sulfa-containing amino acids
D. Anaerobic respiration of glucose

8. Which of the following is the immediate effect of high-altitude climbing?
A. Hyperventilation
B. Polycythemia
C. Acidosis
D. Increased 2,3-DPG

9. What is the mechanism of the hemoglobin buffer system?
A. Chloride shift
B. Neutralize excess acidity via uptake the H by imidazole histidine ring
C. Neutralize excess alkalinity via uptake the H by imidazole histidine ring
D. Shift the curve to the right

10. Why is venous blood more acidic than arterial blood?
A. Deoxygenated hemoglobin has more affinity for H+
B. No carbonmyl enzyme
C. Oxygenated hemoglobin has more affinity for H+
D. Less oxygen

5	6	7	8	9	10
A	D	B	A	B	A



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11. A 35-year-old diabetic female presents with abdominal pain and vomiting after attending a party. Her blood sugar is 600 mg/dl (normal <140 mg/dl). The values obtained from the ABG were: pH=7.03 (normal 7.35-7.45), PCO2=36 mm Hg (normal 35-45 mm Hg), HCO3-=14 mEq/L (normal 22-26 mEq/L). How will the body compensate?

A. Increase H+ to buffer HCO3-
B. Hypoventilate
C. Decrease HCO3- reabsorption
D. Hyperventilate

1. Which one of the following is negative to atmospheric pressure?

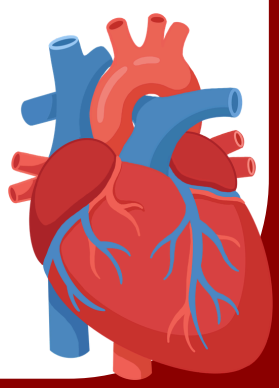
A. Intra-alveolar Inspiration
B. Intra-alveolar Expiration
C. Intra-alveolar pressure equals zero
D. None of the above

2. When is sternocleidomastoid used during breathing?

A. Deep expiration
B. At rest
C. Deep inspiration
D. At sleep

3. During forced expiration what is the contracted muscle?

A. Sternocleidomastoid
B. Diaphragm
C. Abdominal muscles
D. External intercostal muscles



11	1	2	3
D	A	C	C

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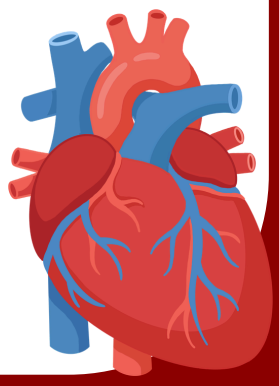
4. Which of the following initiates the tidal expiration?
A. Elastic recoil of the lung
B. Intercostal muscle contraction
C. Contraction of the diaphragm
D. Forced blowing to excrete air from lung

5. During expiration, the intra-alveolar pressure is?
A. Greater than atmospheric pressure
B. Lower than atmospheric pressure
C. Equal to atmospheric pressure
D. Equal to intrapleural pressure

6. When you used the maximum adenosine triphosphate in respiration?
A. Tidal inspiration
B. Tidal expiration
C. Forced inspiration
D. Quite inspiration

7. Which of the following statements is true?
A. During inspiration Intra-alveolar pressure is less than atmospheric pressure
B. Inspiration decrease venous return
C. In expiration Intra-alveolar pressure is less than atmospheric pressure
D. During inspiration intrpleural pressure is positive

8. Which of the following is responsible for increased lymphatic return during inspiration?
A. Decreased transpulmonary pressure
B. Negative intraalveolar pressure
C. Abdominal muscle contraction
D. Increased negativity of intrathoracic pressure



4	5	6	7	8
A	A	C	A	D

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9. Contraction of which of the following muscles is responsible of the major lung compliance that increases lung volume ?
A. Sternocleidomastoide muscle
B. External intercostal muscles
C. Diaphragm
D. Internal intercostal muscles

10. What is the primary force behind expiration during normal breathing cycle?
A. Abdominal muscles contraction
B. Elastic recoil
C. Increase intrapleural pressure
D. Transmural pressure gradient

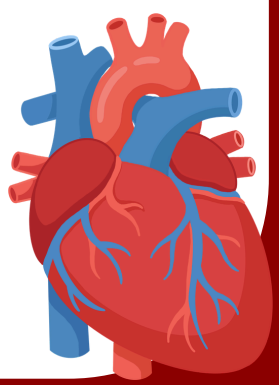
1. Lung compliance increased in?
A. Pulmonary Fibrosis
B. Pulmonary edema
C. Pulmonary emphysema
D. Surfactant deficiency

2. Deficiency in pulmonary surfactant leads to?
A. Decrease surface tension
B. Decrease lung compliance
C. Decrease filtration in alveoli
D. Increase FEC

3. During inspiration?
A. Expend less energy than expiration
B. Increase transmural pressure
C. Intra-pleural pressure increases
D. Venues return decrease

4. Regarding surfactant, which of the following is true?
A. It reduces surface tension
B. It increases work of breathing
C. It increases alveolar collapse
D. It causes bronchoconstriction

9	10	1	2	3	4
C	A	C	B	B	a



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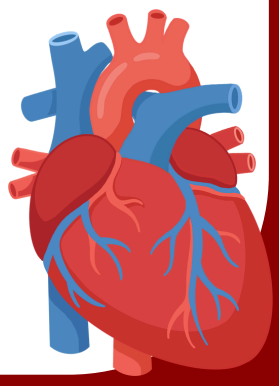
5. Which of the following increases lung compliance?
A. Old age
B. Emphysema
C. Lung fibrosis
D. Obesity

6. What is the role of surfactant in breathing in a healthy subject?
A. Enhance lung immunity
B. Prevent alveolar collapse
C. Facilitate gas diffusion
D. Reduce lung expansion

7. Which of the following statements is true regarding intrapleural pressure?
A. Increases during inspiration
B. Equal to the atmospheric pressure
C. Sub-atmospheric pressure
D. Equals to the alveolar pressure

8. Which of the following prevent lung collapse during expiration?
A. Surfactant
B. Increase negativity of intrapleural pressure during inspiration
C. Increase negativity of intra alveolar pressure during inspiration
D. Transmural pressure

9. What prevents fluid from moving from the pulmonary capillaries into the alveoli?
A. The repelling forces of the surfactant
B. Negative intrapleural pressure
C. Negative transthoracic pressure
D. Pulmonary Hydrostatic pressure



5	6	7	8	9
A	B	C	A	A

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10. Which of the following prevent leakage of fluid from blood capillaries to Alveoli?
A. Repull surfactant force
B. Negativity of intrathoracic pressure
C. Negativity of intrapleural pressure
D. Hydrostatic pressure

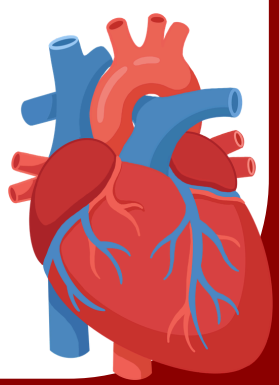
11. What prevents fluid from leaking out of the capillaries into the alveoli?
A. Hydrostatic pressure
B. Surfactant
C. Intra-pulmonary pressure
D. Intra-thoracic pressure

12. A 70-year-old man who has been a heavy smoker for nearly 50 years with possible harm to type II alveolar cells. Which of the following would probably decrease?
A. Lung compliance
B. Filtration into the alveoli
C. Work of breathing
D. Surface tension in the alveoli

13. An 18 years old man presents to the emergency department after a stab wound to the chest. He is combative and with severe shortness of breath. What will happen to him?
A. Intrapleural pressure increased with possible lung collapse
B. No healing
C. Increase of intrathoracic pressure
D. Interalveolar pressure increased with possible lung collapse

14. A patient came in from a car accident and had a crush injury, he is presented with shallow rapid breathing, and chest pain. What is the most likely situation?
A. Increased intrapleural pressure and right lung collapse
B. Increased intrapleural pressure and no effect on the right lung
C. Increased intra alveolar pressure
D. Overinflation of the lung

10	11	12	13	14
A	B	A	A	A



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1. Which of the following, if reduced, will decrease the rate of diffusion of the gas?
A. Molecular size
B. Fibrosis
C. Surface area of alveoli
D. Thickness of alveolar membrane

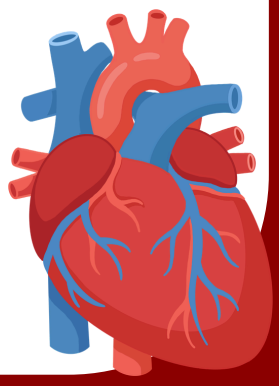
2. Fick's law of diffusion is regarding?
A. Diffusion and surface area
B. Diffusion and pore diameter
C. Diffusion and water pressure
D. All of the above

3. Which of the following factors causes increased alveolar PCO2?
A. Increased respiratory membrane thickness
B. Increased oxygen concentration
C. Severe bronchoconstriction
D. Decreased lung compliance

4. Compared to the base of the lung, in a person who is in a recumbent position, the apex of the lung has?
A. Higher ventilation rate
B. Higher perfusion rate
C. Higher ventilation/perfusion (V/Q) ratio
D. The same V/Q ratio

5. What occurs as a result to hyperventilation?
A. Increased PO2 increased PCO2
B. Decreased PO2 increased PCO2
C. Increased PCO2 decreased PH
D. Decreased PCO2 increased PH

1	2	3	4	5
C	A	C	D	D



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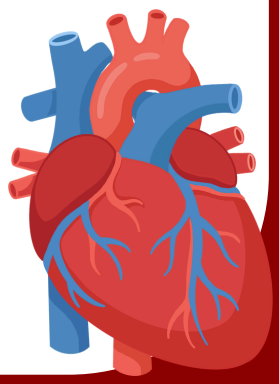
6. What is the pressure of CO2 in the lungs?
A. 40
B. 100
C. 90
D. 50

7. Why arterial blood shows increased oxygen saturation than venous blood ?
A. Due to increased PaO2 in arterial blood more than venous blood
B. Due to increased PaCO2 in arterial more than venous blood
C. Due to decreased amount of hemoglobin in venous blood
D. Due to increased temperature in arterial blood increase oxygen saturation of hemoglobin

8. What happens to a patient who smokes that makes gas diffusion more difficult?
A. Iron deficiency anemia
B. Increasing thickness of respiratory membrane
C. Increase red blood cell
D. Increase surface area

9. Hypothetically if drug X blocks the alveolar cells ATP synthesis what will happen to the diffusion rate?
A. It will affect the diffusion on CO2 rather than O2
B. No effect on the diffusion rate
C. Decrease the diffusion rate
D. Increase the diffusion rate

10. Someone has an obstruction in the bronchus, what is the expected PO2 in the capillaries?
A. Equal to the partial pressure of atmospheric
B. Equal to normal arterial PO2
C. More than PO2 of inspired air
D. Equal to venous PO2



6	7	8	9	10
A	A	B	B	D

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1. Why is normal adult hemoglobin considered an efficient O2 carrier?
A. Hb has changeable affinity to O2 according to bioavailability
B. Due to highest affinity of binding to O2 compared to other gasses
C. Due to linear relationship between PaO2 and % hemoglobin saturation
D. Due to the stability of Hb-O2 binding

2. Carboxyhemoglobin in the blood indicates that hemoglobin (Hb) is combined with?
A. Carbon dioxide
B. Oxygen
C. Carbon monoxide
D. Nitrogen

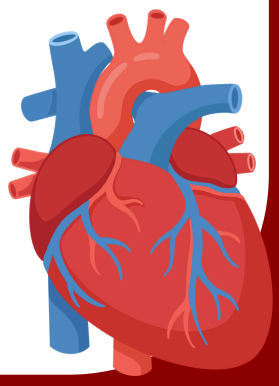
3. Regarding tissue hypoxia, which of the following is true?
A. Left to right shunt can cause hypoxia
B. Occur only when oxygen saturation low
C. Carbon monoxide can cause hypoxia
D. Is unlikely to happen in Polycythemia

4. What is the most common form of oxygen carried in the blood?
A. Physically dissolved in plasma
B. Bound to hemoglobin
C. Bound to plasma protein
D. As bicarbonate

5. Increased acidity (lower pH) in the blood is likely to have which of the following effects?
A. Decreased oxygen delivery to the tissue
B. Increased affinity of HB to oxygen
C. Increased oxygen delivery to the tissue
D. Decreased affinity of HB to CO2

6. Why is hemoglobin the suitable carrier of 02 in human blood?
A. Greater affinity for O2 than for CO
B. Curve shift to the left as a result of alkaline pH
C. Releases most of its content when PO2 between 5 and 20 mmHg
D. O2 binds loosely and reversibly with hemoglobin

1	2	3	4	5	6
A	C	C	B	C	D



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1. Most CO2 is transported in blood as?
A. Carbamino/hemoglobin
B. Bound to reduced hemoglobin
C. Dissolved in plasma
D. Bicarbonate

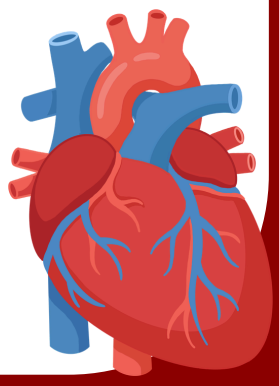
3. What is the percentage of carbon dioxide in plasma?
A. 80%
B. 40 %
C. 10%
D. 20 %

4. CO2 is mainly transported in the blood in the form of?
A. Dissolved in plasma
B. Bind to proteins
C. Carboxy-Hb
D. Bicarbonate

5. The concentration of CO2 is highest in which of the following?
A. Bicarbonate
B. Dissolved in plasma
C. Carbamino acid
D. Oxygenated hemoglobin

6. Which of the following statements is true about CO2?
A. It's more soluble in blood than oxygen
B. Its dissociation curve is sigmoidal in shape
C. Tidal CO2 adds 20 cc to arterial blood
D. None of the above

1	3	4	5	6
D	C	D	A	A



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1. Which receptors are located on the alveolar walls, close to the pulmonary capillaries?
A. J receptors
B. Proprioceptors
C. Stretch receptors
D. Irritant receptors

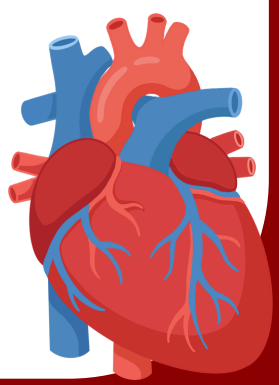
2. Which of the following won't produce apnea?
A. Exercise
B. Increase blood pressure
C. Signal from cerebral cortex
D. Swallowing

3. Hyperinflation of the lung can result in which of the following ?
A. Contraction of the diaphragm
B. Stimulation of the ventral respiratory neurons
C. Sneezing and coughing
D. Stimulate vagal impulses to the medullary centers

4. A patient is experiencing hyperventilation. Which of the following receptors is not normally active during quiet breathing but becomes active to help
decrease ventilation?
A. Ventral respiratory group
B. Dorsal respiratory group
C. Apneustic center
D. Pneumotaxic Center

5. Which of the following limits inspiration?
A. Ventral respiratory group
B. Lung stretch receptors
C. Irritant receptor
D. Central chemoreceptor

1	2	3	4	5
A	A	D	A	B



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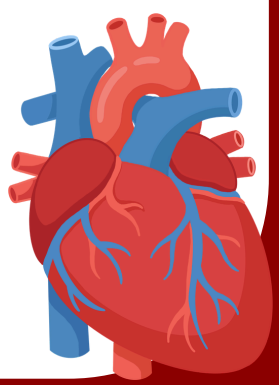
6. Which of the following neurons limit the extent of lung inflation?
A. Stretch lung receptor
B. Irritant receptor
C. J receptors
D. Chemoreceptors

7. Which pattern occurs in mid-pontine transection with the vagi intact?
A. Tachypnea
B. Normal respiratory breathing
C. Apneustic breathing
D. Deep and slow respiration

8. Which signal is released to limit lung inflation ?
A. Lung stretch receptor
B. Lung irritant receptor
C. Arterial baroreceptor
D. Atrial receptor

9. A patient with a stroke results in blocking blood supply from the medulla center , which of the following will be the outcome ?
A. Immediate cessation of breath
B. Apneustic breathing
C. Ataxia breathing
D. Irregular breathing

10. A 24-year-old male was playing American football and collided head-to-head with another player. He was brought to the ER with cyanosis and no respiration, requiring ventilation support. Which of the following spinal levels is most likely affected?
A. C1
B. C5
C. C8
D. T2



6	7	8	9	10
A	D	A	A	A

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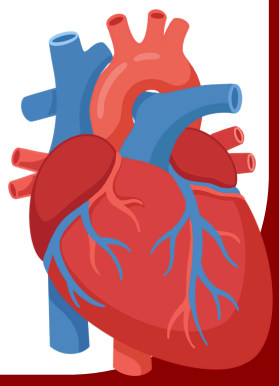
11. Which reflex protects the respiratory tract during coughing?
A. Pharyngeal receptor
B. Irritant receptor
C. Central receptor
D. Peripheral receptor

12. A 54 year old man with a history of hypertension he missed to receive his anti hypertensive for days , but there was a decrease in the pressure noticed. What cause this decrease in pressure?
A. Cerebral cortex
B. Carotid body
C. Arterial baroreceptor
D. Proprioreceptor

13. What is the role of the neurons in the ventral respiratory group?
A. Regulate tidal volume
B. Control rhythmic breathing
C. Affect accessory muscles of respiration
D. Mediate gas exchange in the lungs

1. Bronchiectasis causes?
A. Resorption atelectasis
B. Compression atelectasis
C. Contraction atelectasis
D. Restrictive airway disease

2. Which of the following diseases cause a decreased FEV1/FVC ratio?
A. Chronic Bronchitis
B. Pneumoconiosis
C. Acute interstitial lung disease
D. Sarcoidosis



11	12	13	1	2
B	C	C	A	A

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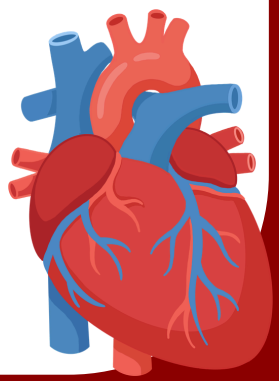
3. A patient is in a car accident and suffers major trauma in the mediastinum and chest wall that leads to hemothorax. Many major arteries are damaged and the patient is unable to catch his breath. What is the cause of this?
A. Resorption Atelectasis
B. Contraction Atelectasis
C. Compression Atelectasis
D. Restrictive lung disease

4. (Case) A child came into the ER, after aspiring a strawberry that has lodged into his right bronchi, what is the likely result?
A. Contraction atelectasis
B. Constrictive atelectasis
C. Resorption atelectasis
D. Pneumothorax

5. (Case) A patient came in with interstitial fibrosis. What may be the result of the pulmonary function test?
A. FVC reduced
B. TLC normal
C. FEV1/FVC decreased
D. TLC increased

6. (Case) A man was stabbed in the right 5th intercostal space at the midaxillary level. X-ray showed air in the pleura. What's the most probable complication of the underlying case?
A. Atelectasis
B. Pneumonia
C. Congestive heart failure
D. Pneumoconiosis



3	4	5	6	7
C	C	A	A	B

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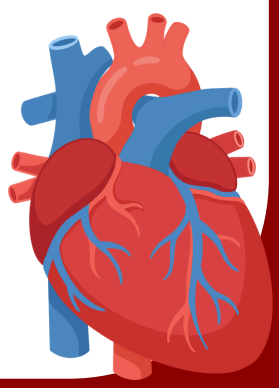
7. (Case) Hemothorax causes?
A. Contraction atelectasis
B. Compression atelectasis
C. Resorption atelectasis
D. Pneumothorax

8. A 6 year old child aspirates a peanut and comes into the ER with shortness of breath, dyspnea, and light-headedness. What is the cause of his symptoms?
A. Compression Atelectasis
B. Contraction Atelectasis
C. Resorption Atelectasis
D. Pneumothorax

9. A patient is in a car accident and suffers major trauma in the mediastinum and chest wall. Many major arteries are damaged and the patient is unable to catch his breath. What is the cause of this?
A. Resorption Atelectasis
B. Contraction Atelectasis
C. Compression Atelectasis
D. Pneumoconiosis

10. A middle age man presents to clinic with chronic bronchitis. After the spirometry results show ?
A. Increased FEV1/ FVC AND increased TLC
B. Decreased FEV1/ FVC AND decreased TLC
C. Decreased FEV1/ FVC AND increased TLC
D. Increased FEV1/ FVC AND normal TLC

11. Resorption atelectasis is mainly caused by?
A. Lung fibrosis or pleura
B. Pleural effusion
C. Commonly in bedding patients
D. Obstructive bronchi



7	8	9	10	11
B	C	C	C	D

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12. Bronchiectasis causes?
A. Contraction atelectasis
B. Resorption atelectasis
C. Reduced forced vital capacity
D. Compression atelectasis

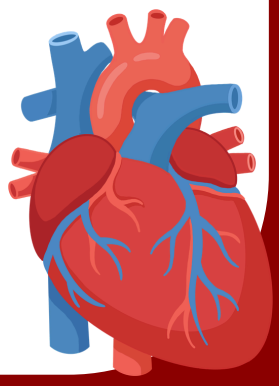
13. Atelectasis cause hypoxia by which of the following?
A. Ventilation perfusion imbalance
B. Not cause hypoxia
C. Fused but not collapse
D. Ventilation perfusion balance

14. Bronchiectasis causes which of the following complications?
A. Contraction atelectasis
B. Compressive atelectasis
C. Resorption atelectasis
D. Restrictive lung disease

15. The disease that reduced the FEV1/FVC ratio?
A. Pneumoconiosis
B. Sarcoidosis
C. Chronic bronchitis
D. Idiopathic pulmonary fibrosis

16. A 6 year old child aspirates a peanut and comes into the ER with shortness of breath. After 3 days he had a fever and diarrhea. What is the complication that may happen to him?
A. Bronchopneumonia
B. Bronchiectasis
C. Resorption
D. Acute bronchitis

12	13	14	15	16
B	A	C	C	C



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17. (Case) A smoker came to the clinic with dyspnea and chronic productive cough that lasted 3 months over the last 2 years. What is true about the laboratory findings?
A. Reduced FEV1/FVC ratio
B. Normal FEV1/FVC ratio
C. Decreased total lung capacity
D. Reduced FVC

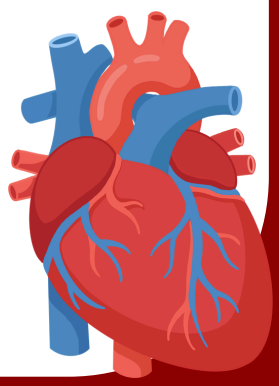
18. What is correct about acute respiratory distress syndrome?
A. It is a restrictive lung disease
B. FVC is increased
C. TLC increased
D. FEV1/FVC decrease

19. Which of the following will most likely result in contraction atelectasis?
A. Pleural fibrosis
B. Pneumothorax
C. Bronchial asthma
D. Bronchiectasis

1. Cigarettes smoking contribute to pathogenesis of emphysema by all except?
A. Stimulation of macrophages elastase activity
B. Stimulate release of neutrophil elastase
C. Attracts neutrophils to the lung
D. Inhibit the ability of the pulmonary neutrophil to destroy the bacteria

2. Decrease alpha 1 anti-trypsin occurs in?
A. Distal Acinar
B. Pan-acinar
C. Centri-acinar
D. Para-septal acinar

17	18	19	1	2
A	A	A	D	B



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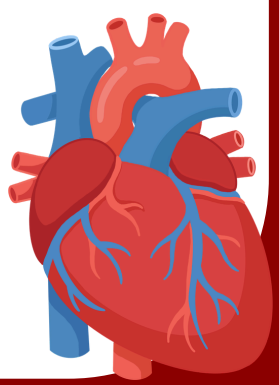
3. An old man who is a chronic smoker had a progressively increasing dyspnea, he had barrel chest appearance and had histological changes more in his upper lobes, his condition is most likely due to which of the following?
A. Due to mucous plug
B. Due to release of elastase from neutrophils
C. Due to Alpha-1-antitrypsin deficiency
D. Due to cystic fibrosis

4. Which of the following diseases cause a decreased FEV1/FVC?
A. Chronic Bronchitis
B. Pneumoconiosis
C. Acute interstitial lung disease
D. Sarcoidosis

5. What type of emphysema associated with antitrypsin deficiency?
A. Distal Acinar
B. Pan-acinar
C. Centri-acinar
D. Para-septal acinar

6. A 58 years old patient has obesity, loud snoring at night, deveolped cor pumlonae what is the most likely cause of his condition?
A. Emphysema
B. Lung tumor
C. Hypoxia
D. Infection

7. Redix index?
A. The ratio of thickness of submucosal gland to bronchial wall
B. The ratio of thickness of cartilage to bronchial wall
C. The ratio of thickness of mucosa to bronchial wall
D. The ratio of thickness of muscular part to bronchial wall



3	4	5	6	7
B	A	B	c	A

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8. A 67 years old male, two-pack a day smoker since age 18, has had a productive cough above the last 20 years, his symptoms worsens the last 4 years, Arterial pO2 is decreased Arterial pCO2 is increased, total lung capacity is normal, these finding are most suggestive of which pulmonary disease?

A. Adult respiratory distress syndrome
B. Bronchial Asthma
C. Chronic bronchitis
D. Panacinar emphysema

9. A 56 year old man came to the clinic complaining of weight loss and progressive dyspnea, the doctor noticed that the patient shows pursed lip breathing. What is the histological feature of the underlying disease ?

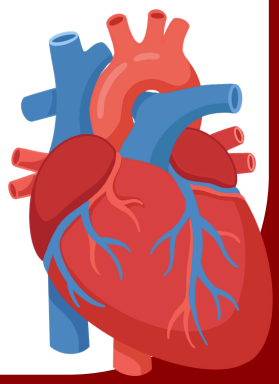
A. Dilation of bronchi with destruction of their wall
B. Bronchial mucus glands hyperplasia
C. Alveolar damage with proteinaceous fluid in alveolar space
D. Destruction and enlargement of alveolar walls

10. (Case) Patient have a productive cough with purulent septum & situs inversus?

A. Bronchectasis
B. Emphysema
C. Atelectasis
D. Chronic bronchitis

11. A 40-year-old female had suffered from multiple attacks of necrotising pneumonia. She now has a productive cough of a large amount of offensive purulent sputum. The chest X-ray suggested dilatation of the right lower lobe bronchi. Which of the following mechanisms is the cause of bronchial dilatation?

A. Unopposed action of neutrophil-derived elastase
B. Damage to bronchial mucosa by inflammatory mediators released from eosinophils
C. Destruction of bronchial walls by recurrent inflammation with fibrosis
D. Congenital weakness of supporting structures of the bronchial wall



8	9	10	11
C	D	A	C

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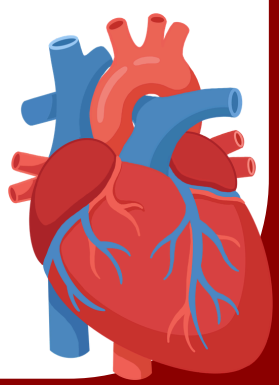
12. 86 years old woman has difficulty and excreting the air all the time. She has no history of smoking or being exposed to fumes. There is no history of weight loss and no persistent cough. The most common reason for her condition is?
A. lose the elasticity of the lung
B. Bronchial asthma
C. Lung cancer
D. Decrease surfactant

1. What is the virulence factor of Corynebacterium diphtheriae?
A. Enterotoxin
B. Exotoxin
C. Enzyme
D. Capsule

2. (Case) A patient came with fever, pharyngeal edema, bull neck appearance, and gray membrane coating the tonsils. What is the organism?
A. Corynebacterium diphtheriae
B. Streptococcal tonsillitis
C. Staphylococcal aureus
D. Streptococcus pneumonia

3. (Case) A patient came with inflammation of the pharynx, tonsils, and gums and offensive halitosis caused by borrelia vincentii and fusobacterium. What is the diagnosis?
A. Vincent's Angina
B. Epiglottitis
C. Croup
D. Viral pharyngitis



12	1	2	3
A	B	A	A

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4. (Case) A patient came with high fever, purulent exudates, and enlarged cervical lymph node. What is the immunological complication of this disease?
A. Rheumatic fever
B. Scarlet fever
C. Septicemia
D. Glandular fever

5. A patient comes to the clinic and has inflammation in the pharynx, tonsils, and gums with necrotic debris. Gram stains reveal a gram-negative spiral organism and gram-negative long tapering bacilli. What is the disease?
A. Streptococcal tonsillitis
B. Epiglottitis
C. Fever
D. Vincent angina

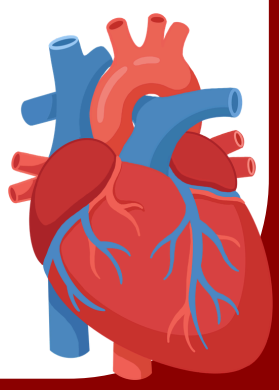
6. The disease that can be prevented with vaccination is?
A. Epiglottitis
B. Legionellae's disease
C. Croup
D. Common cold

7. The following microorganism is the most cause of acute otitis externa?
A. Haemophilis Influenza
B. Pseudomonas aeroigenosa
C. Streptococcus pneumonia
D. B-haemolytic streptococci

8. What is the mode of transmission of rhinovirus ?
A. Aerosol
B. Food
C. Contact
D. Blood

9. How is croup transmitted?
A. Blood
B. Aerosol
C. Contact
D. Food

4	5	6	7	8	9
A	D	A	B	A	B



MidNight RS - MED23

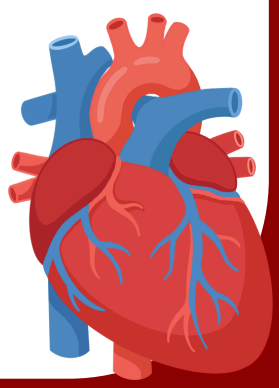
10. What is the treatment of acute epiglottitis?
A. Ampicillin
B. Amantadine
C. Cefotaxime
D. Penicillin

1. (Case) A child comes to the hospital with paroxysmal cough, cyanosis, eye congestion, high pitched sound during intaking air between bouts of coughing. He was infected with gram negative coccobacilli. What is the diagnosis?
A. Whooping cough
B. Epiglottitis
C. Severe acute respiratory syndrome (SARS)
D. Tuberculosis

2. (Case) 5 months infant with bronchiolitis ?
A. Respiratory syncytial virus
B. Adenovirus
C. Influenza virus
D. Rhinovirus

3. A 44 year old patient came to you complaining of a persistent cough for three weeks, night sweats, fever and weight loss. Ziel Neelsen stain showed red beaded bacilli in a blue background. what is the virulence factor of its pathogenesis?
A. Bacteria can destroy macrophages
B. Bacteria can survive in macrophages
C. Bacteria can live in bronchi
D. Bacteria can destroy pulmonary tissue

4. A patient presented with lower respiratory tract symptoms, he give history of bird keeping, what is the causative organism?
A. Bordetella
B. Diphtheria
C. SARS
D. Chlamydia psittaci



10	1	2	3	4
C	A	A	B	D

MidNight RS - MED23

5. What is the most common cause of acute bronchitis?
A. Bacterial infection
B. Viral infection
C. Fungul infection
D. Non-infectious cause

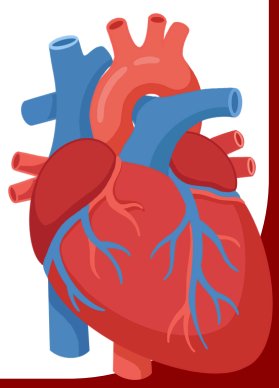
6. A 68 year old diabetic man presented to the ER with productive cough, purulent sputum, and shortness of breath. Chest X-ray shows consolidation in the lobe of the lung. What organism is most likely the cause of his symptoms?
A. Chlamydia Psittacosis
B. Haemophilus Influenzae
C. Streptococcus Pneumonia
D. Staphylococcus Aureus

7. A 60 years old male goes to Alulaa , after 2-3 days he start to complain of runny nose, cough, sore throat, and fever , also its progressed to acute pneumonia and diarrhea . What is the organism that he affected with after contact with camels ?
A. SARS
B. MERS-COV
C. Chlamydia pneumonia
D. Staphylococcus aureus

8. Which of the following is the most causative organism to community acquired pneumonia ?
A. Streptococcal pneumonia
B. Mycoplasma pneumonia
C. Respiratory syncytial virus
D. Candida

9. Species of the following genus causes zoonotic disease?
A. Legionella
B. Chlamydia
C. Bordetella
D. Escherichia

5	6	7	8	9
B	C	B	A	B



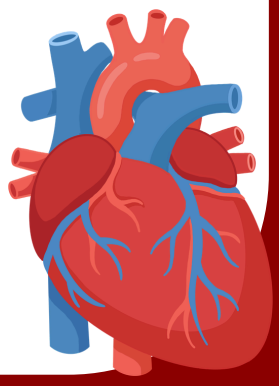
MidNight RS - MED23

10. A ten month old boy presented to the clinic with cough, fever, runny nose, and otitis media. Pulmonary examination showed him to have bronchiolitis. These clinical findings are most commonly caused by?
A. Rhinovirus
B. Respiratory syncytial virus
C. Para influenzas
D. Adenovirus

11. BCG vaccine is given?
A. Intradermal
B. Intramuscular
C. Subcutaneous
D. Oral

12. A 30 years old female, under chemotherapy, was diagnosed with an infection. What's the most common microorganism that affects immunocompromised patients?
A. Salmonella
B. Leishmanial
C. Aspergillosis
D. Pneumonia jiroveci

13. Which of the following is not taken into consideration when prescribing a drug for community acquired pneumonia?
A. Family history of lung disease
B. Previous antibiotic use
C. Excessive alcohol intake
D. Co-morbidities



10	11	12	13
B	A	D	A

MidNight RS - MED23

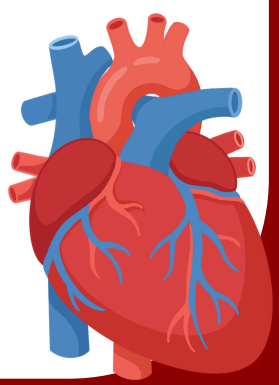
1. The causes of anaphylactic shock?
A. Entamoeba histolytica
B. Hydatid cyst
C. Trematodes
D. Ascaris lumbricoides

2. Infection with hydatid cyst occurs after?
A. Ingestion of parasitic cyst
B. Eating an organ with hydatid cyst
C. Ingestion of Echinococcosis granulosus egg
D. Inhalation of contaminated water

3. A 37 year old Patient came with dyspnea, chest pain, and cough. ct scan and chest x-ray showed that there is pleural effusion and right lung abscess.what is the most causative agent?
A. Ascaris
B. Echinococcus garnulus
C. Entamoeba histolytica
D. Streptococcus pneumoniae

4. A Patient go trip in alula and after two weeks he have upper right quadrant pain , jaundice, and abdominal mass with lung abscess. During surgery they find liver cyst. What the parasite that he infected with?
A. Echinococcus granulosus
B. Schistosomae
C. Entamoeba histolytica
D. Paragonimus westermani

5. Larval form of Echinococcus granulosus is present in which host?
A. Dog
B. Wolf
C. Man
D. Sheep



1	2	3	4	5
B	C	C	A	C+D

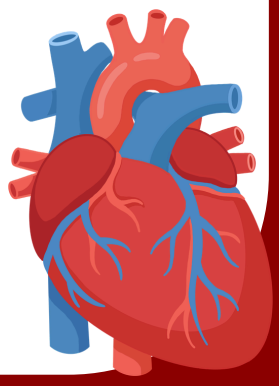
MidNight RS - MED23

6. A patient presented with cough, dyspnea, chest pain. CXR/investigation revealed a small amount of pleural effusion and abscess in the lung. What is the most likely causative parasite?
A. Entamoeba histolytica
B. Echinococcus granulosus
C. Paragonimus westermani
D. Schistosomiasis

7. A patient came to the ER with a cough and fever, X-rays showed elevated right diaphragm and lower right pleural effusion and abscess. What is the causative agent?
A. Entamoeba histolytica
B. Ascariasis
C. Hydatid cyst
D. Toxocariasis

8. A patient came in with skin flushed, they found a ruptured hepatic cyst. What is the causative agent?
A. Echinococcus
B. Ascariasis
C. Toxocariasis
D. Entamoeba histolytica

9. What is the preventative measure for Echinococcus?
A. Removal of snails
B. Avoid contamination of hands, food, and water with dog feces
C. Wear preventive clothes (gloves - shoes)
D. Proper washing of raw vegetables



6	7	8	9
A	A	A	B

MidNight RS - MED23

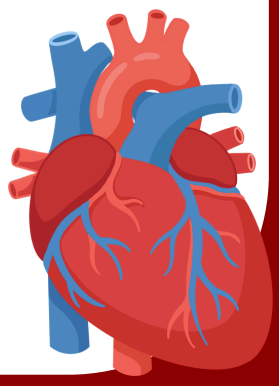
10. A male shepherd from AlUla came complaining of hepatomegaly, upper right quadrant pain, jaundice, and a non-painful hepatic cyst was found. What is the likely causative agent?
A. Echinococcus
B. Entamoeba histolytica
C. Schistosomiasis
D. Ascaris lumbricoides

11. Where are the larval forms of Echinococcus found?
A. Man
B. Dog
C. Cat
D. Wolf

12. What is the common extrahepatic site of ambiosis?
A. Peritoneum
B. kidney
C. Spleen
D. Lung

1. Loffler's Syndrome in the lung is due to?
A. Sensitization to ascaris antigen
B. inflammation reaction against ascaris larva in the lung
C. viral infection in the lung
D. Bacterial infection in the lung

2. Which of the following is a prevention of Ancylostoma?
A. Wear shoes
B. Wash vegetables
C. Wash hands
D. Proper sewage disposal



10	11	12	1	2
A	A	D	B	A

MidNight RS - MED23

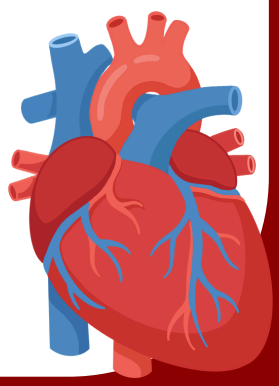
3. Which one of the following infections lead to Cor pulmonale?
A. Toxocariasis
B. Hydatid
C. Amoebiasis
D. Schistosomiasis

4. A 42 year old man returned two weeks ago from a five month trip to Ghana. During his stay he had frequent visits to Lake which included swimming in the lake. He was present with fever, cough wheeze myalgia headache and urticaria. stool analysis revealed 2-3 eggs of schistosoma mansoni. What is the stage?
A. chronic schistosomiasis
B. Cercarial penetration (invasion stage)
C. Cercaria (migration stage)
D. Acute Schistosomiasis (Katayama)

5. Which of the following is the egg that causes Ascaris infection to humans by raw vegetables and contaminated water?
A. Fertilised egg
B. Unfertilized egg
C. Mature egg
D. Immature egg

6. Patient presented with 3 weeks of cough, dyspnea, CXR revealed bilateral zone patch consolidation, eosinophilia and Ascaris larva in sputum, what is this patient have ?
A. Katameya fever
B. Liver abscess
C. Lung cyst
D. Loffler's syndrome

7. Which of the following can lead to Cor pulmonale?
A. Schistosomiasis
B. Toxocariasis
C. Echinococcosis
D. Ascariasis



3	**4**	**5**	6	**7**
D	**D**	**C**	D	**A**

MidNight RS - MED23

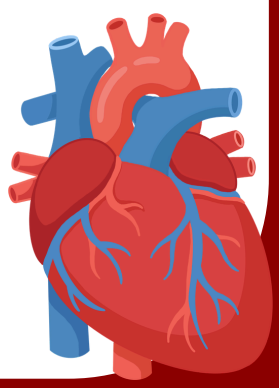
8. A 6-year-old boy played with dogs on a school trip, later he came in with chest tightness,and bronchospasm. What is the possible organism?
A. Toxocara
B. Schistosoma
C. Ascaris
D. Ancylostoma

9. Patient ate green salad and got a parasitic infection with increased eosinophilia?
A. Ascariasis
B. Schistosomes
C. Toxocariasis
D. Entamoeba histolytica

1. A 32-year-old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?
A. Levofloxacin
B. Ampicillin with Sulbactam
C. Cefuxime
D. Penicillin

2. A child was infected and upon blood culture the Gram + causative agent showed beta hemolytic colonies. The most suitable treatment is?
A. Penicillin
B. Ampicillin
C. Erythromycin
D. Clindamycin

3. The mechanism of action of Doxycycline?
A. Inhibit bacterial protein synthesis
B. Inhibit bacterial topoisomerase
C. Inhibit bacterial
D. Inhibit bacterial wall synthesis



8	9	1	2	3
A	**A**	**A**	A	**A**

MidNight RS - MED23

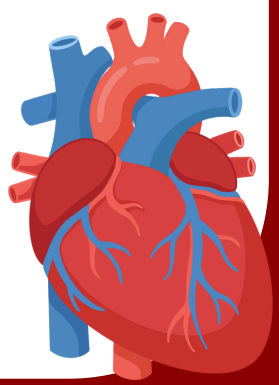
4. In case of Mycoplasma pneumoniae infection, the suggested treatment will be?
A. Vancomycin
B. Cephalosporins
C. Penicillin
D. Erythromycin

5. A 67 years old man has fever and cough, 3 days ago, he suffered from nasal congestion and sore throat, productive cough, the X-ray showed pulmonary infiltrate left lower lobe, what is the antibiotic choice for treatment?
A. Amoxicillin, clavulanic acid
B. Levofloxacin, gentamicin
C. Doxycycline
D. Ceftriaxone

6. A 35 years old pregnant lady visited the clinic with a mild fever and scanty sputum, CXR showed diffuse bilateral infiltration. What is the suggested drug for this patient?
A. Azithromycin
B. Doxycycline
C. Levofloxacin
D. Cephalexin

7. A 32-year-old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?
A. Levofloxacin
B. Ampicillin with Sulbactam
C. Cefuxime
D. Penicillin

8. A 4-year-old child presented with fever, runny nose, and inspiratory whoop following bouts of coughing. What is the drug of choice?
A. Erythromycin
B. Ciprofloxacin
C. Cephalosporin
D. Tetracycline



4	**5**	6	**7**	**8**
D	**A**	**A**	A	**A**

MidNight RS - MED23

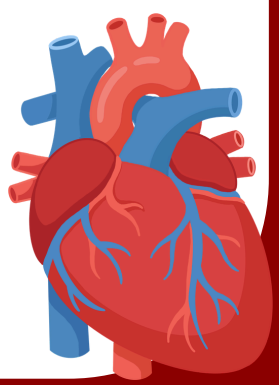
9. Which of the following antibiotics is impaired with calcium?
A. Tetracycline
B. Macrolides
C. Amoxicillin
D. Cephalosporin

10. A 75-year-old patient with a history of diabetes and chronic heart disease suffered from severe pneumonia. What is the first-line drug used for empirical treatment?
A. Ceftriaxone
B. Ciprofloxacin
C. Amoxicillin
D. Azithromycin

11. A young adult came in with community acquired pneumonia, they have no history of illness, they described a history of resistance to penicillin, what is the best drug to give them?
A. Levofloxacin
B. Cefuramine
C. Ceftriaxone
D. Ampicillin

12. Which drug binds to penicillin-binding protein?
A. Macrolides
B. Tetracycline
C. Beta lactam
D. Fluoroquinolones

13. Which drug is given to overcome the resistance of beta-lactamase-producing bacteria?
A. Ciprofloxacin
B. Clavulanic acid
C. Doxycycline
D. Piperacillin

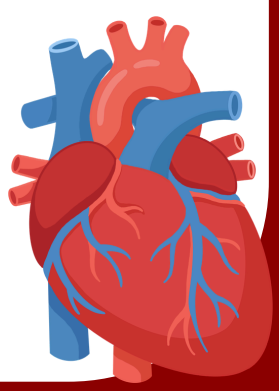


9	10	11	12	13
A	A	A	C	B

MidNight RS - MED23

14. A 28-year-old pregnant woman, currently in her second trimester, presents to the emergency department with fever and a respiratory rate of 27 breaths per minute. Upon examination, diffuse bilateral infiltrates are noted on her chest X-ray. Given her condition, what is the most appropriate antibiotic treatment?

A. Azithromycin
B. Dofloxacin
C. Doxycycline
D. Levofloxacin



14
A

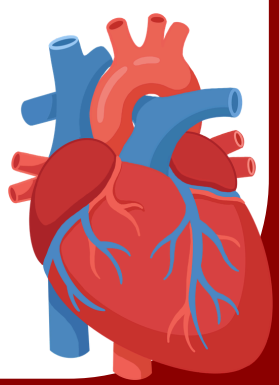
MED09 MIDTERM

1. FEV1/FVC is reduced in:
A. Chronic bronchitis
B. Pneumoconiosis
C. Interstitial pulmonary fibrosis
D. Sarcoidosis

2. The following is a weak acid that plays an essential role in compensating for blood alkalinity:
A. HCl
B. H2CO3
C. HCO3
D. NAHCO3

3. Which chamber of the heart mainly forms the cardiac impression on the medial surface of the right lung?
A. Right atrium
B. Right ventricle
C. Left atrium
D. Left ventricle

4. The following centre helps to prevent overinflation:
A. Apneustic Centre
B. Pneumotaxic Centre
C. Dorsal Respiratory Group
D. Ventral Respiratory Group



1	2	3	4
A	B	A	B

MED09 MIDTERM

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B. Pneumoconiosis
C. Interstitial pulmonary fibrosis
D. Sarcoidosis

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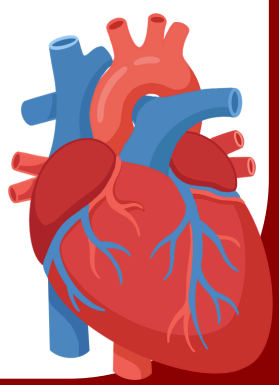


1	2	3	4
A	B	A	B

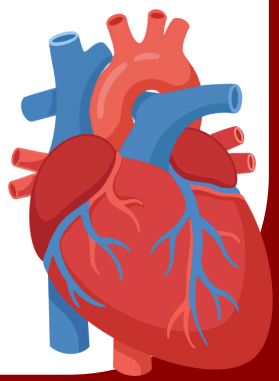
MED09 MIDTERM

5. What drug should be available for all asthma patients (intermittent and persistent)?
A. Low dose of ICS
B. Short-acting beta-2 agonist
C. Long-acting beta-2 agonist
D. Leukotriene antagonists
This reflects older guidelines (B); current guidance recommends ICS-containing treatment for all patients, so A is the closest option, although the preferred reliever is ICS–formoterol.

6. During lung development, continuous division of alveoli and bronchioles till when?
A. 4th intrauterine month
B. 6th intrauterine month
C. 8th intrauterine month
D. 8th year of age after birth

7. A 34-year-old man was brought to the emergency with severe dyspnea and wheezing; the sputum cytologic microscopy shows Charcot–Leyden crystals, Curschmann's spirals, increased acute inflammatory cells, mainly eosinophils. The diagnosis in this case?
A. Chronic bronchitis
B. Bronchial asthma
C. Centriacinar emphysema
D. Bronchiectasis

8. Which statement about pulmonary surfactant is false?
A. It reduces surface tension within the alveoli.
B. It is produced by type I pneumocytes.
C. It contains both lipids and proteins.



5	6	7	8
B	D	B	B

MED09 MIDTERM

9. Which drug strikingly induces bronchial asthma?
A. Corticosteroids
B. Leukotrienes
C. Aspirin

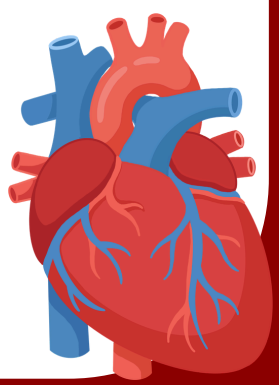
10. Previous smoker: COPD treatment without severe adverse medical effects
A. Tiotropium

11. Infective stage of Entamoeba histolytica
A. Egg
B. Cyst
C. Larva
D. Trophozoite

12. Which medication is preferred for long-term control of bronchial asthma during pregnancy?
A. Inhaled corticosteroid
B. Salbutamol
C. Theophylline

13. Basal atelectasis is most commonly seen in:
A. Failure to breathe deeply in a patient with compression type
B. Diffuse fibrosis affecting the lung
C. Patchy fibrosis affecting the right lung
D. Cases of obstruction of the lung by mucous plugs

9	10	11	12	13
C	A	B	A	A



MED09 MIDTERM

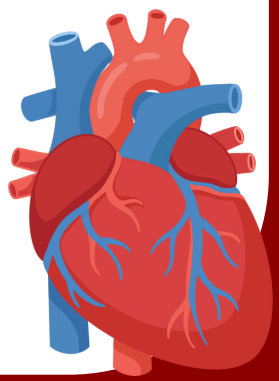
14. Two 16-carbon saturated fatty acids that bind to phospholipids that give alveoli reduced surface tension:
A. Dipalmitoyl phosphatidylcholine
B. phosphatidylglycerol
C. Phosphatidylinositol

15. When do expiratory muscles contract?
A. Inspiration
B. Forced expiration

16. A 38-year-old man had a thyroidectomy; the nerve supplying the superior thyroid artery is cut. Which function of the following is affected:
A. Narrowing of laryngeal inlet
B. Abduction of vocal cords
C. Tension of vocal cords
D. Relaxation of vocal cords

17. Which of the following is most affected by binding of CO (carbon monoxide) to Hemoglobin :
A. PO2
B. PCO2
C. SO2
D. HCO3

18. The following hormone is secreted in excess one month after climbing a mountain:
A. Natriuretic peptide.
B. Adrenaline.
C. Erythropoietin.



14	15	16	17	18
A	B	C	C	C

MED09 MIDTERM

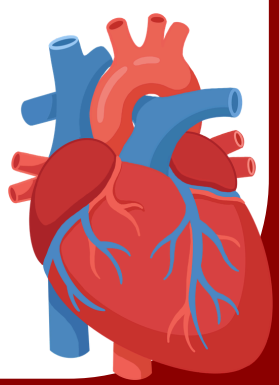
19. A 50-year-old farmer is presented with cough, fever, and dyspnea. X-ray shows homogeneous opacities almost replacing the right lung. Which of the following parasites is associated with this disease:
A. Ascaris lumbricoides
B. Ancylostoma duodenale
C. Echinococcus granulosus
D. Toxocara cati

20. The diffuse membrane between the alveolar thin wall is composed of :
A. alveolar macrophage and alveolar cell membrane
B. alveolar cell membrane and capillary membrane with fused basement membrane
C. alveolar macrophage, cell membrane and capillary membrane

21. Lung compliance increases by:
A. Lung emphysema
B. Lung fibrosis
D. Premature baby's lungs

22. In pneumoconiosis, the principal cell that leads to fibrosis is
A. Macrophage
B. Neutrophil
C. Eosinophil

19	20	21	22
C	B	A	A



MED09 MIDTERM

23. A 17-year-old boy had a street fight; he broke his nasal septum. What would be affected?
A. Vomer and inferior conchae
B. Vomer and perpendicular plate of ethmoid bone
C. Septal cartilage and superior conchae
D. Frontal bone

24. An 8-year-old boy came to the clinic with fever and cough. On examination, pharyngitis with pseudomembrane was found. A swab was taken to the laboratory for diagnosis; it showed Gram-positive bacilli with Chinese letter arrangement. The most important virulence factor of this organism is :
A. Polysaccharide capsule
B. Endotoxin
C. Exotoxin
D. Enterotoxin

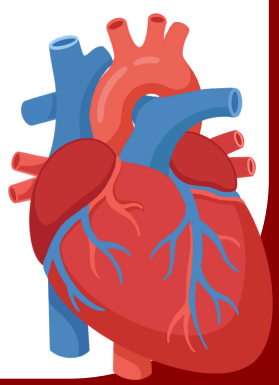
25. The free upper border of the Crico-vocal membrane is:
A. Vocal ligament

26. Laryngotracheobronchitis is common in:
A. Children
B. Elderly people
C. Smokers

27. Foreign body aspiration will cause:
A- Resorption atelectasis

28. Ghon foci is:
A. Associated with primary TB
B. Associated with secondary TB

23	24	25	26	27	28
B	C	A	A	A	A



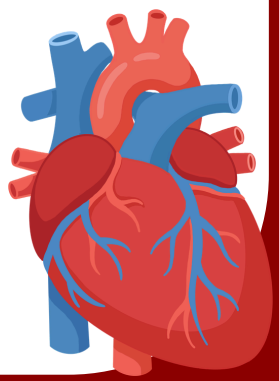
Final Respiratory 🫁

1. What's the infective stage of Entamoeba histolytica?
A. Egg
B. Trophozoites
C. Cyst
D. Cercaria

2. Question about TB medications and warfarin in a case of DVT? (in other words) The patient is already taking warfarin and recently developed tuberculosis; the doctor wants to prescribe first-line treatment for tuberculosis. Which of the following will he do?
A. Review warfarin dose
B. Stop taking pyrazinamide
C. Increase dose of rifampicin
D. Decrease dose of isoniazid

3. Picture of lungs, arrow pointing towards hilar part of the right lung. Which of the following arteries supplies it?
A. Right pulmonary artery
B. Right 3rd posterior intercostal artery
C. Left 3rd intercostal artery
D. Descending aorta

4. A patient had a previous stroke; she got a second one which stopped blood supply to the medullary region. What will occur?
A. Apneustic breathing
B. Immediate cessation of breathing
C. Ataxic breathing
D. Tachypnea



1	2	3	4
C	A	B	B

Final Respiratory

5. Client Z is admitted to the hospital and is to undergo brain surgery. The client is very anxious and scared of the upcoming surgery. He begins to hyperventilate and becomes very dizzy. The client loses consciousness, and the STAT ABGs reveal pH 7.61, PaCO2 22 mmHg, and HCO3 25 mEq/L. What is the ABG interpretation based on the findings?

A. Metabolic Acidosis, Uncompensated
B. Respiratory Alkalosis, Partially Compensated
C. Metabolic Alkalosis, Partially Compensated
D. Respiratory Alkalosis, Uncompensated

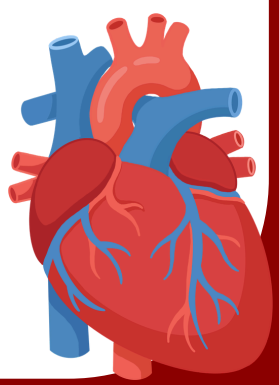
The stated pH, PaCO2, and HCO3 values are not mathematically consistent.

6. Which of the following is a complication of streptococcal pneumonia?

A. Pneumothorax
B. Pulmonary Embolism
C. Tuberculosis
D. Parapneumonic effusion

7. Which of the following will yield a transudate fluid?

A. Malignancy
B. Heart failure
C. Tuberculosis
D. Pneumonia



5	6	7
D	D	B

Final Respiratory 🫁

8. A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing. Typically results from dust inhalation. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. Which of the following histologic features is most likely to characterize the lung in her condition?

A. Charcot-Leyden crystals

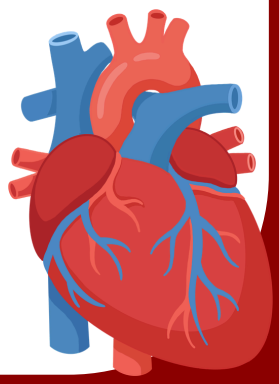
9. A patient presents with fever and features of lower respiratory tract infection. He gives the history of keeping birds and parrots. What is the causative organism?

A. Chlamydia psittaci
B. Legionella pneumophila
C. Mycoplasma pneumoniae
D. Respiratory syncytial virus (RSV)

10. You are given an ABG report which states: PaO2 = 80mmHg, SaO2 = 95%, and CaO2 = 20ml O2/dl. Which of the following fits the description?

A. The total amount of oxygen in the blood is 80 mmHg
B. The oxygen bound to hemoglobin and that dissolved in the plasma is 20ml O2/dl

8	9	10
A	A	B



Final Respiratory

11. A 53-year-old patient has difficulty breathing through the nose. On examination, his physician finds that he has Female edit slide 30 swelling of the mucous membrane of the superior nasal meatus. Which of the following paranasal sinuses is affected?

A. Middle ethmoidal
B. Posterior ethmoidal
C. Frontal
D. Maxillary

12. A young girl is complaining of a dry palate and nose. The lesion is most probably occurring in which ganglion?

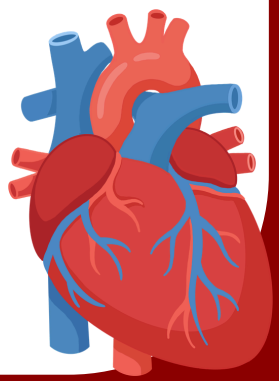
A. Submandibular
B. Ciliary
C. Pterygopalatine
D. Otic

13. A surgery was performed on the superior thyroid artery, and it is suspected that nerve damage occurred. What would result from this nerve damage?

A. Increased Vocal Cord Length
B. Decreased Tension
C. Enhanced Pitch Control
D. Complete Vocal Cord Paralysis

14. Which of the following lymph nodes is found inside the lungs?

A. Tracheobronchial
B. Bronchopulmonary
C. Pulmonary
D. Paratrachea



11	12	13	14
B	C	B	C

Final Respiratory

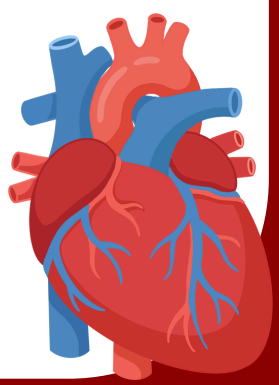
15. Why do aspirated bodies go into the right main bronchus?
A. Because the right bronchus is wider, shorter and more vertical than the left bronchus
B. Because the right bronchus is narrower than the left bronchus
C. Because the right bronchus is more oblique than the left bronchus
D. Because the right bronchus is longer than the left bronchus

16. Which of the following lung tissues lack smooth muscle?
A. Alveoli
B. Trachea
C. Bronchus
D. Bronchioles

17. During maturation of the lung, the Canalicular stage contains which of the following?
A. Alveolar ducts
B. Alveolar sacs
C. Alveoli
D. Primitive alveoli

18. Surface area of alveoli continues to increase until:
A. Adult stage
B. Before birth
C. 8 Years
D. Childhood

15	16	17	18
A	A	A	A



Final Respiratory

19. Which of the following metabolic disorders is caused by hypoventilation?

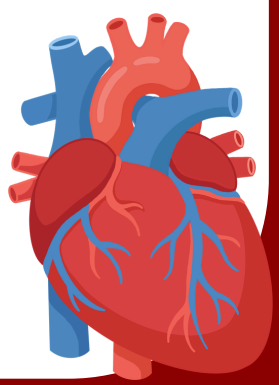
A. Respiratory acidosis
B. Respiratory alkalosis
C. Metabolic acidosis
D. Metabolic alkalosis

20. Which of the following factors shift the oxyhemoglobin curve to the right?

A. Decreased CO2 pressure
B. Decreased DPG
C. Increased PH (alkalosis)
D. Increased Temperature

21. A 52 year old female patient presents to the clinic with progressive shortness of breath, fatigue, and a chronic cough. Laboratory tests reveal elevated liver enzymes both ALT and AST. A diagnosis of cirrhosis is made. Which of the following types of emphysema is most likely present in this scenario?

A. Centri Acinar emphysema
B. Panacinar emphysema
C. Para Septal emphysema
D. Irregular emphysema



19	20	21
A	D	B

Final Respiratory

22. A 35-year-old male patient presents to the clinic for a routine check-up. He has no significant past medical history and denies any symptoms. Physical examination, including auscultation of the chest, reveals normal breath and heart sounds. However, a routine chest X-ray shows situs inversus. Given the patient's clinical history and imaging findings, what is the most likely diagnosis?

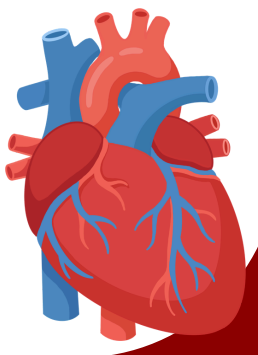
A. Asthma
B. COPD
C. Bronchiectasis
D. Pulmonary Fibrosis

The question is incomplete: situs inversus alone does not establish bronchiectasis. Recurrent respiratory infections or evidence of primary ciliary dyskinesia would support this answer.

23. A 42 year old farmer presents with dyspnea, chest pain, and a chronic cough. Stool examination revealed no parasitic eggs. Additional testing revealed eosinophilia and leukocytosis. Which of the following is the most likely diagnosis?

A. Pulmonary hydatid cyst
B. Loffler syndrome
C. Schistosomiasis
D. Amoebiasis

Transient pulmonary infiltrates would make the diagnosis clearer; eosinophilia and a negative stool examination alone are not specific.



22	23
C	B

Final Respiratory

24. A 42 year old man returned two weeks ago from a five month trip to Ghana . During his stay he had frequent visits to Lake which included swimming in the lake. He was present with fever, cough wheeze myalgia headache and urticaria.stool analysis revealed 2-3 eggs of schistosoma mansoni. What is the stage?

A. Chronic schistosomiasis
B. Cercarial penetration (invasion stage)
C. Cercaria (migration stage)
D. Acute Schistosomiasis (Katayama)

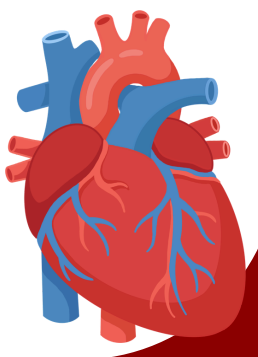
25. A 57 year old farmer presents with dyspnea, chest pain, and a chronic cough. CXR shows homogeneous opacities that almost fill up the right lung. Which of the following is the most likely diagnosis?

A. Amoebiasis
B. Ancylostomiasis
C. Toxocariasis
D. Echinococcosis

26. Reid index?

A. The ratio of thickness of submucosal gland to bronchial wall
B. The ratio of thickness of cartilage to bronchial wall
C. The ratio of thickness of mucosa to bronchial wall
D. The ratio of thickness of muscular part to bronchial wall

24	25	26
D	D	A



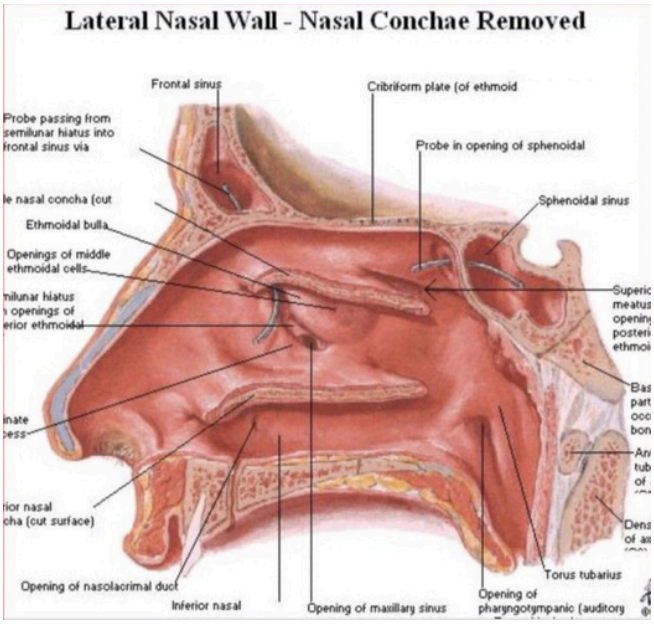
Final Respiratory

27. A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing. The attacks are particularly common in the spring. During an episode of acute respiratory difficulty, a physical examination shows that she is afebrile. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. A sputum sample examined microscopically also has increased numbers of eosinophils. Which of the following histologic features is most likely to characterize the lung in her condition?

A. Remodeling of airways with smooth muscle hyperplasia
B. Patchy areas of consolidation with leukocytic exudate in alveoli
C. Dilationofrespiratorybronchioleswithlossofelasticfibers
D. Inflammatory destruction of bronchial walls

28. Which air sinus opens onto the bulla ethmoidalis?

Answer: Middle ethmoidal air sinus

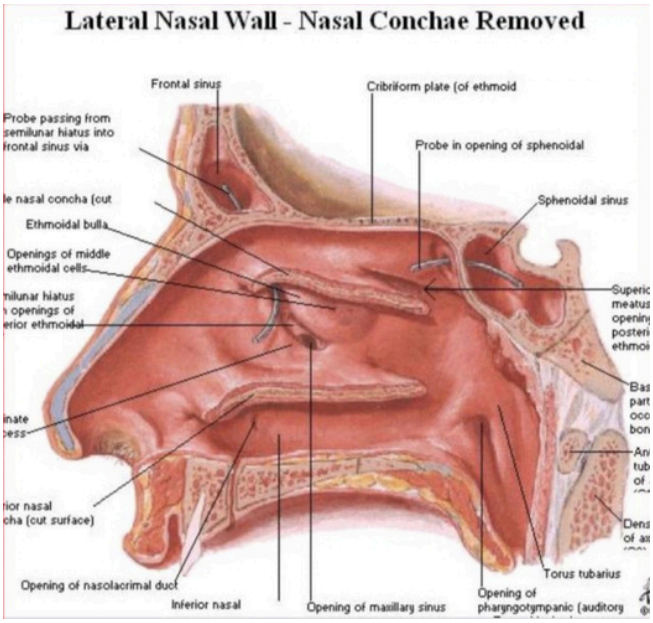


27
A

Final Respiratory

29. Which air sinus opens into the hiatus semilunaris at the indicated opening?

Answer: Maxillary air sinus

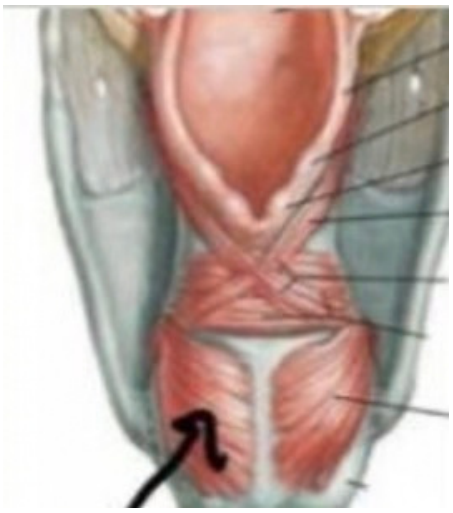


30. Which nerve supplies the muscle indicated in the image?

A. Recurrent laryngeal

B. External laryngeal

C. Internal laryngeal

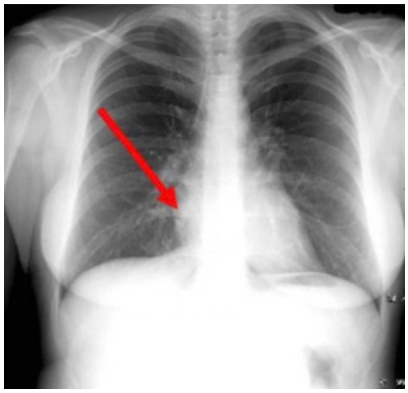


30
A

Final Respiratory

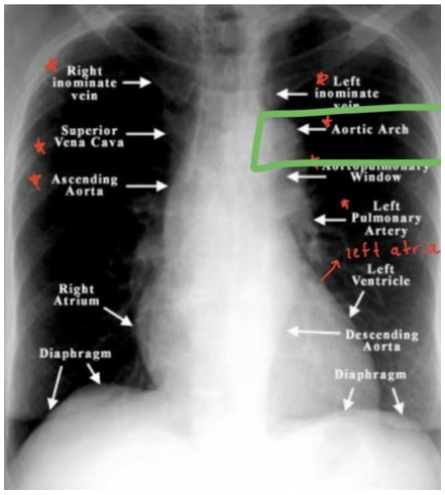
31. The arrow is pointing to:

A. Thoracic aorta
B. Right ventricle
C. Right atrium
D. Inferior vena cava



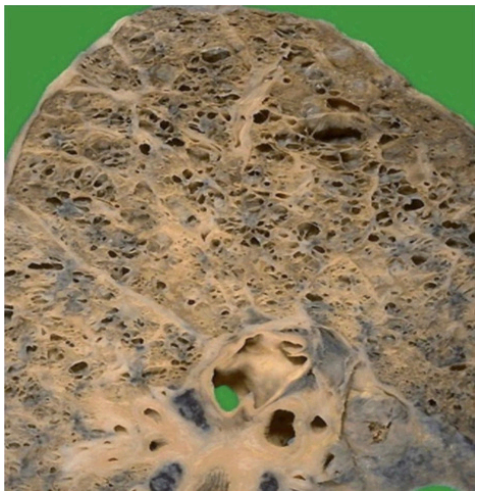
32. Which structure is indicated in the chest X-ray?

A. Aortic arch
B. Left pulmonary artery
C. Left hilum
D. Left ventricle

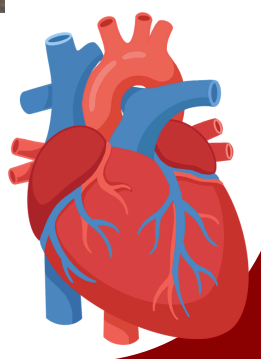


33. What does the image show?

A. Diffuse dilatation of airspace involving the entire lung lobe & lobules.
B. The area of tan yellow consolidation surrounding lung.
C. Focal dilatation of airspace with intervening normal rspace.



31	32	33
C	A	A

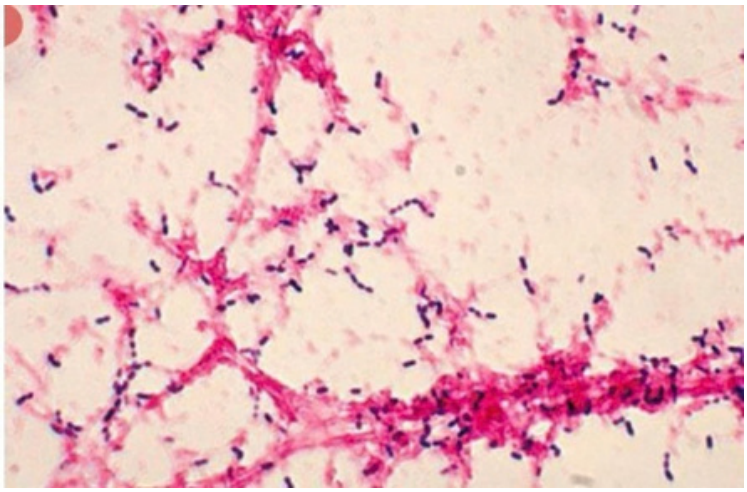


Final Respiratory 🫁

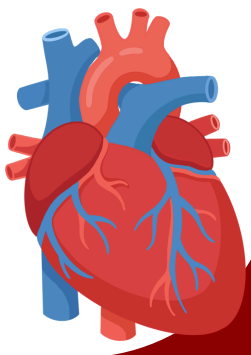
34. What does the image show

A. Streptococcus pneumonia

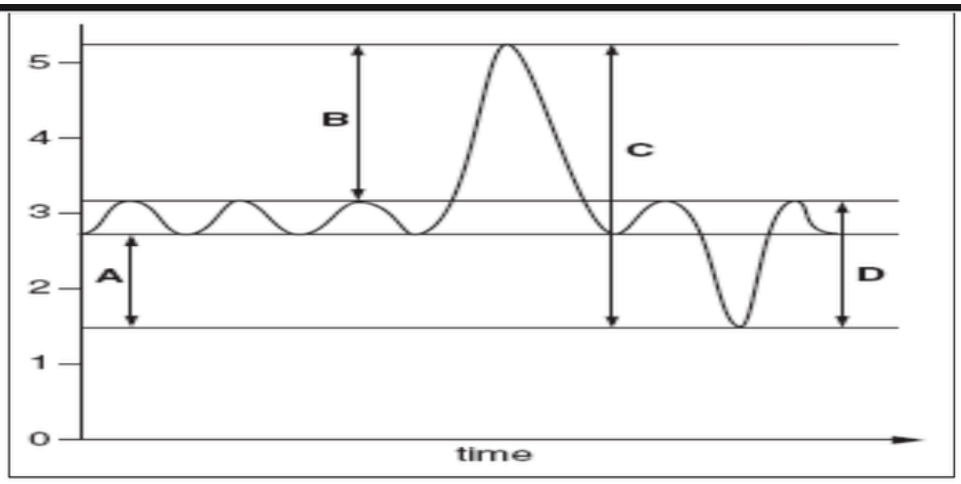
B. Haemophiles influenzae



34
A

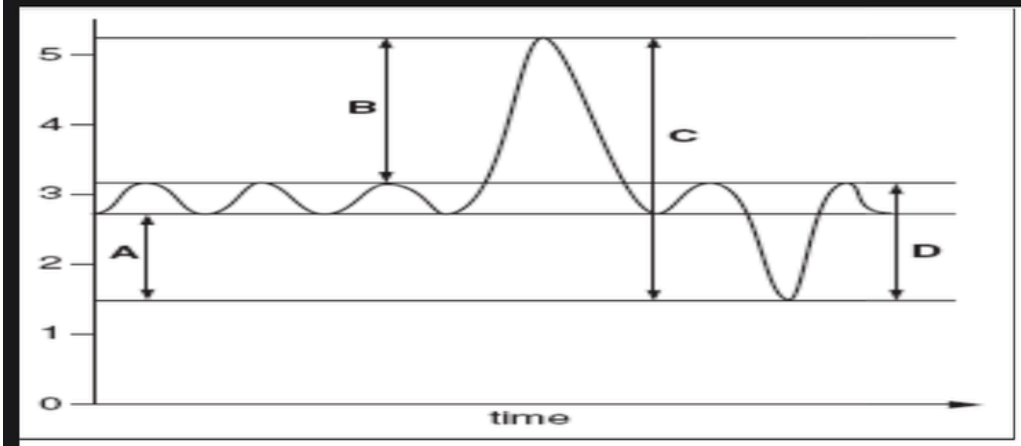


Practical test bank



5. B refers to:

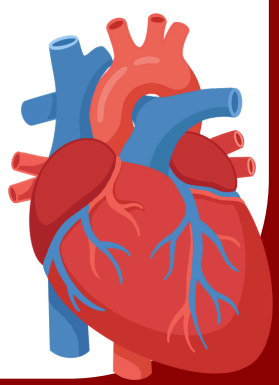
a. Inspiratory reserve volume
b. Expiratory reserve volume..
c. Inspiratory capacity.
d. Functional residual capacity



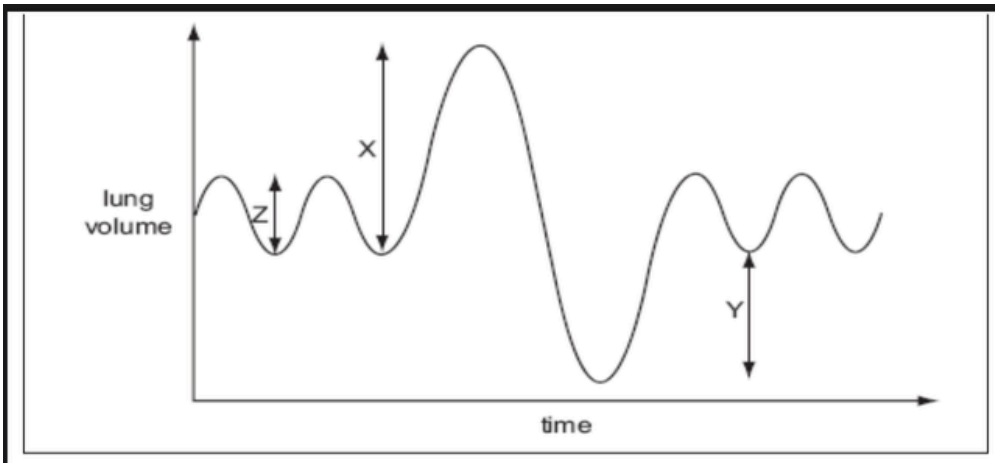
6. C refers to:

a. Inspiratory reserve volume
b. Expiratory reserve volume..
c. Inspiratory capacity.
d. Vital capacity

#	#	#	#



Practical test bank



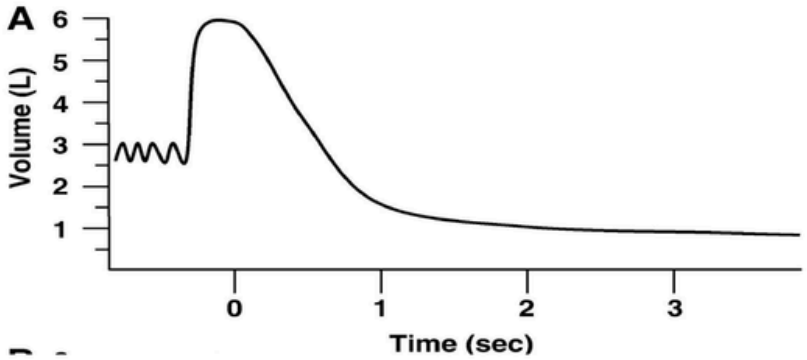
7- **The letter X refers to:**

a. Functional Residual Capacity

b. Inspiratory Capacity

c. Vital Capacity.

d. Total lung capacity.



8 – **The shown test is used to differentiate between:**

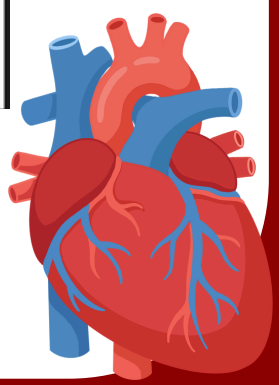
a. Old and young persons.

b. Male and female persons.

c. Obstructive and restrictive lung diseases.

d. Obese and non-obese persons.

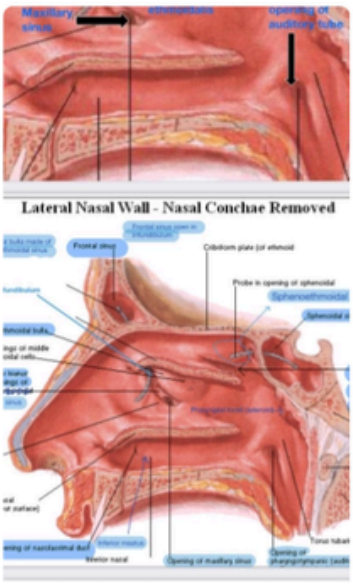
#	#	#	#



Practical test bank

1-identify the structure: bulla ethmoidalis

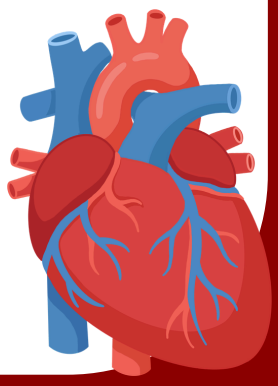
2-what paranasal sinus forms it: middle ethmoidal sinuses



الصورة ذي جبا مختارة و واضحة
(لا تسحبو عليها بعضي)

3 ,4- identify the muscle and its action (thyroarytenoid muscle, relaxation and shorting of vocal cord)

#	#	#	#

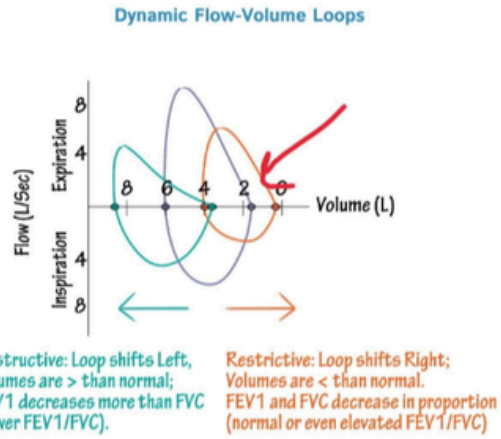


Practical test bank

6 the arterial supply of this lung (right lung) comes from?
Ans: 3rd right posterior intercostal artery (which gives bronchial artery)

13- The graph indicates ?

14-Normal FEV1/FVC ratio indicates?



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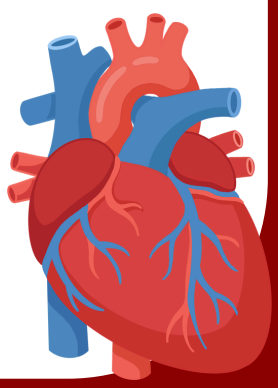
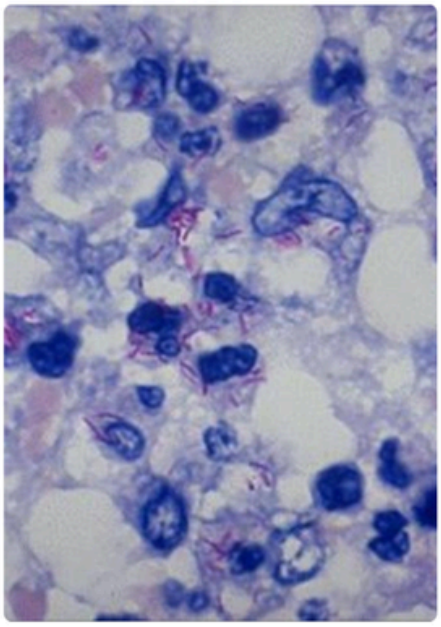


Practical test bank

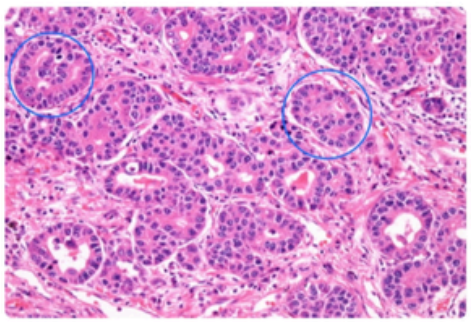
15 a case describing TB and asking what is it



16 what is the first line treatment for the previous disease

Q18))A female comes to the ER suffering from long episodes of cough, haemoptysis, weight loss. The investigations show's a peripheral mass in her lung and a biopsy was taken. What is the best Description of the microscopic appearance of the biopsy?

A-Malignant squamous cells with keratin pearls.
B-Malignant small round cells with foci of punctate necrosis.
C-Irregular glands lined by malignant epithelial cells with hyperchromasia and prominent nucleoli.



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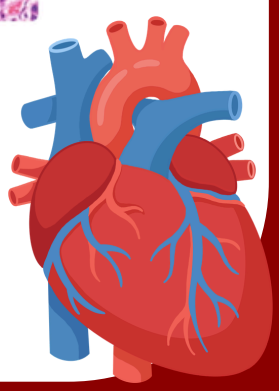
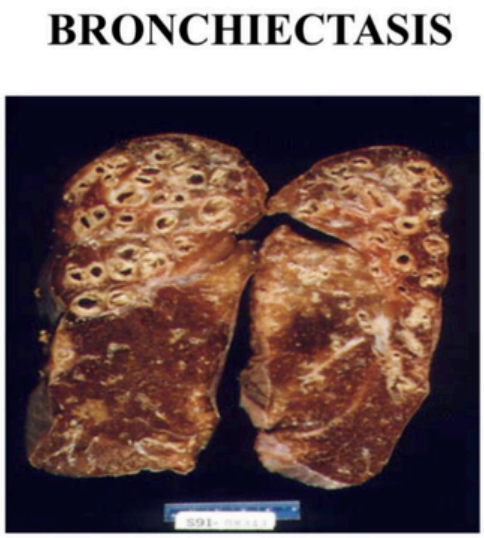
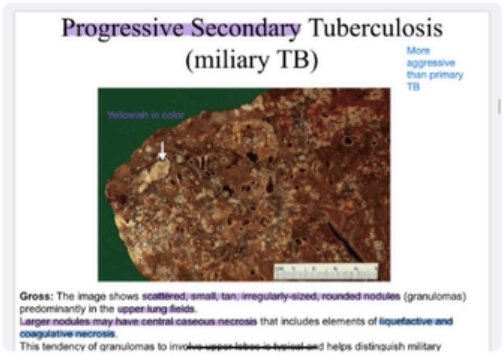


Practical test bank

Q19) identify



20)A man came after 3 weeks of productive cough weight loss and fever
What is he diagnosed with?
Answer:miliary TB



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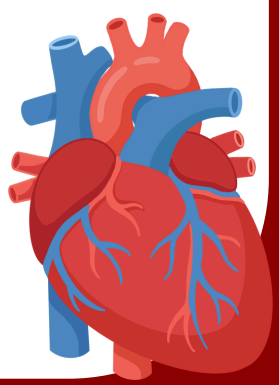


Practical test bank

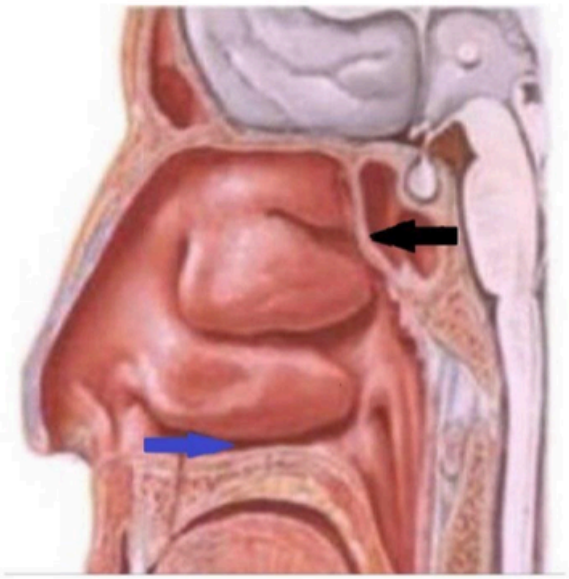
1- the marked sinus open in ? (**black**)

A- infundibulum

B- anterior ethmoidal

C-sphenoethmoidal recess

D- posterior ethmoidal



ANSWER: **C**

2- The marked structure have the opening of ? (blue)

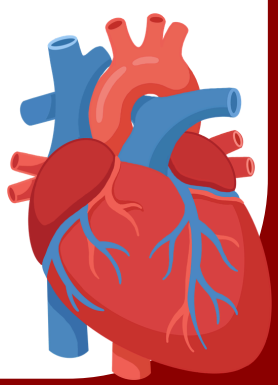
A-infundibulum

B-nasolacrimal duct

C-anterior ethmoidal

D-posterior ethmoidal

ANSWER: **B**



#	#	#	#

Practical test bank

3- what's the marked structure related to this grove ? (black)

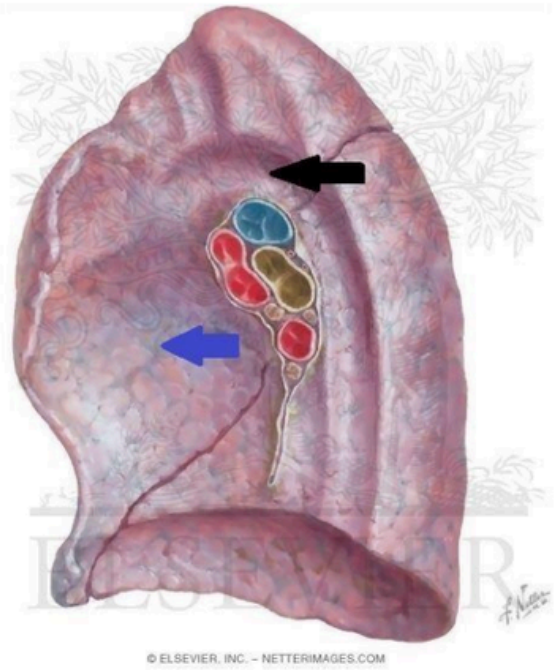
A-arch of aorta

B-esophagus

C-pulmonary vein

D-arch of azygous

ANSWER: **D**



4 what's the marked structure related to this grove ? (Blue)

A-left ventricle

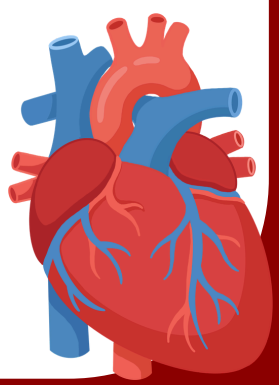
B-right atrium

C- left atrium

D-right ventricle

ANSWER: **B**

#	#	#	#



Practical test bank

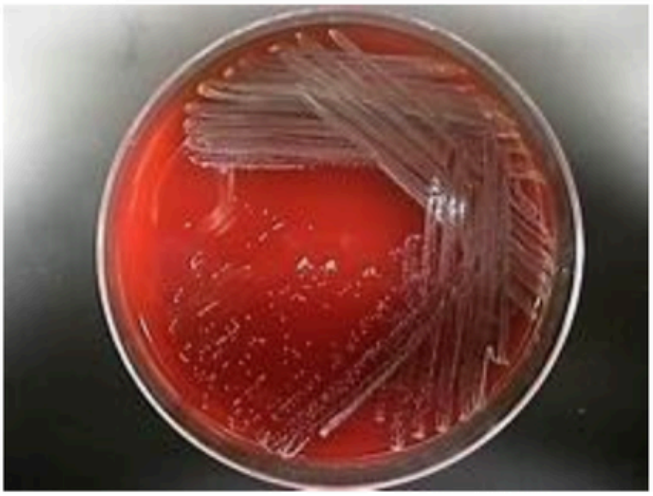
7- Identify the microorganism :

A-staph. Aureus

B- Corynebacterium diphtheria

C- streptococcus pyogens

D-Hemophilus influenza



ANSWER: **B**

8- the identified microorganism is :

A- Gram (+) cocci

B-Gram (+) rods

C-Gram (-) cocci

D-Gram (-) rods

ANSWER: **B**

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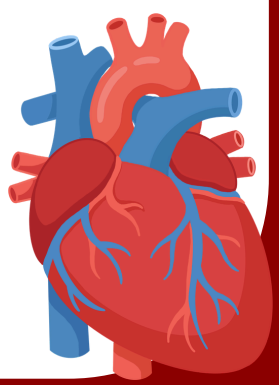


Practical test bank

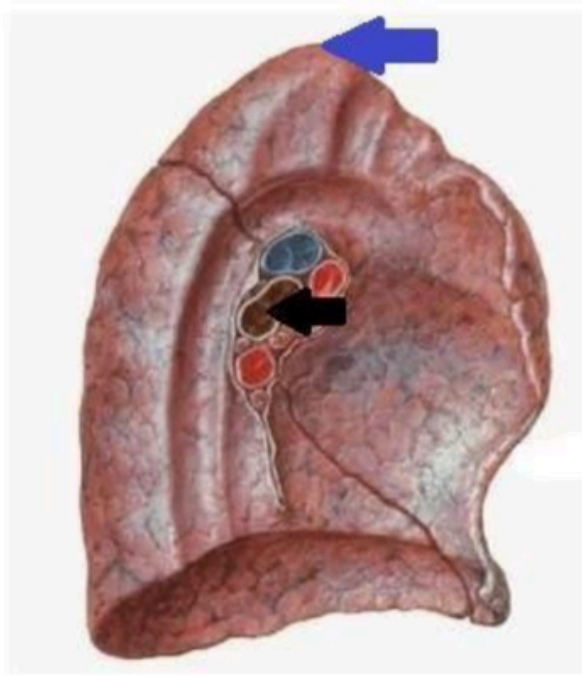
5- what's the marked structure? (black)

A-pulmonary vein

B-pulmonary artery

C-bronchus

D-bronchial artery



ANSWER: **C**

6- the marked area in black covered by ?

A-cervical pleura

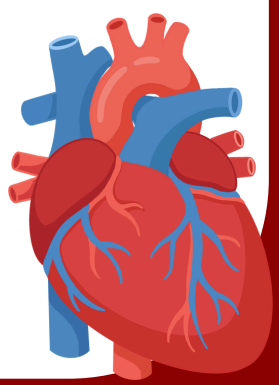
B-diaphragmatic pleura

C-cardiac pleura

D-costal pleura

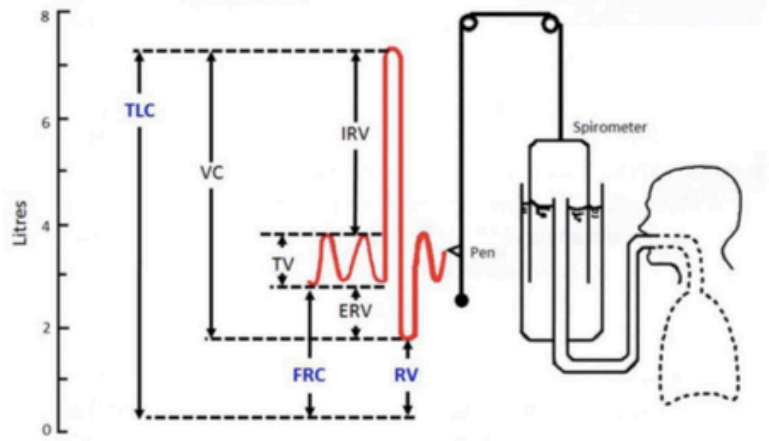
ANSWER: **A**

#	#	#	#



Practical test bank

9- what is the apparatus in the graph?



A-Spirometer

B-Spirograph

C-pulse oximetry

D-oxygen thereby

ANSWER: **A**

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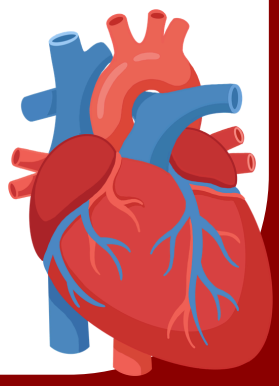


Practical test bank

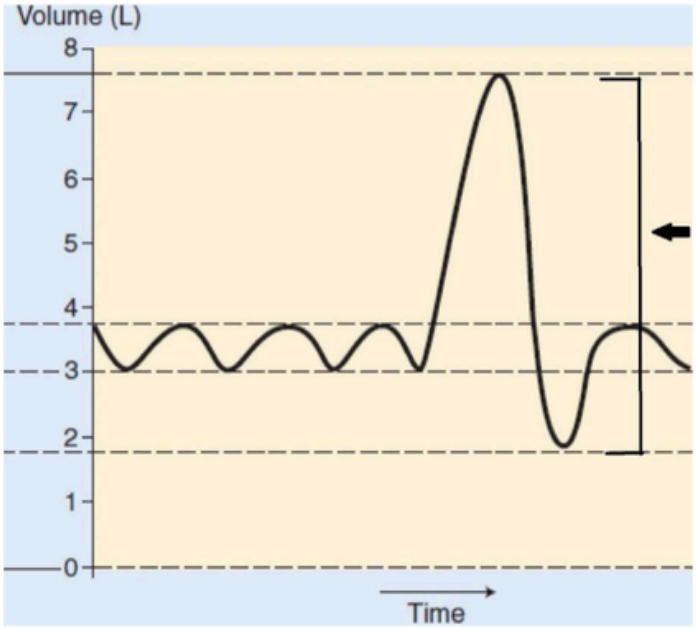
10- An asthmatic patient took an inhaler, what happened to FEV1 post the inhaler?

A-FEV1 is enhanced

B-FEV1 is worsened

C-It doesn't effect FEV1

ANSWER: **A**



11- graph showing:

A-inspiratory reserve volume

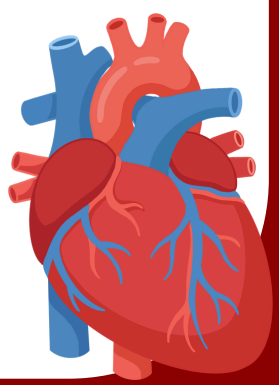
B-expiratory reserve volume

C-vital capacity

D-tidal volume

ANSWER: **C**

#	#	#	#



Practical test bank

12- measure the FRC :

A- 2.5

B- 2 C- 3

D- 3.5

ANSWER: **C**

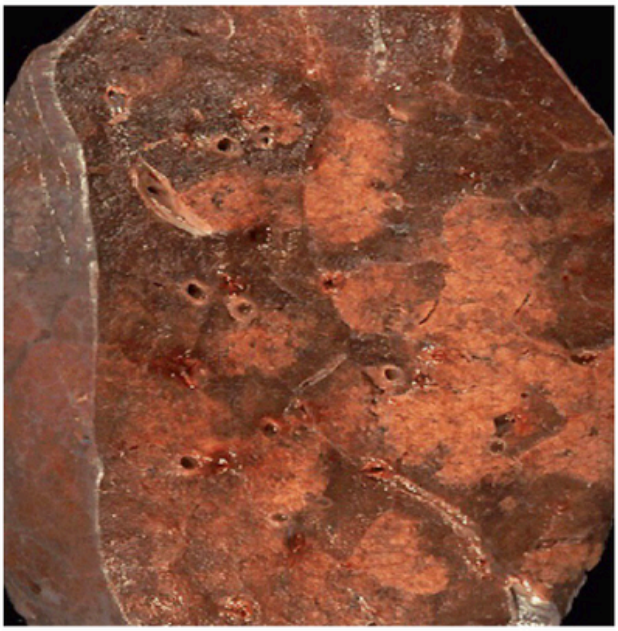
13- Identify the picture :

A-lobar pneumonia

B-atypical pneumonia

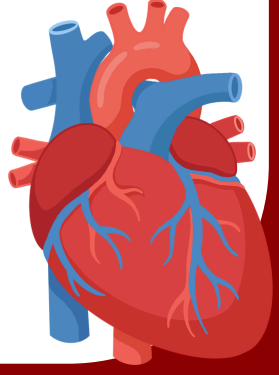
C- Bronchopneumonia

D-lung abscess



ANSWER: **C**

#	#	#	#



Practical test bank

14- description :

ANSWER: **Patchy consolidation**

15- Identify the slide :

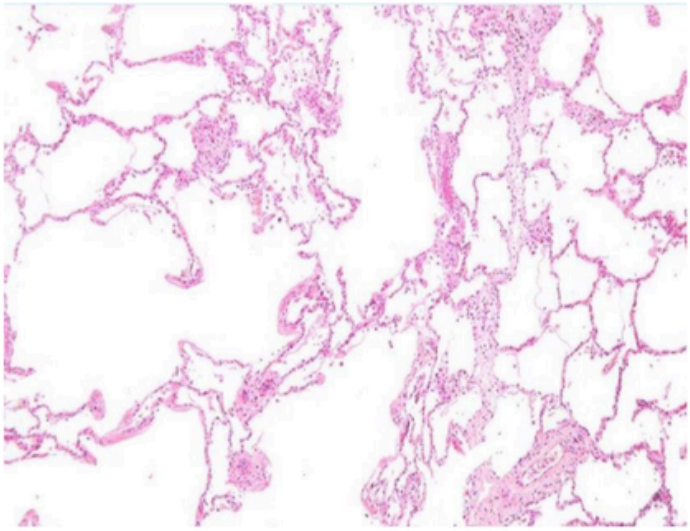
A- lung abscess

B- Bronchopneumonia

C- emphysema

D- lobar pneumonia

ANSWER: **C**



16- pathogenesis ?

A-Protease anti-protease imbalance

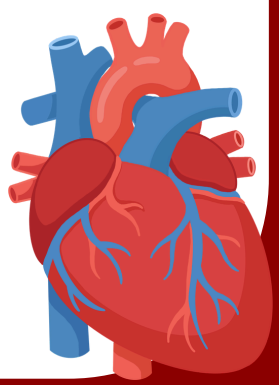
B-kartagnar syndrome

C-DVT

D-Mycobacterium Tuberculosis

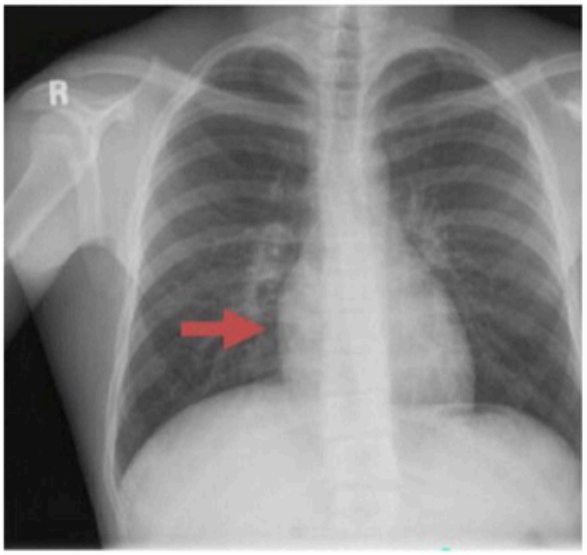
ANSWER: **A**

#	#	#	#



Practical test bank

17- Identify the structure:



A-esophagus

B- trachia

C-left ventricle

D- right atrium

ANSWER: **D**

18-what part of lung is attached:

A-left lower lobe

B- Right middle lobe

C- right lower lobe

D-right upper lobe

ANSWER: **B**

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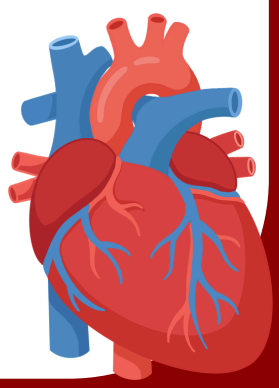


Practical test bank

RS module OSPE exam med16

1- What type of organism?

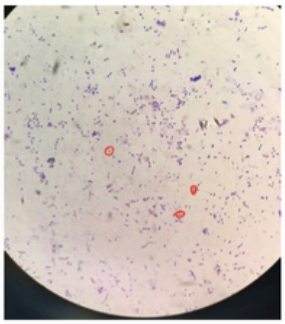
Answer :

Staphylococcus pneumonia

2- What is the type of hemolysis that appears on blood agar ?

Answer :

Alpha hemolysis



1- Describe the following gar :

Answer :

Lung abscess

2- Describe its morphology:

Answer :

Irregular cavity with necrotic material

1- Describe the following gar :

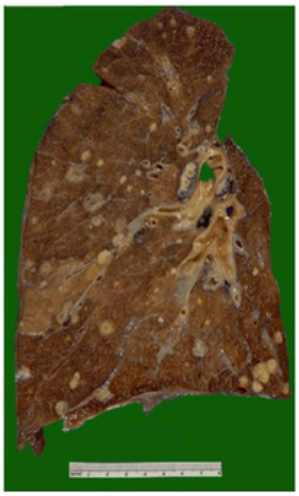
Answer :

Metastatic to the lung

2- Describe the morphology:

Answer :

Dense multiple nodules



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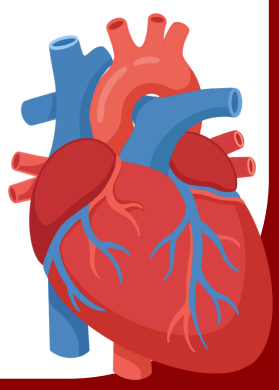


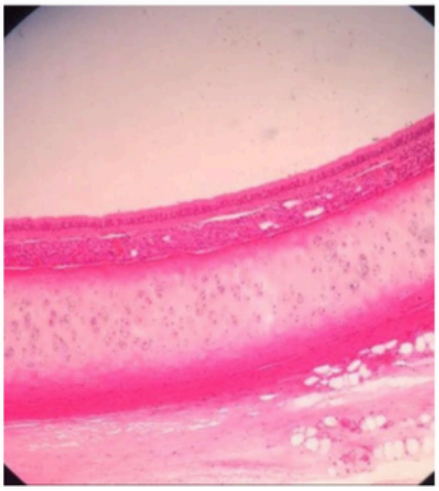
Practical test bank

RS module OSPE exam med16

1- What is the lining of the structure ?

Answer :

Pseudostratified columnar epithelium



2- The seromucous gland is present in which layer ?

a. Epithelial

b. Lamina probria

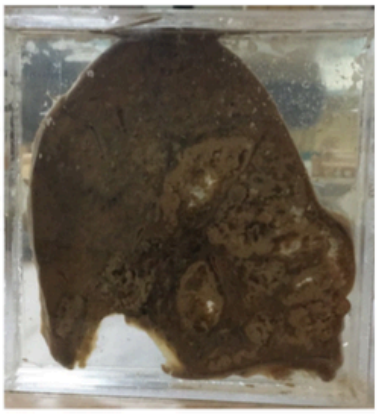
c. Submucosa

Answer : c

1- The causative agent is :

Answer :

Mycobacterium tuberculosis



1- The arrow is pointing to :

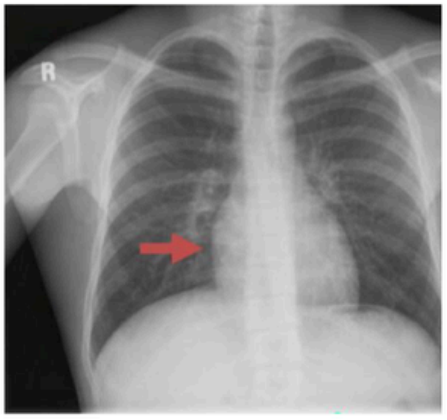
a. Right atrium

b. Inferior vena cava

c. Right ventricle

d. Thoracic aorta

Answer : a



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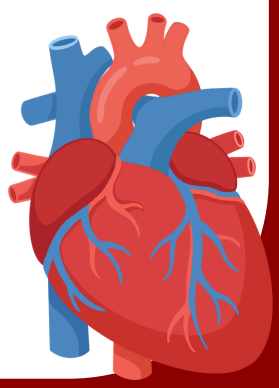


Practical test bank

RS module OSPE exam med16

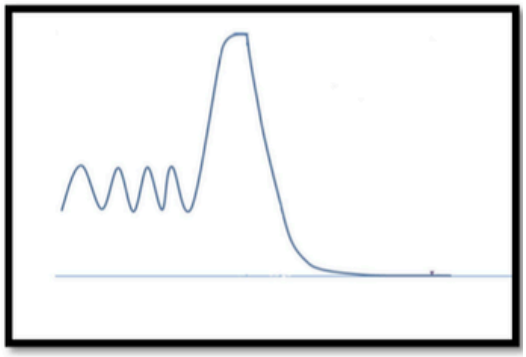
1- This test is used to test :

a. Old and young

b. Obstructive and restrictive lung diseases

c. Obstruction and obesity

Answer : b



1- The marked groove (black arrow) contain the opening of :

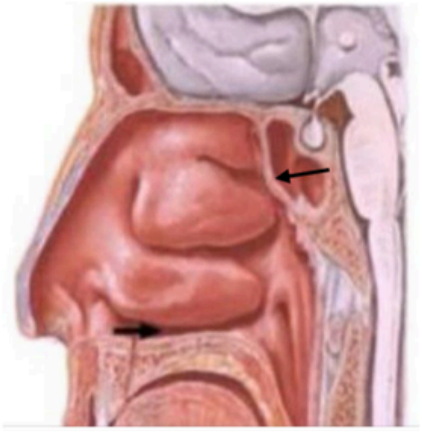
Answer :

Nasolacrimal duct

2- The marked sinus (blue arrow) opens in the :

Answer :

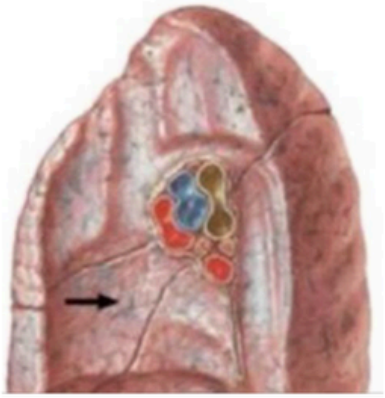
Sphenoethmoidal recess



1- The structure related to the marked area is :

Answer :

Right atrium



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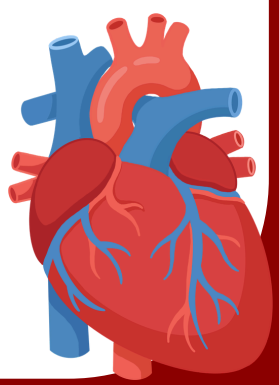
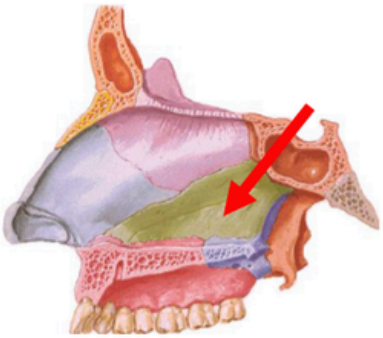
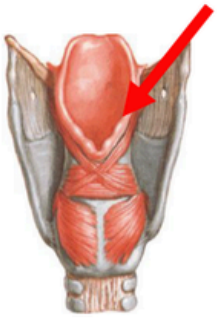


Practical test bank



1. The marked part contains:
   a. Sphenoid bone
   b. Cribriform plate of ethmoid
   c. Nasal bone
   d. Vomer bone

D



2. The marked muscle is supplied by:
   a. External laryngeal nerve.
   b. Internal laryngeal nerve.
   c. Glossopharyngeal nerve.
   d. Recurrent laryngeal nerve.

A

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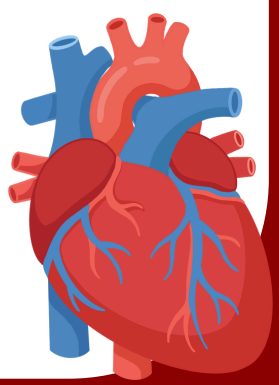
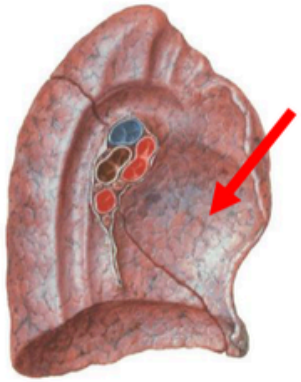


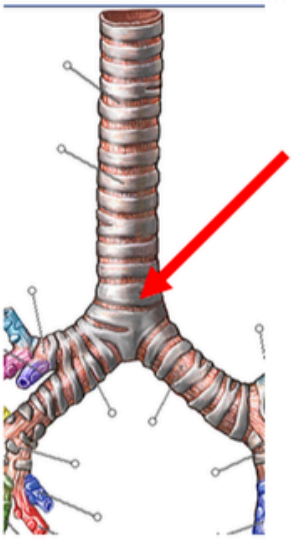
Practical test bank

Left Lung
Medial View



3. The marked impression is related to:
    a. Right atrium
    b. Left ventricle
    c. Right ventricle
    d. Left atrium

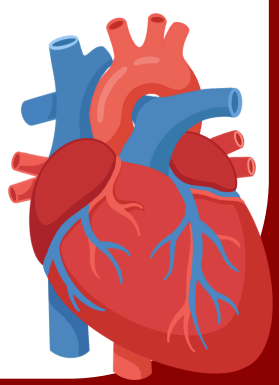
B



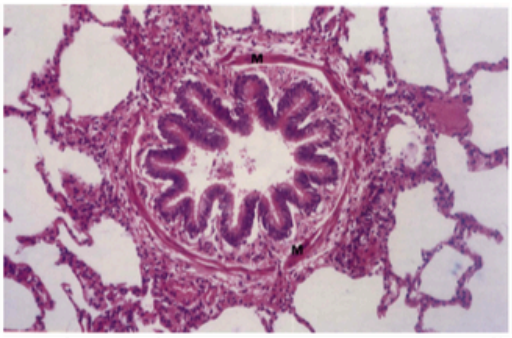
4. The marked part is present at level of:
    a. Fifth thoracic vertebra
    b. Fourth thoracic vertebra
    c. Second thoracic vertebra
    d. Third thoracic vertebra

B

#	#	#	#

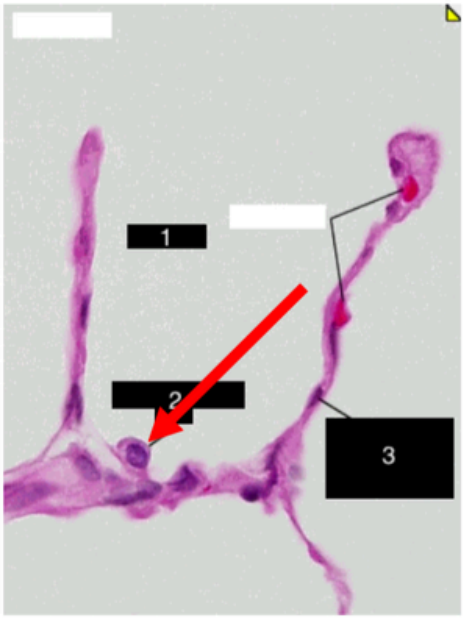


Practical test bank



5. Choose the correct statement regarding this structure:
    a. It contains seromucous glands in its submucosa
    b. It is a respiratory portion of the respiratory system
    c. It has cartilage plates in its wall
    d. It is lined with ciliate simple columnar epithelium

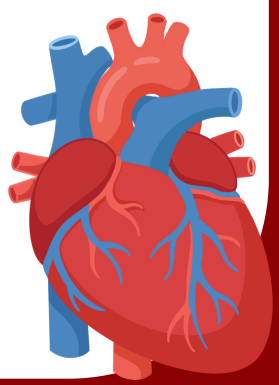
D



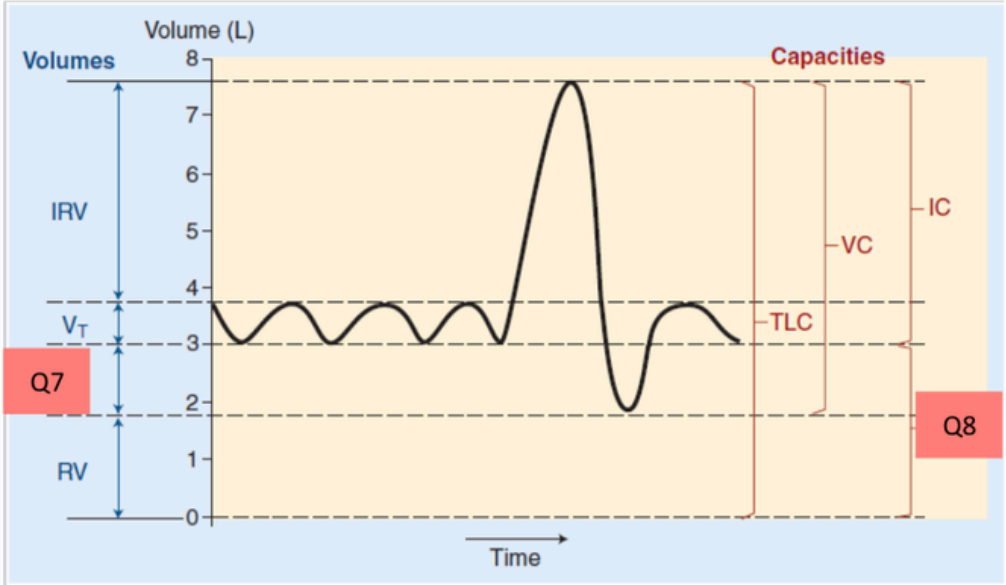
6. Identify a cell type:
    a. Alveolar type I cell
    b. Macrophage
    c. Clara cell
    d. Alveolar type II cell

D

#	#	#	#



Practical test bank



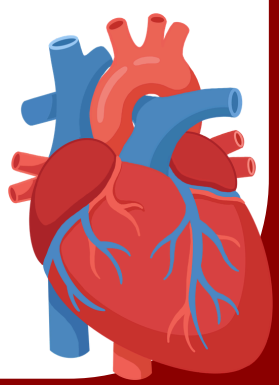
7. The following marked segment of spirogram represents:
    a. Functional residual capacity
    b. Expiratory reserve volume
    c. Inspiratory capacity
    d. Inspiratory reserve volume

B

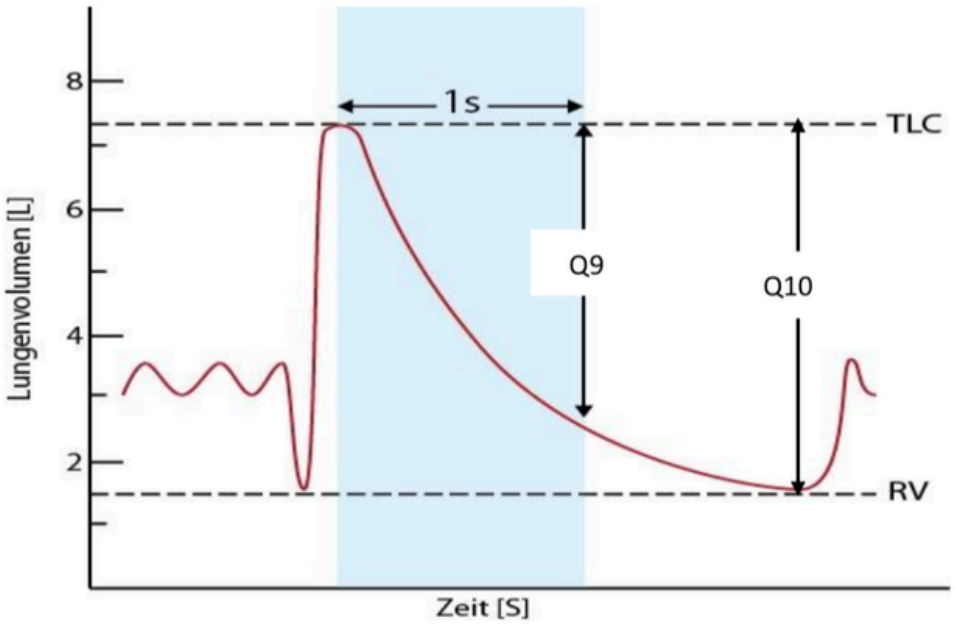
8. The following marked segment of spirogram represents:
    a. Residual volume
    b. Expiratory reserve volume
    c. Functional residual capacity
    d. Inspiratory reserve volume

C

#	#	#	#



Practical test bank



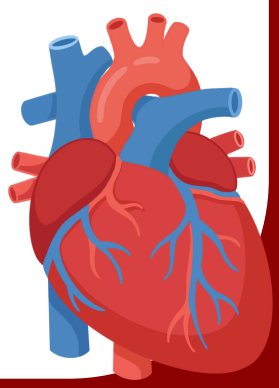
9. The following marked segment of spirogram represents:
    a. FVC
    b. FEV1
    c. ERV
    d. TLC

B

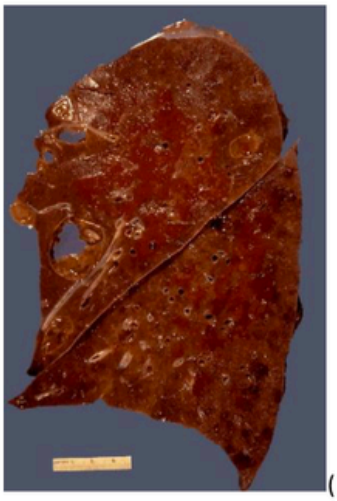
10. The following marked segment of spirogram represents:
    a. ERV
    b. FVC
    c. FEV1
    d. TLC

B

#	#	#	#

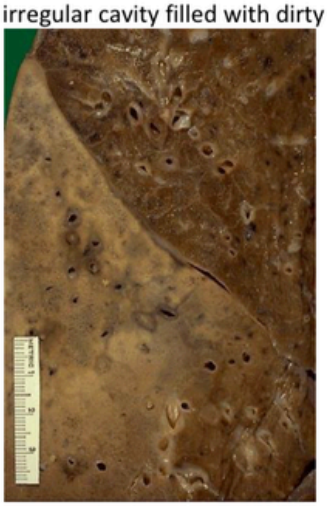


Practical test bank


(not sure about the pic)

11. The best description of this lung lesion is :
    a. Multiple variable sized nodules scattered throughout the lung
    b. Lobe of the lung with large area of consolidation
    c. Lung lobe showing multiple foci of caseating necrosis
    d. Lung lobe showing irregular cavity filled with dirty yellow material

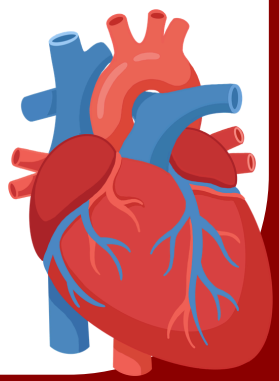
B



12. Identify the given lesion, then answer the question which of the following is a complication of such lesion:
    a. Pulmonary infection
    b. Lung abscess
    c. Carcinoma
    d. Left sided heart failure

B

#	#	#	#

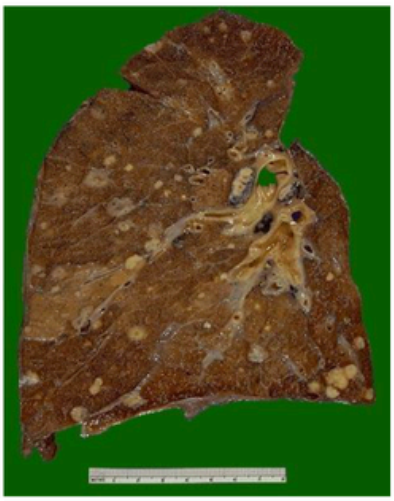


Practical test bank

(not sure about the pic)

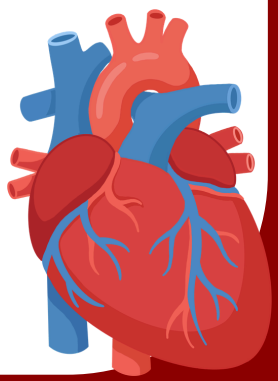
13. A portion of lung that shows.....
    a. Diffuse lobar consolidation with central cavitation
    b. Single capsulated round mass
    c. Extensive area of fibrosis replacing the lung parenchyma
    d. A single irregular gray white infiltrating mass

D


(not sure about the pic)

14. The given lesion in this image most likely represents:
    a. Inflammatory process
    b. Obstructive lung disease
    c. Malignant neoplasm
    d. Benign neoplasm

C



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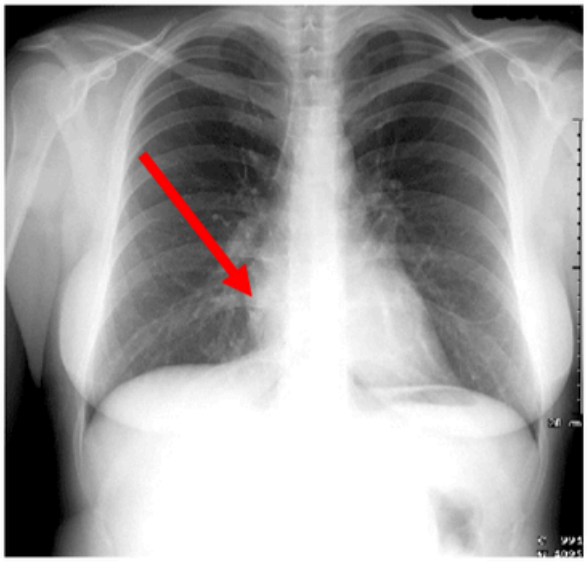
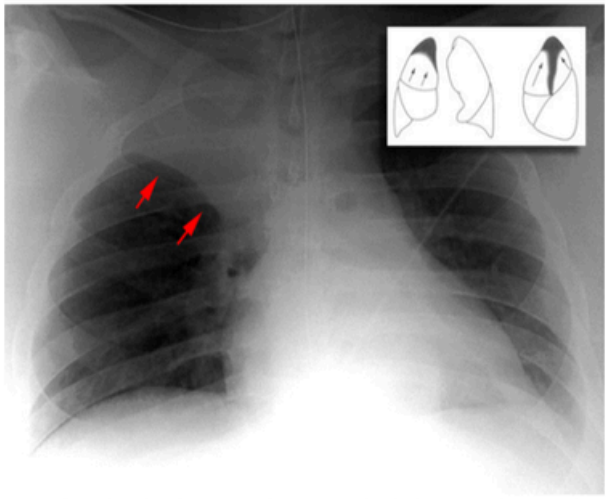
Practical test bank



15. The arrow is pointing to:
    a. Thoracic aorta
    b. Right ventricle
    c. Right atrium
    d. Inferior vena cava

C



16. The most likely cause for the findings in the provided chest radiograph is:
    a. Massive right-sided pleural effusion
    b. Pulmonary edema
    c. Right lung collapse
    d. Left pneumothorax

C

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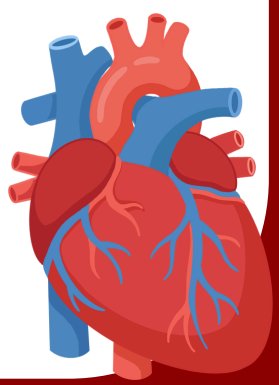
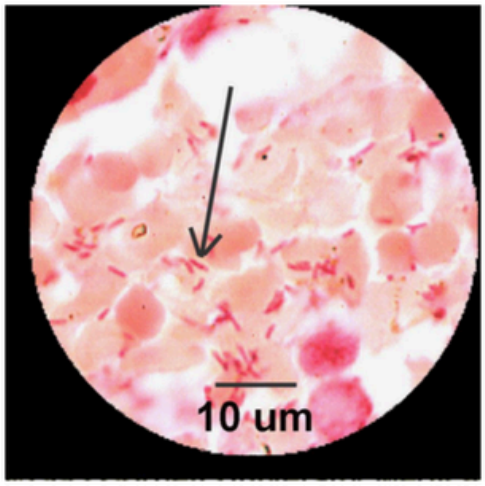
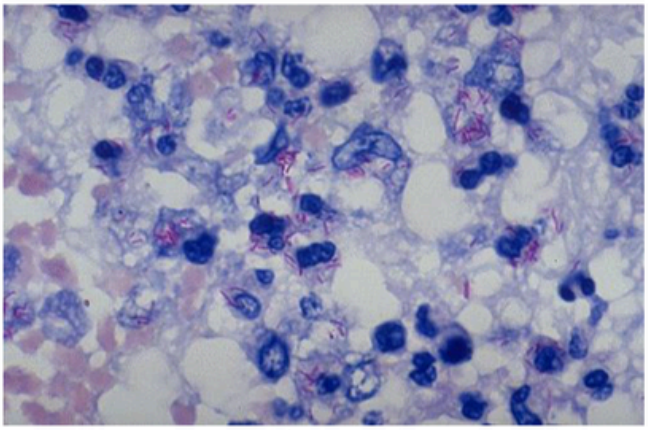


Practical test bank



19. Sputum sample from a patient with pneumonia reveals the following organism on microscopy. this the sensitive agent is likely to be:
    a. Staphylococcus pneumoniae
    b. E.coli
    c. klebsiella pneumoniae
    d. Proteus mirabilis

C



20. A patient with night sweats, weight loss, fever and cough with blood stained sputum, Gram stain is negative, bipedal stain used shows the following microscopy, The causative agent is likely to be:
    a. klebsiella pneumoniae
    b. E.coli
    c. mycobacterium tuberculosis

C

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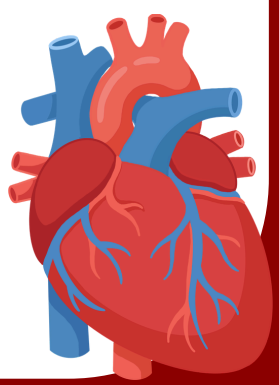
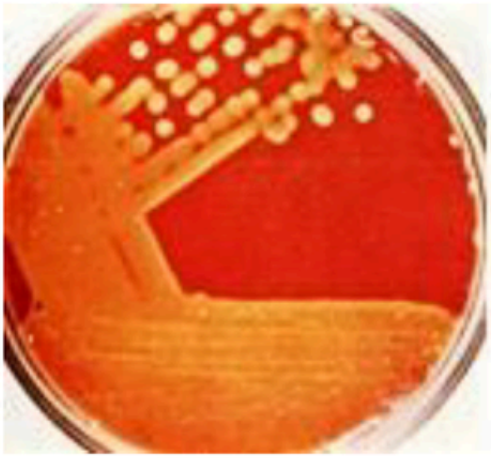
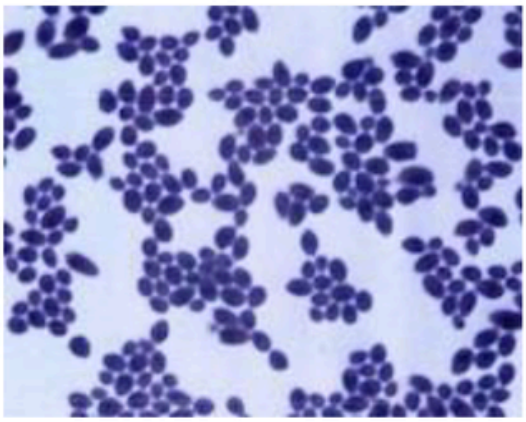


Practical test bank



17. Throat swab from a patient with pharyngitis reveals the following organism, the causative agent is likely to be:
    a. Staphylococcus epidermidis
    b. Staphylococcus pneumoniae
    c. Staphylococcus pyogenes
    d. Staphylococcus aureus

C



18. Patient presents with white patches in the mucosa. Microscopy shows the following figure, the causative agent is likely to be:
    a. Staphylococcus pyogenes
    b. Candida albicans
    c. Staphylococcus
    d. Bacterium diphtheria

B

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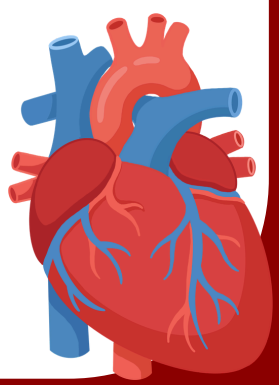
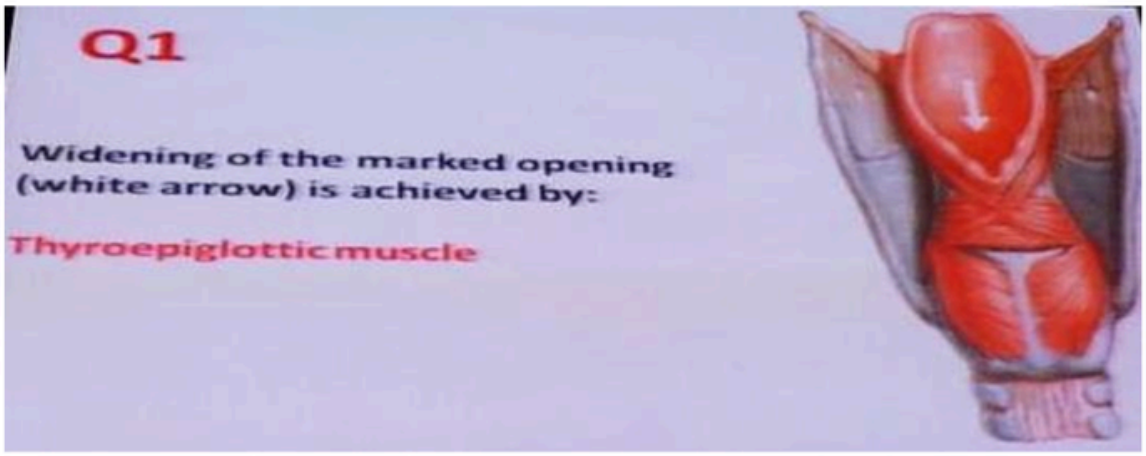
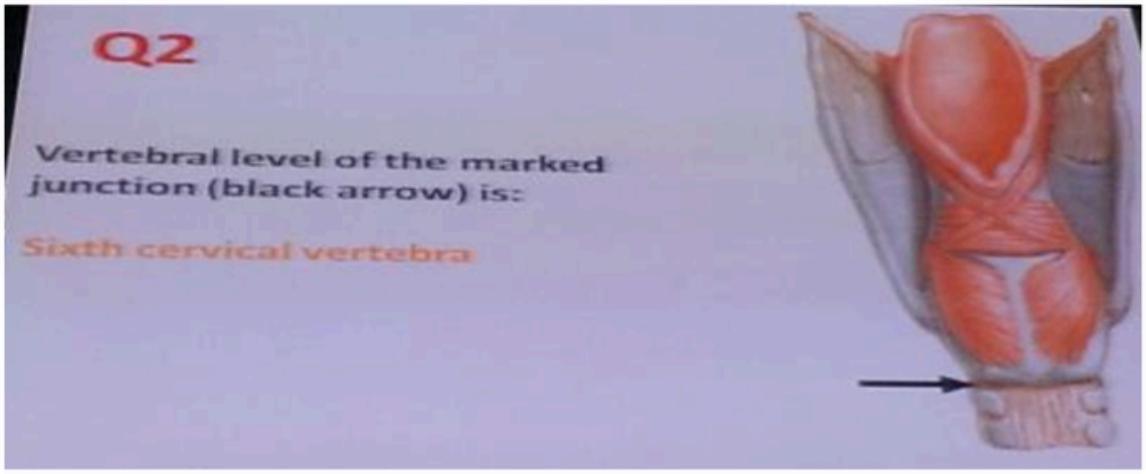
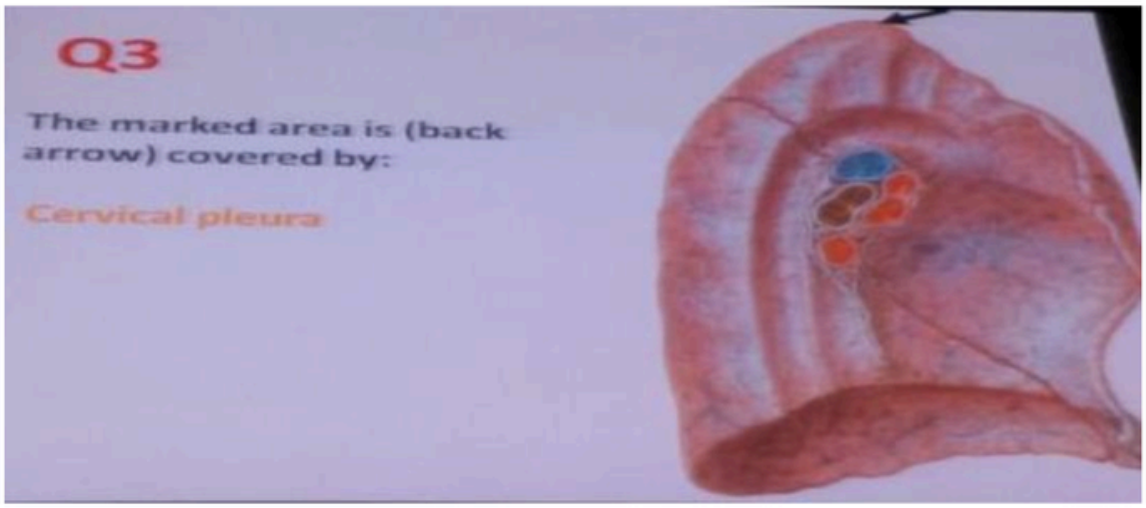


Practical test bank







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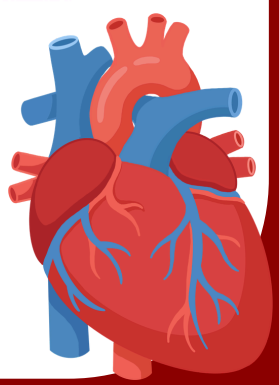
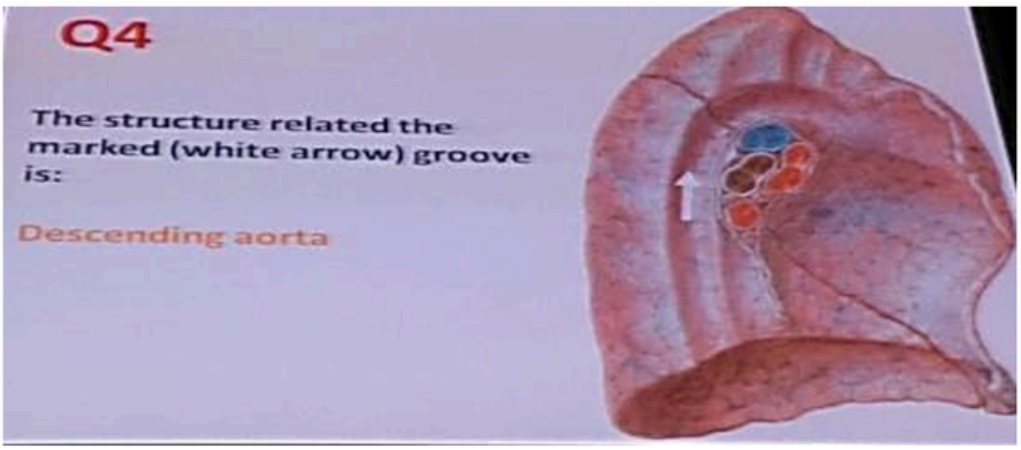
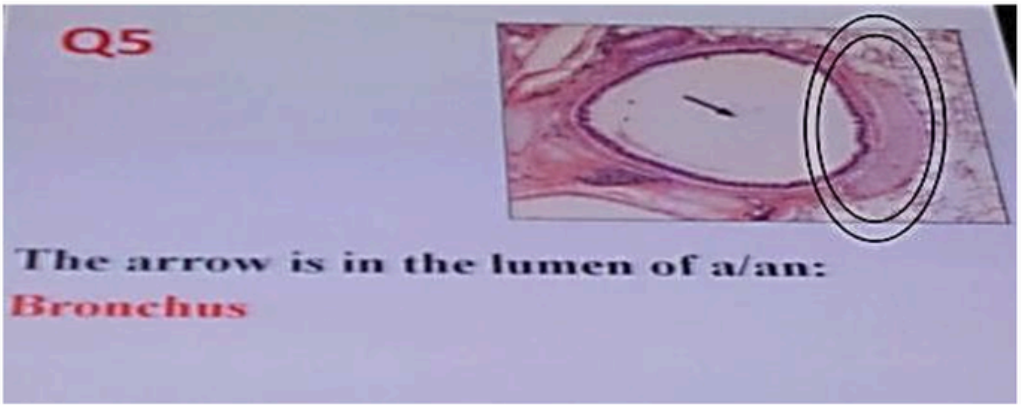


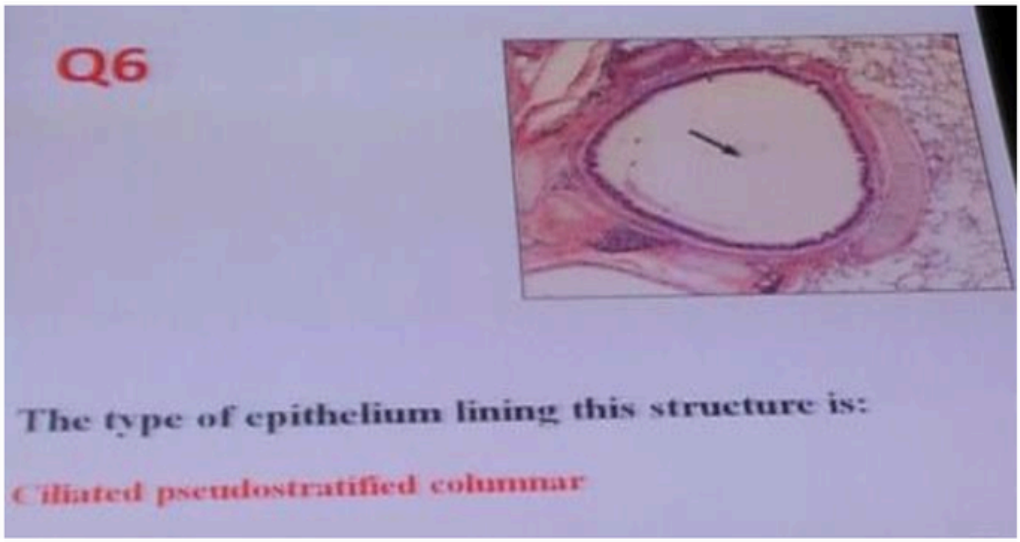
Practical test bank





Q5 explanation: Why this is bronchus ?

Because of the presence of cartilage .



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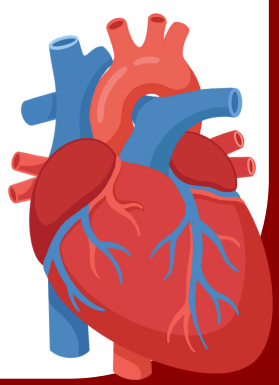
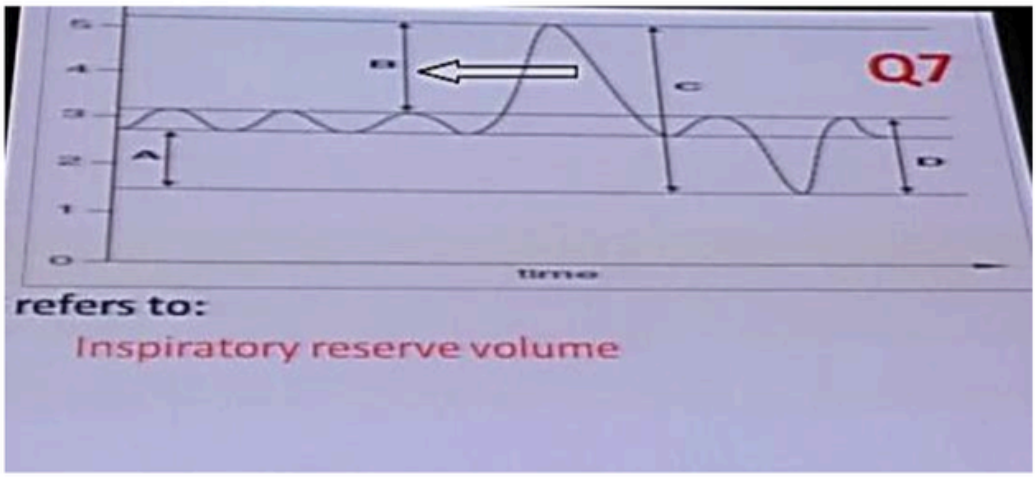
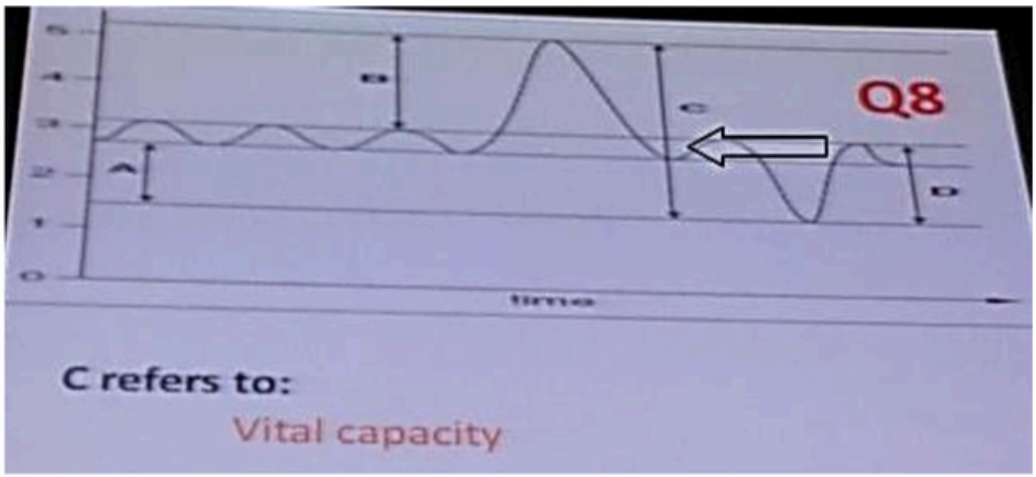
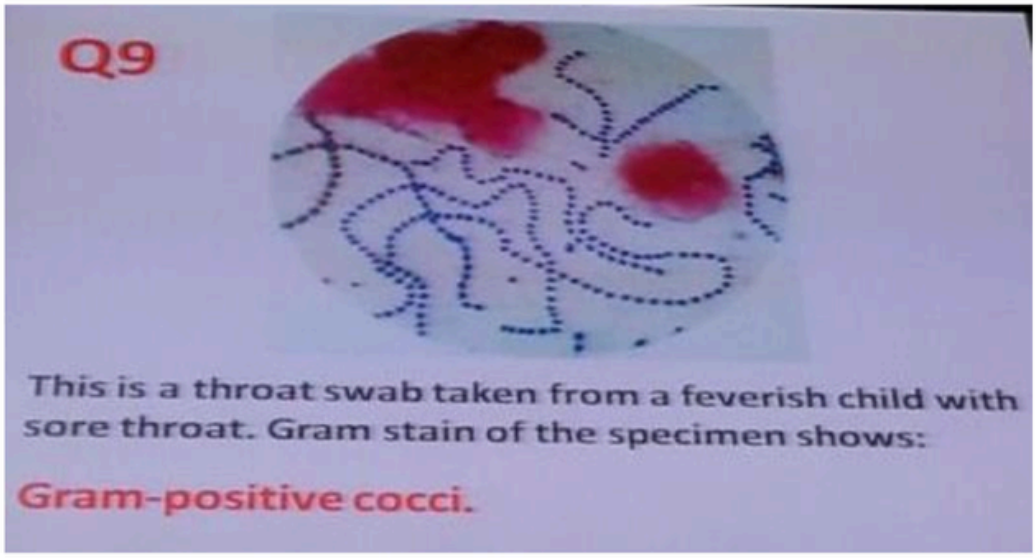


Practical test bank







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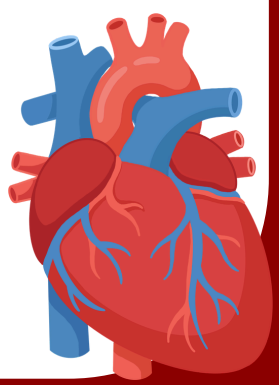
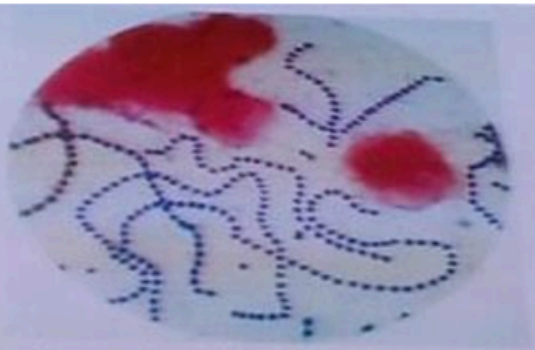


Practical test bank

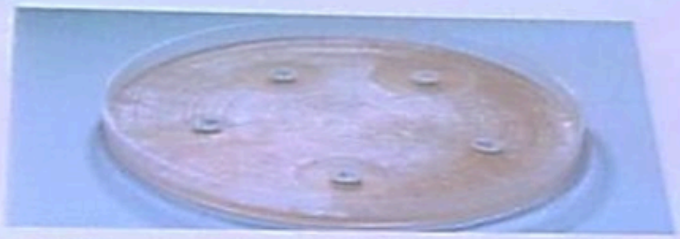
Q10



This is a throat swab taken from a feverish child with sore throat. The causative organism is:

Streptococcus pyogenes.

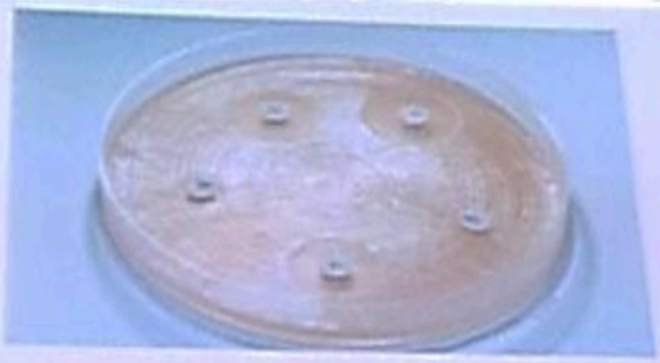
Q11



This is a plate showing antibiotic sensitivity test. For this test, the sample should be taken from the patient:

Before start of antibiotic treatment

Q12



This is a plate showing antibiotic sensitivity test. The best antibiotic chosen is that:

Shows the largest zone of inhibition

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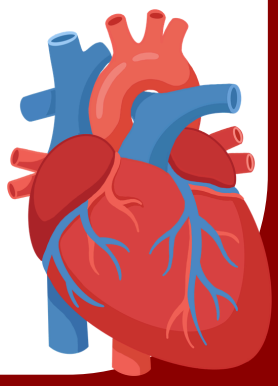
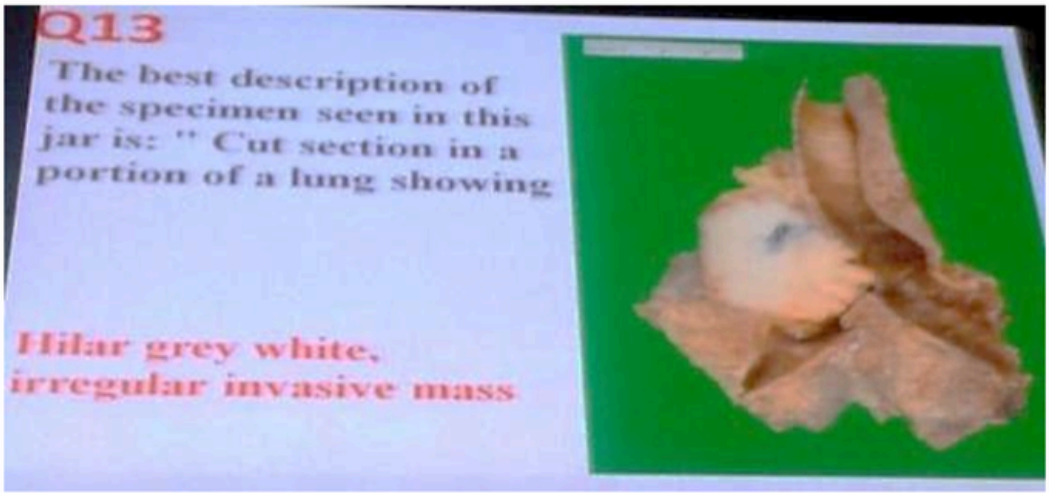
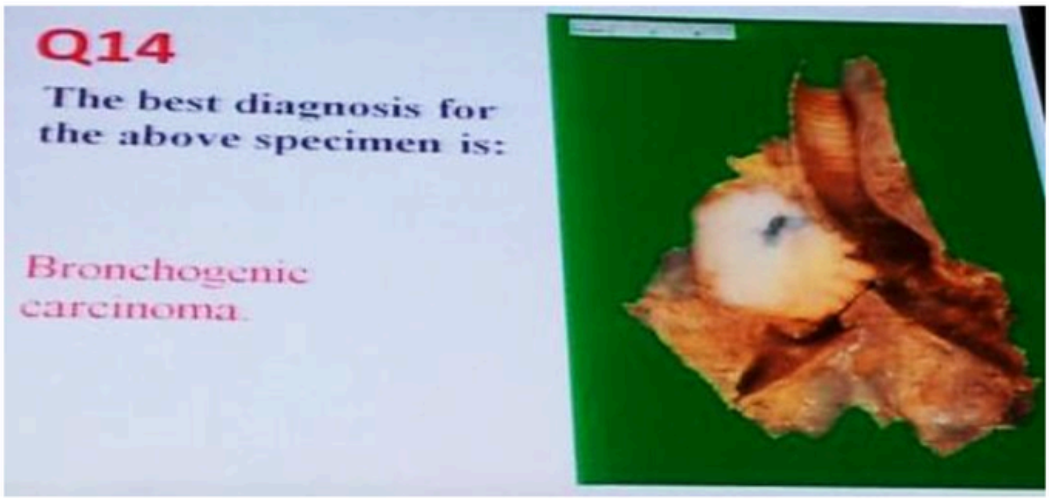
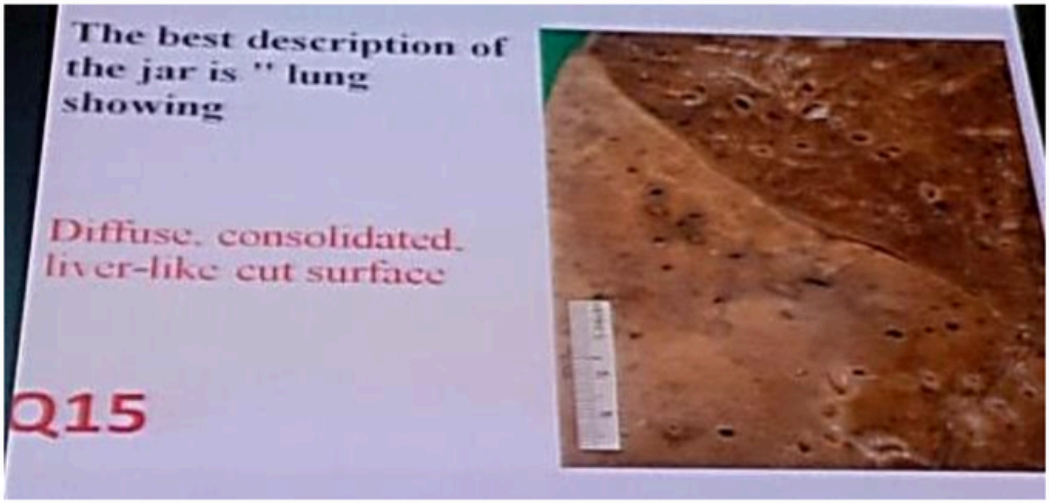


Practical test bank







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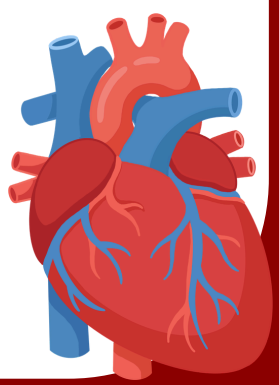
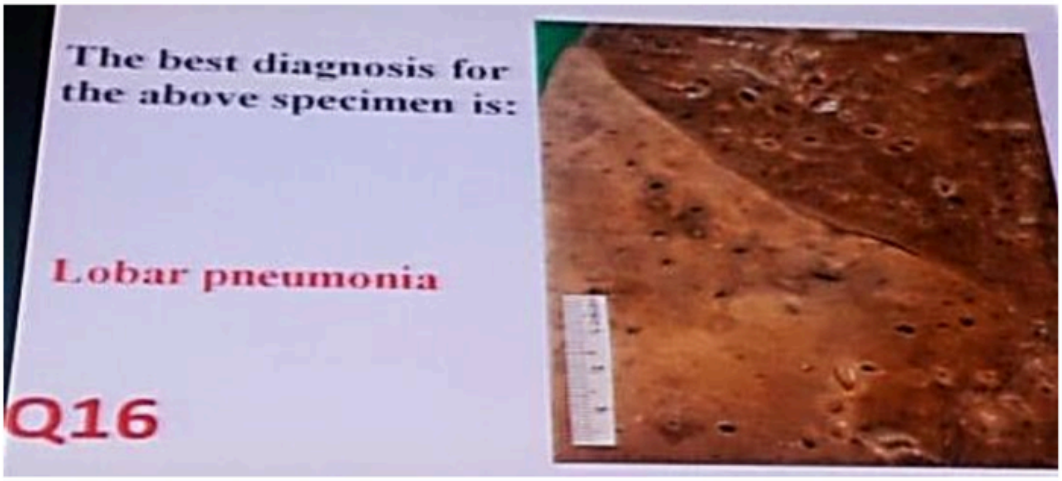


Practical test bank



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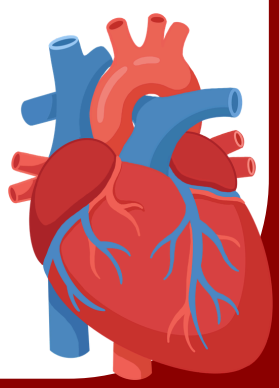
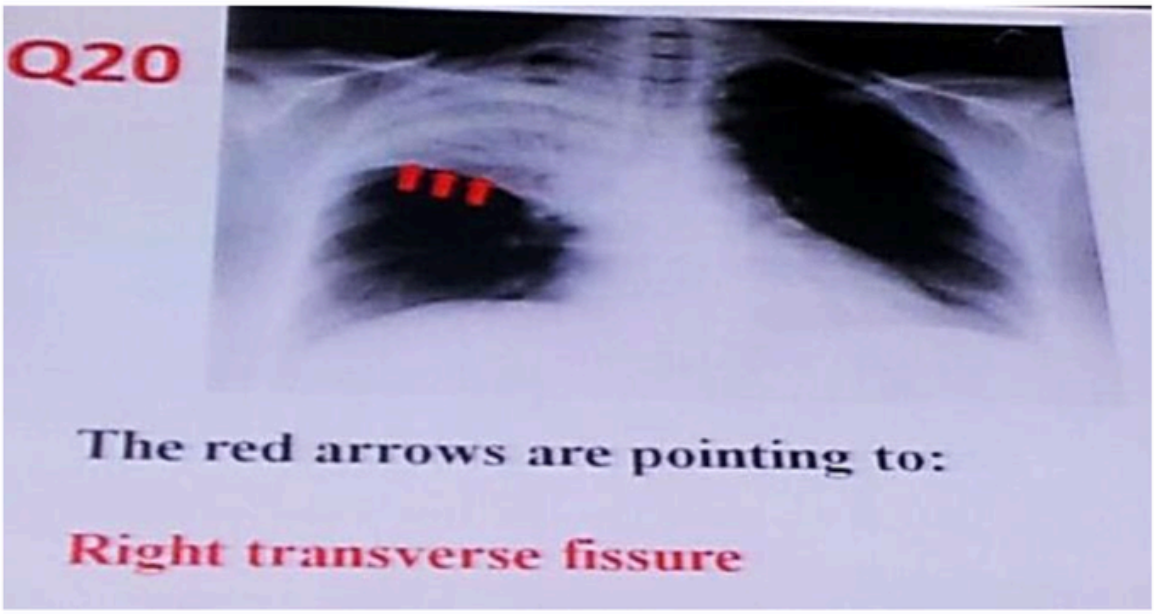


Practical test bank



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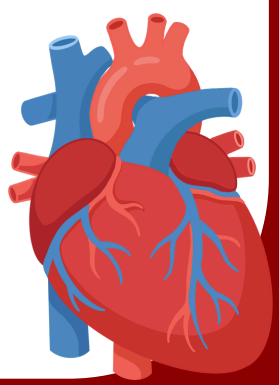

الحمد لله حمداً كثيراً طيباً مباركاً فيه